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Central project evaluation

Support to social protection including social health protection, Pakistan (Project number 2019.2140.2)

Evaluation Report

On behalf of GIZ by Justine Garrigue (Syspons) and Sabina Ahmad (consulting)
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Abbreviations

ADB	Asian Development Bank
AJK	Azad Jammu and Kashmir
AO	(BMZ marker) Poverty Orientation
ASP	Adaptive Social Protection
AV	Auftragsverantwortlicher; person responsible for the commission, sometimes seen as the project manager
BHCP	Balochistan Health Card Programme
BISP	Benazir Income Support Programme
BMZ	German Federal Ministry for Economic Cooperation and Development
BRS	Beneficiary Registration System
CSA	Civil Service Academy
DAC	Development Assistance Committee
FGD	Focus Group Discussion
GB	Gilgit-Baltistan
GF	Gates Foundation
GG	(BMZ marker) Gender Equality
GIZ	Deutsche Gesellschaft für Internationale Zusammenarbeit (GIZ) GmbH
HSA	Health Services Academy
ILO	International Labour Organization
IMSciences	Institute for Management and Sciences
KfW	German Development Bank
KP	Khyber Pakhtunkhwa
KPIs	Key Performance Indicators
MIS	Management Information Systems
MoPASS	Ministry for Poverty Alleviation and Social Safety
MO	Module Objective
M&E	Monitoring and Evaluation
NADRA	National Database and Registration Authority
NRSP	National Rural Support Programme
NSER	National Socio-Economic Registry
OECD	Organisation for Economic Co-operation and Development
PD/GG	(BMZ marker) Participatory Development/Good Governance
PHIMC	Punjab Health Initiative Management Company
PIDE	Pakistan Institute of Development Economics
PSPA	Punjab Social Protection Agency
SDGs	Sustainable Development Goals
SDPI	Sustainable Development Policy Institute

SHP	Social Health Protection
SHPI	Social Health Protection Initiative KP
SLIC	State Life Insurance Cooperation of Pakistan
SOP	Standard Operating Procedure
SP	Social Protection
SSP	Sehat Sahulat Programme
TC	Technical Cooperation
UHC	Universal Health Coverage
WB	World Bank
WHO	World Health Organization

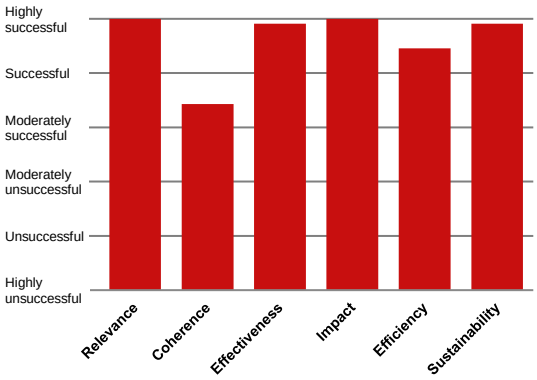
Executive Summary

Project title	Support to social protection including social health protection
Country/region/global	Pakistan
Sector and creditor reporting system code	16010: Social Protection (100%)
Project number	2019.2140.2
Commissioning party/co-financier	German Federal Ministry for Economic Cooperation and Development (BMZ), Gates Foundation (GF)
Lead executing agency/partner organisation(s)	Ministry of Finance, Revenue and Economic Affairs
Development cooperation (DC) programme	Social Protection in Pakistan
Implementing organisations of the DC programme	GFA Consulting Group, Oxford Policy Management, Nutrition International, Center for Economic and Policy Research
Project value	EUR 17,370,054, of which EUR 2,370,054 is co-financed by the GF
Project term	October 2020 – September 2025
Sample year CPE	2022

Objectives of the module

The module aimed to contribute to improving the conditions for the targeted and effective implementation of social protection programmes so that the population in the selected provinces is better protected against poverty caused by health issues, economic hardship and to a certain extent climate-induced crises.

Overall rating: Successful (88/100)



Context of the project

Structural poverty continues to be a challenge for welfare in Pakistan, a situation only exacerbated by the COVID-19 pandemic, economic crisis, inflation and natural disasters. Political instability and fluctuating governance have affected the consistency and implementation of social protection programmes. A significant proportion of the population living below the poverty line lack social safety nets, leaving them exposed to financial shocks from illness, accidents, unemployment, climate risks and extreme weather events. This situation has further increased the need for robust social protection systems in the country. However, the fragmentation of social protection programmes has led to inefficiencies, duplications and gaps in coverage, with only a fraction of the population benefiting adequately from these initiatives. Gender disparities with regard to social protection also remain a significant concern, considering women typically have a higher risk of poverty and are often the main and targeted beneficiaries. Knowledge of social protection programmes and their eligibility criteria is often not widespread among the population who should benefit from it, especially for women who in some rural areas are largely excluded from public life and have greater difficulty accessing social protection programmes than men. The policy and regulatory environment have seen attempts to address these challenges, e.g. through the Ehsaas programme, which aimed to consolidate existing social safety nets and address multidimensional poverty, launched in 2019 (but discontinued with government change). Despite ambitious goals, the implementation of Ehsaas and other initiatives has been hampered by coordination challenges, underfunding, and inclusion and exclusion errors (e.g. likely fraudulent behaviour of service officers signing up their families or limited coverage in hard-to-reach areas). In 2022, the country experienced devastating floods which affected

over 33 million people, displaced communities, destroyed infrastructure worth billions of dollars, and exposed the vulnerabilities in Pakistan's emergency response and readiness of the social protection systems.

Brief description of the project

The **module's objective** was to improve the supply- and demand-side conditions for the implementation of social protection programmes. The module combined its support for policy, technical and technological capacity development. The aim was to strengthen local systems, enhance governance (by addressing policy gaps through political engagement and fostering interprovincial coordination), facilitate data availability (including through data-sharing mechanisms) for evidence-based decision-making, and integrate the links between social protection programmes and better nutrition for adolescent girls and their communities. The main hypotheses selected were: 1) whether the support to operationalising a communication strategy contributed to increasing the familiarity of the population with SHP programmes; 2) whether the development of Monitoring and Evaluation (M&E), based on key performance indicators and a dashboard, increased the capacities of the Sehat Sahulat Programme (SSP) to analyse the beneficiaries' data, to adapt their programme and integrate their national health insurance with other SP programmes, for evidence-based decision-making; and 3) whether the establishment of (mobile) registration centres contributed to improving the registration of beneficiaries into the dynamic National Socio-Economic Registry under the Benazir Income Support Programme (BISP) and other social protection programmes, to improve the connection and information-sharing between supply and demand sides in social protection programmes, and ultimately to improve the response to these needs. A predecessor module was implemented from July 2016 until September 2020, when the current module commenced.

Table: Project objective/areas of intervention

Project objective	The supply- and demand-side conditions for the implementation of social protection programmes have improved.
Output 1	The Social Health Protection Initiative in Khyber Pakhtunkhwa (KP) and the SSP in Punjab and in the federally administered territories are strengthened.
Output 2	Greater evidence-based decision-making is made.
Output 3	Social protection capacities for coordination in the provinces are reinforced.
Output 4	Mechanisms of the BISP for interacting with the target groups are more efficient.
Output 5	The implementation of a framework for the cross-sectoral integration of social assistance and nutritional support for adolescent girls has begun in selected parts of the country.

Assessment according to DAC criteria

Relevance

The evaluation finds in dimension 1 that the module is aligned with key international frameworks (Sustainable Development Goals (SDGs) 1, 3 and 5), German policies and priorities (e.g. BMZ's 2023 German development policy with Asia, BMZ's core area strategy on health, social protection and population dynamics, and the German Feminist Development Policy). At national level, despite limited formalised government-led frameworks for social (health) protection, the module aligned with the priorities of key actors in the sector (e.g. support in the governance of the SP and SHP programmes in Pakistan, and support of key actors, such as the Health Services Academy, and systems like Zakat). In dimension 2, the evaluation also finds the module well aligned with the needs of the direct target groups (technical and organisational capacities, financial support, policy framework, technological/digital capacities, communication strategies and networking) and indirect target groups (reinforcement of the entire SP and SHP system to improve their protection). Dimension 3 dives into the strengths and weaknesses of the module design, starting with its solid design, built on a strong multilevel approach, the right targeting of sectors to answer the needs of the target groups, deployment of relevant and timely activities to achieve their objectives, and selection of the right partners. Some weaknesses in the

traceability of the project were observed (e.g. lack of details in the results model, centralised M&E conceptualisation of Output 2 and the compartmentalisation of SP and SHP). Lastly, dimension 4 indicates that the module maintained a high level of relevance by adapting to the emerging needs of national and international partners.

Coherence

For dimension 1, examining the internal coherence of the module with other German development cooperation actors shows good potential at the programme level, despite limited collaboration with actors dealing with financial cooperation. Within the same programme, the module effectively utilised synergies with the Adaptive Social Protection (ASP) module (i.e. shared partners, joint events and integration of SP-SHP activities into ASP) but less so with other GIZ-implemented modules. Furthermore, collaboration with KfW was limited, due to differing implementation approaches and a lack of structured mechanisms for coordination (e.g. joint planning), leading to missed opportunities for synergy. In dimension 2, the evaluation shows examples of concrete opportunities for complementarities, with national partners and international organisations. The collaboration with other international organisations such as the World Bank (WB), International Labour Organization (ILO) and Asian Development Bank (ADB) has been largely successful, demonstrating effective coordination mechanisms which evolved over time (i.e. monthly agency meetings and sector groups) and alignment of objectives. The module also paved the way in strengthening social protection coordination, notably through initiatives like a national conference with BISP, which laid the foundations for the first joint SP approach in 2023, the Cash Transfer Coordination Group, together with the ASP module in late 2023, Pakistan Ministry of Poverty Alleviation and Social Safety, ADB, and WB with inputs from UNICEF, ILO and others. Additionally, the module has fostered strategic collaborations with organisations like GF. The co-financing and collaboration with GF in nutritional support and education for adolescent girls and their households by leveraging BISP social protection programmes serves as a positive example of coordinated efforts, characterised by consistent information-sharing and the effective use of synergies. Lastly, the module actively engaged in coordinated crisis response efforts after the big floods in 2022.

Effectiveness

In dimension 1, the evaluation confirms that all four module objective (MO) indicators are on track for full achievement by the end of the module in September 2025: new processes were successfully implemented (e.g. developing guidelines, updating hospital empanelment, and standardising family unit definitions); the beneficiary knowledge of SHP programmes improved significantly (from 25% to 87%) based on surveys conducted in KP, Azad Jammu and Kashmir, and Gilgit-Baltistan; the KP provincial authority developed its first social protection policy and drafted a strategy; and the dynamic National Socio-Economic Registry (NSER) reached 16.9 million households by October 2024. In dimension 2, the evaluation finds that the module used the right strategies and mechanisms to obtain its results since two hypotheses are confirmed: the operationalisation of communication strategies contributed to increasing the knowledge of beneficiaries on SHP programmes (H1); and by supporting BISP in establishing 647 tehsil centres and introducing mobile registration vans, the module contributed to improved beneficiary registration and information-sharing in social protection (H3). The third hypothesis also shows great advances since the data confirmed that the module contributed to improved evidence-based decision-making and institutional capacities but faced challenges (such as a gap in the level of buy-in of evidence-based decision-making in Pakistan and high staff turnover). In dimension 3, the data indicates that the quality of implementation of the module is generally high, relying on strong steering mechanisms; the collaborative approach ensured strategic alignment and effective interventions. Its multilevel partner selection reinforced structural strength, inclusivity and cooperation. However, GIZ's bureaucratic processes caused delays in implementation despite well-managed administrative procedures. Finally, dimension 4 demonstrates that the module had unintended positive results, notably a data-sharing agreement between the Balochistan Health Card Programme, the provincial health card programme in Balochistan, and

BISP, influenced by its work in KP. It also addressed disability inclusion needs by developing an app in KP to assist social welfare department workers and people with disabilities and their households in accessing social protection benefits and programmes and incorporating sexual harassment awareness into its training. While monitoring of unintended results was fragmented, the module adapted to new challenges and opportunities.

Table: Achievement of the project's objective indicators

Indicator	%
Core health insurance processes were adapted on the basis of evidence-based recommendations.	100
The proportion of the target group who know the benefits and processes of the SHP programmes increased.	100 (138)
A provincial government has developed a strategy for the development of an integrated social protection system.	100
The number of people registered in the new dynamic NSER has increased.	100 (104)

Impact

At impact level, in dimension 1, the data finds indications of positive trends at structural level of: greater access to SP and SHP programmes, the establishment of dynamic registration tools, and improved governance of the SP and SHP programmes in Pakistan, among others, although the impact at country scale will need time to materialise. In addition, dimension 2 shows that the module plausibly contributed to these changes, as both hypotheses show that the module used effective mechanisms to develop a system for social protection that can be adjusted to improve the coverage and benefits for vulnerable populations for better protection (H4) and to improve the links between provinces and with the federal government (H5). Nevertheless, little information is available on improvements of the protection of vulnerable households at the beneficiary level, apart from greater access to SP and SHP programmes, since changes are still relatively recent and not yet visible and measurable. In addition, the effects of the COVID-19 pandemic, the economic crisis, the security challenges in some parts of the country and the climate-induced shocks have challenged the latest achievements of Pakistan in reducing poverty sustainably. Lastly, in dimension 3, the evaluation finds no further negative unintended impacts but one positive one was that the results of the module were used to further improve access to expanded healthcare (e.g. neonatal intensive care units, cardiotocography machines and Cath Labs).

Efficiency

Starting with **production efficiency**, the evaluation shows that the module resources were applied sensibly to the five outputs, and the instrument and staff costs were conducive to the observed results. The allocation of costs in terms of indicator achievement is also plausible, as most of the output indicators have been (over-)achieved. Some challenges were identified (e.g. political turmoil led to some delays in conducting the communication campaigns or there were challenges ensuring the right national team members could be integrated into the team); however, the latest modification offer submitted in 2024 means the evaluation is able to predict that all indicators will be achieved by the end of the module. Overall, it can be concluded that the distribution of costs between the outputs is in line with the project's foci: increasing the registration capacities (Output 4), increasing coordination (Output 3), and increasing familiarity of beneficiaries with SHP programmes (Output 1). The efficiency of the module and its approach became evident with the changes in the context (e.g, 2022 floods), as the team managed to successfully adapt to changing conditions to adjust their resources towards BISP's emergency response. **Allocation efficiency** is also rated as successful. The evaluation finds that the module has managed to effectively transform its activities into its outcomes. This is shown by the overall expected achievement at outcome level. While there might be some potential for cost minimisation, since the MO indicators have mostly been achieved with time and budget remaining, the module generally aimed to maximise results beyond the module outcomes. Allocation efficiency comes from identifying opportunities for scalable impact and new cooperation prospects, such as with the BISP programme, which is

the largest provider of social cash transfers in Pakistan; however, opportunities with German DC projects were only identified to a limited extent.

Sustainability

The evaluation concludes that, in dimension 1, the direct target groups have strong capabilities that they can use, especially technical, institutional and technological capacities and tools. Institutional capacity, although currently strong, could be tied to a single person which would affect the use over time at the level of entire institutions. The indirect target groups show promising examples of capacities that could be used over time but cannot be entirely confirmed yet. Dimension 2 shows that the module has established a strong foundation for sustainable social protection in Pakistan through strategies supporting institutional (e.g. integrated sustainability approach, stakeholder engagement, needs-based approach), technological (e.g. development of tools and systems to reinforce actors at multiple levels relevant for the sectors), and financial sustainability (e.g. tailoring its activities to the partners' capacities and adopting a pilot approach), though it lacks systematic monitoring and documentation of sustainability outcomes. Lastly, in dimension 3, while risks such as the politicisation of SP and SHP sectors, staff turnover, funding constraints, and data security concerns present significant challenges, the module has adopted successful strategies to alleviate some of these challenges (such as policy work, integration of sustainability across activities since their initiation, and stakeholder engagement) and the ongoing institutional reforms offer promising pathways to ensure the durability of the module's results.

Overall rating

Considering all the evidence described above, the programme is rated successful, with 88 points out of 100. The strengths and weaknesses of the module are presented in the table below.

Table: Rating of OECD DAC evaluation criteria

Criteria	Score (Max. 100)	Rating 1 (highly successful) to 6 (highly unsuccessful)	Criteria	Score (Max. 100)	Rating 1 (highly successful) to 6 (highly unsuccessful)
Relevance	92	Level 1: highly successful	Impact	93	Level 1: highly successful
Coherence	73	Level 3: moderately successful	Efficiency	86	Level 2: successful
Effectiveness	91	Level 1: highly successful	Sustainability	91	Level 1: highly successful
Overall				**88**	**Level 2: successful**

Conclusions and factors of success and failure

The following are examples of success factors:

- ☑ The systemic approach to SP-SHP provides comprehensive support by combining support for policy, technical and technological capacity development to improve the coverage and protection of the poorest-of-the-poor households.
- ☑ In line with the systemic approach, the module also worked with actors to increase outreach and impact using multilevel tactics.
- ☑ The module relied on an adaptable design for the development of the SP-SHP sector to address the key barriers to improve the supply and demand conditions for the implementation of social protection programmes.
- ☑ The module design included a clear orientation towards sustainability.

- ☑ The module made strategic use of its technical and administrative staff to ensure the smooth implementation of the activities and limit delays.
- ☑ The module focused on obtaining concrete results and changes, not only fulfilling indicators, which drove innovative responses in its interventions.
- ☑ The module could also rely on GIZ's long-standing work on SHP in Pakistan to capitalise on previous results and on trust-based relationships with key stakeholders and to continue developing further cooperation opportunities.
- ☑ The quality of the systemic approach of the module was further reinforced by the selection of proactive partners that are well integrated in the selected sectors.
- ☑ The presence of multiple donors interested in the module's thematic and geographic focus is an opportunity to build on existing results and tools and to further disseminate products to other target groups.

The following are examples of failure factors:

- ☒ There were some flaws in the design and management of the research hub, which was expected to encourage the use of research for policy development (under Output 2).
- ☒ The M&E system could be improved to better adhere to the systemic approach of the module to reflect changes in sustainability and to become a solid component of a strong module steering mechanism.
- ☒ The main external risk in Pakistan revolved around the political environment; this led to issues such as compartmentalisation of SP-SHP, an unclear governance structure between provinces and with the federal government under development, and the uncertainty created by political turmoil.

Recommendations

1. The GIZ module and future GIZ projects should consolidate their M&E system needs to improve the quality and availability of the monitoring data. In particular, they should ensure that the strong sustainability orientation of the module is reflected in the M&E system.
2. The GIZ module and future GIZ projects should continue to improve coordination with KfW.
3. The GIZ module and future GIZ projects should compile information and lessons learned from their innovative interventions.
4. The GIZ module and future GIZ projects should continue to create links between their key stakeholders to complement service offering and coordination efforts, in line with the integrated approach of GIZ and the module.
5. The GIZ module and future GIZ projects should consider creating an advanced course on SP.
6. Future GIZ projects should consider adopting the adaptable design of the module.
7. Future GIZ projects should systematise their use of the Regional West Bureau administrative unit.
8. Future GIZ projects should reinforce the coherence between social protection and social health protection for greater information, searching for complementarity, synergies, effectiveness and impact.
9. Future GIZ projects should continue to support federal and provincial governments to consolidate their social protection and social health protection frameworks.
10. Future projects should reinforce the support to evidence-based development, in particular at policy level in the field of social protection.

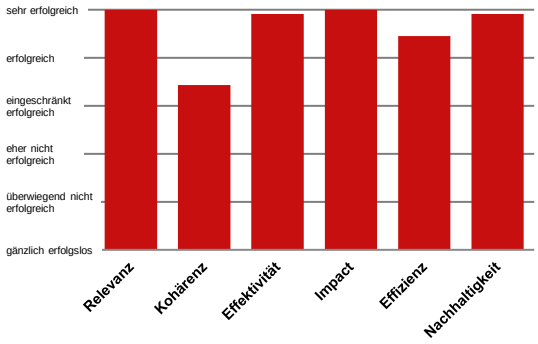
Executive Summary (German)

Titel	Unterstützung bei Sozialer Sicherung, einschließlich Absicherung im Krankheitsfall
Land/Region/global	Pakistan
Sektor und Förderbereichsschlüssel	16010: Soziale Sicherung (100%)
Projektnummer	2019.2140.2
Auftraggeber	Bundesministerium für Wirtschaftliche Zusammenarbeit und Entwicklung (BMZ), Gates Foundation (GF)
Politischer Träger/Partnerorganisationen	Ministry of Finance, Revenue and Economic Affairs
Titel EZ-Programm	Soziale Sicherung in Pakistan
Durchführungsorganisationen EZ-Programm	GFA Consulting Group, Oxford Policy Management (OPM), Nutrition International, Center for Economic and Policy Research CERP
Projektvolumen	EUR 17,370,054 davon EUR 2,370,054 kofinanziert durch die Gates Foundation (GF)
Projektlaufzeit	Oktober 2020 – September 2025
Stichprobenjahr ZPE	2022

Ziele des Projekts

Das Modul sollte dazu beitragen, die Bedingungen für eine gezielte und wirksame Umsetzung von Programmen der sozialen Sicherung zu verbessern, damit die Bevölkerung in den ausgewählten Provinzen besser vor Armut geschützt ist, die durch Gesundheitsprobleme, wirtschaftliche Not und in gewissem Maße auch durch klimabedingte Krisen verursacht wird.

Gesamtbewertung: Erfolgreich (88/100)



Rahmenbedingungen und Einordnung des Projekts

Strukturelle Armut ist nach wie vor eine Herausforderung für das Wohlbefinden in Pakistan, eine Situation, die durch die COVID-19-Pandemie, die Wirtschaftskrise, die Inflation und Naturkatastrophen noch verschärft wurde. Politische Instabilität und schwankende Regierungsführung beeinträchtigten die Kontinuität und Umsetzung von Programmen der sozialen Sicherung. Ein erheblicher Teil der Bevölkerung, der unterhalb der Armutsgrenze lebt, verfügt über keine sozialen Sicherheitsnetze und ist daher finanziellen Schocks durch Krankheit, Unfälle, Arbeitslosigkeit, Klimarisiken und extreme Wetterereignisse ausgesetzt. Diese Situation hat den Bedarf an robusten Systemen für soziale Sicherung im Land weiter erhöht. Die Fragmentierung der Programme für soziale Sicherung hat jedoch zu Ineffizienzen, Doppelungen und Lücken in der Versorgung geführt, sodass nur ein Bruchteil der Bevölkerung angemessen von diesen Initiativen profitiert. Geschlechtsspezifische Ungleichheiten auch im Bereich der sozialen Sicherung sind nach wie vor ein großes Problem, da Frauen in der Regel einem höheren Armutsrisiko ausgesetzt sind und oft die Hauptbegünstigten sind. Die Kenntnisse über Programme für soziale Sicherung und deren Anspruchsvoraussetzungen sind in der Bevölkerung, die davon profitieren sollte, oft nicht weit verbreitet, insbesondere bei Frauen, die in einigen ländlichen Gebieten weitgehend vom öffentlichen Leben ausgeschlossen sind und einen schwierigeren Zugang zu sozialer Sicherung haben als Männer. Im politischen und regulatorischen Umfeld gab es Versuche, diese Herausforderungen anzugehen, beispielsweise durch das 2019 ins Leben gerufene Ehsaas-Programm, das darauf abzielte, bestehende soziale Sicherheitsnetze zu konsolidieren und mehrdimensionale Armut zu bekämpfen (mit dem Regierungswechsel eingestellt). Trotz ehrgeiziger Ziele wurde die Umsetzung von Ehsaas und anderen Initiativen durch Koordinierungsprobleme, Unterfinanzierung sowie Fehler bei der Einbeziehung und Ausgrenzung (z. B. mutmaßlich betrügerisches Verhalten von Sachbearbeitern, die ihre Familien

angemeldet haben, oder Unterversorgung in schwer erreichbaren Gebieten) behindert. Im Jahr 2022 wurde das Land von verheerenden Überschwemmungen heimgesucht, von denen über 33 Millionen Menschen betroffen waren, Gemeinden vertrieben wurden, Infrastruktur im Wert von mehreren Milliarden Dollar zerstört wurde und die Schwachstellen der Notfallmaßnahmen und der sozialen Sicherung in Pakistans offenbart wurden.

Kurzbeschreibung des Projekts

Das **Ziel des Moduls** bestand darin, „die Bedingungen auf der Angebots- und Nachfrageseite für die Umsetzung von Programmen der sozialen Sicherung zu verbessern“. Das Modul kombinierte seine Unterstützung für die Entwicklung politischer, technischer und technologischer Kapazitäten. Durch den Aufbau von Kapazitäten zur Stärkung lokaler Systeme, die Verbesserung der Regierungsführung (Schließung politischer Lücken durch politisches Engagement, Förderung der interprovinziellen Koordinierung), die Erleichterung der Datenverfügbarkeit (u. a. durch Datenaustauschmechanismen) für eine evidenzbasierte Entscheidungsfindung und die Integration der Verbindungen zwischen verschiedenen Programmen und einer besseren Ernährung für jugendliche Mädchen und ihre Gemeinschaften. Die wichtigsten Hypothesen lauteten: 1) Hat die Unterstützung bei der Umsetzung einer Kommunikationsstrategie dazu beigetragen, die Bevölkerung besser mit den SHP-Programmen vertraut zu machen? 2) Hat die Entwicklung von Monitoring- und Evaluierungsmechanismen auf der Grundlage von KPIs und einem Dashboard die Kapazitäten des Sehat Sahulat Programs und der Social Health Protection Initiative verbessert, die Daten der Begünstigten zu analysieren, sein Programm anzupassen und die Krankenversicherung mit anderen Programmen der sozialen Sicherung zu integrieren, um evidenzbasierte Entscheidungen zu treffen? Und 3) Trug die Einrichtung von (mobilen) Registrierungszentren dazu bei, die Registrierung der Begünstigten im dynamischen NSER im Rahmen des BISP und anderer Programme der sozialen Sicherung zu verbessern, die Verbindung und den Informationsaustausch zwischen Angebot und Nachfrage in Sozialschutzprogrammen zu verbessern und letztlich die Reaktion auf die Bedürfnisse zu verbessern? Ein Vorgängermodul wurde von Juli 2016 bis September 2020 umgesetzt, als das aktuelle Modul begann.

Tabelle: Projektziel/Interventionsbereiche

Projektziel	Die angebots- und nachfrage-seitigen Voraussetzungen für die Umsetzung von Programmen der sozialen Sicherung sind verbessert.
Output 1	Ansätze zur Stärkung der Umsetzungskapazitäten der SHPI-Stakeholder in KP und SSP-Stakeholder auf der föderalen Ebene sind institutionalisiert.
Output 2	Die evidenzbasierte Entscheidungsfindung relevanter Akteure im Bereich der sozialen Sicherung ist verbessert.
Output 3	Die Kapazitäten für eine bessere Koordinierung von Programmen der sozialen Sicherung in den Partnerprovinzen sind gestärkt.
Output 4	Die Mechanismen des Benazir Income Support Programms für die Interaktion mit den Zielgruppen sind leistungsfähiger.
Output 5	In ausgewählten Tehsils hat die Umsetzung eines Handlungsrahmens für die sektorübergreifende Integration von Sozialhilfe und Ernährungssicherung für jugendliche Mädchen begonnen.

Bewertung nach DAC-Kriterien

Relevanz

Die Evaluierung kommt in Dimension 1 zu dem Ergebnis, dass das Modul mit den wichtigsten internationalen Rahmenwerken (SDGs 1, 3 und 5), deutschen Strategien und Prioritäten (z. B. der deutschen Entwicklungspolitik mit Asien bis 2023 des BMZ, der Kernbereichsstrategie Gesundheit, soziale Sicherung und Bevölkerungsdynamik des BMZ und der deutschen feministischen Entwicklungspolitik) im Einklang steht. Auf nationaler Ebene stand das Modul trotz begrenzter formalisierter staatlicher Leitlinien für soziale (Gesundheits-)Sicherung im Einklang mit den Prioritäten der wichtigsten Akteure des Sektors (Unterstützung bei der

Steuerung der sozialen (Gesundheits-)Sicherungsprogramme in Pakistan, Unterstützung wichtiger Akteure wie der Health Service Academy und von Systemen wie dem Zakat-System). In Dimension 2 kommt die Evaluierung ebenfalls zu dem Ergebnis, dass das Modul gut auf die Bedürfnisse der direkten Zielgruppen (technische und organisatorische Kapazitäten, finanzielle Unterstützung, politischer Rahmen, technologische/digitale Kapazitäten, Kommunikationsstrategien und Vernetzung) und der indirekten Zielgruppen (Stärkung des gesamten sozialen (Gesundheits-)Sicherungs-Systems zur Verbesserung ihres Schutzes) abgestimmt ist. Dimension 3 befasst sich mit den Stärken und Schwächen des Modulkonzepts, beginnend mit seinem soliden Aufbau, der auf einem starken mehrstufigen Ansatz basiert, der richtigen Ausrichtung auf Sektoren, um den Bedürfnissen der Zielgruppen gerecht zu werden, dem Einsatz relevanter und zeitnaher Aktivitäten zur Erreichung der Ziele und der Auswahl der richtigen Kooperationspartner. Es wurden einige Schwächen bei der Nachvollziehbarkeit des Projekts festgestellt (z. B. mangelnde Details im Ergebnismodell und Fehlen eines zentralisierten M&E Systems, die Konzeption des Outputs 2 und die Aufteilung der Themen soziale Sicherung und soziale Gesundheitssicherung). Schließlich zeigt Dimension 4, dass das Modul durch die Anpassung an die sich abzeichnenden Bedürfnisse der nationalen und internationalen Partner ein hohes Maß an Relevanz beibehalten hat.

Kohärenz

Die Untersuchung der internen Kohärenz des Moduls mit anderen Akteuren der deutschen Entwicklungszusammenarbeit in Dimension 1 zeigt trotz der begrenzten Zusammenarbeit mit der Finanzielle Zusammenarbeit ein gutes Potenzial auf Programmebene. Innerhalb desselben Programms nutzte das Modul Synergien mit dem Modul Adaptive Social Protection (ASP) (d. h. gemeinsame Partner, gemeinsame Veranstaltungen und Integration von Aktivitäten) effektiv, jedoch weniger mit anderen von der GIZ umgesetzten Modulen. Darüber hinaus war die Zusammenarbeit mit der KfW aufgrund unterschiedlicher Umsetzungsansätze und fehlender strukturierter Koordinierungsmechanismen (z. B. gemeinsame Planung) begrenzt, wodurch Synergiepotenziale nicht genutzt wurden. In Dimension 2 zeigt die Evaluierung Beispiele für konkrete Möglichkeiten der Komplementarität mit nationalen Partnern und internationalen Organisationen. Die Zusammenarbeit mit anderen internationalen Organisationen wie der Weltbank (WB), der Internationalen Arbeitsorganisation (ILO) und der Asiatischen Entwicklungsbank (ADB) war weitgehend erfolgreich und zeigte wirksame Koordinierungsmechanismen, die sich im Laufe der Zeit entwickelt haben (d. h. monatliche Treffen und Sektorgruppen), sowie eine Angleichung der Ziele. Das Modul hat auch den Weg für eine stärkere Koordinierung der sozialen Sicherung geebnet, insbesondere durch Initiativen wie eine nationale Konferenz mit dem Benazir Income Support Programme (BISP), die die Grundlagen für den ersten gemeinsamen Ansatz der sozialen Sicherung im Jahr 2023 legte, die Cash Transfer Coordination Group zusammen mit dem ASP-Modul Ende 2023, das pakistanische Ministerium für Armutsbekämpfung und soziale Sicherheit, die ADB und die WB mit Beiträgen von UNICEF, ILO und anderen. Darüber hinaus hat das Modul strategische Kooperationen mit Organisationen wie der Gates Foundation (GF) gefördert. Die Kofinanzierung und Zusammenarbeit mit der GF bei der Ernährungsunterstützung und Bildung für jugendliche Mädchen und ihre Haushalte die Begünstigte des BISP sind, ist ein positives Beispiel für koordinierte Bemühungen, die sich durch einen konsequenten Informationsaustausch und die effektive Nutzung von Synergien auszeichnen. Schließlich engagierte sich das Modul aktiv an koordinierten Krisenreaktionsmaßnahmen nach den schweren Überschwemmungen im Jahr 2022.

Effektivität

In Dimension 1 bestätigt die Evaluation, dass alle vier Zielindikatoren des Moduls auf dem besten Weg sind, bis zum Ende des Moduls im September 2025 vollständig erreicht zu werden: Neue Prozesse wurden erfolgreich umgesetzt (z. B. Entwicklung von Leitlinien, Aktualisierung der Krankenhauszulassungen und Standardisierung der Definitionen von Familieneinheiten); das Wissen der Begünstigten über die sozialen Krankenversicherungen hat sich verbessert (von 25 % auf 87 %) gemessen in Umfragen in KP, AJK und GB; die Provinzbehörde von KP hat ihre erste Leitlinie für soziale Sicherung entwickelt und eine Strategie entworfen, und die Registrierungen im dynamischen Nationalen Sozioökonomischen Register (NSER) haben

bis Oktober 2024 16,9 Millionen Haushalte erreicht. In Dimension 2 kommt die Evaluierung zu dem Ergebnis, dass das Modul die richtigen Strategien und Mechanismen zur Erreichung seiner Ergebnisse eingesetzt hat, da zwei Hypothesen bestätigt wurden: Die Operationalisierung von Kommunikationsstrategien trug dazu bei, das Wissen der Begünstigten über Programme der sozialen Gesundheitssicherung zu verbessern (H1); durch die Unterstützung des BISP bei der Einrichtung von 647 Tehsil-Zentren und der Einführung mobiler Registrierungswagen trug das Modul zu einer verbesserten Registrierung der Begünstigten und zum Informationsaustausch im Bereich der sozialen Sicherung bei (H3). Auch die dritte Hypothese zeigt große Fortschritte, da die Daten bestätigten, dass das Modul zu einer verbesserten evidenzbasierten Entscheidungsfindung und institutionellen Kapazitäten beitrug, jedoch mit Herausforderungen konfrontiert war (wie z. B. einer Lücke im Grad der Akzeptanz evidenzbasierter Entscheidungsfindung in Pakistan und einer hohen Personalfluktuation). In Dimension 3 deuten die Daten darauf hin, dass die Qualität der Umsetzung des Moduls im Allgemeinen hoch ist und dass starke Steuerungsmechanismen und ein kooperativer Ansatz eine strategische Ausrichtung und wirksame Maßnahmen gewährleistet haben. Die Auswahl der Partner auf mehreren Ebenen stärkte die strukturelle Stärke, Inklusivität und Zusammenarbeit. Allerdings führten schwerfällige bürokratische Prozesse der GIZ trotz gut geführter Verwaltungsabläufe zu Verzögerungen bei der Umsetzung. Schließlich zeigt Dimension 4, dass das Modul unbeabsichtigte positive Ergebnisse hatte, insbesondere eine Vereinbarung zum Datenaustausch zwischen dem dem Gesundheitsprogramm der Provinz Belutschistan und dem BISP, welche durch die der Arbeit in KP beeinflusst wurde. Außerdem wurden die Bedürfnisse im Bereich der Inklusion von Menschen mit Behinderungen berücksichtigt, indem in KP eine App entwickelt wurde, die Mitarbeiter der Sozialämter und Menschen mit Behinderungen und ihren Haushalten beim Zugang zu Sozialleistungen und -programmen unterstützt, und indem das Thema sexuelle Belästigung in die Schulungen aufgenommen wurde. Die Überwachung der unbeabsichtigten Ergebnisse war zwar fragmentiert, das Modul konnte jedoch an neue Herausforderungen und Chancen angepasst werden.

Tabelle: Erreichung der Zielindikatoren des Projekts

Indikator	%
Kernprozesse der Krankenversicherung wurden auf der Grundlage von evidenzbasierten Empfehlungen angepasst.	100
Der Anteil der Zielgruppe, die die Leistungsprozesse der Krankenversicherung kennen ist gestiegen.	100(138)
Eine Provinzregierung hat eine Strategie zur Entwicklung eines integrierten Systems der sozialen Sicherung entwickelt.	100(104)
Die Anzahl der im neuen dynamischen Nationalen Sozio-ökonomischen Register (NSER) registrierten Hause ist gestiegen.	100

Übergeordnete entwicklungspolitische Wirkungen (Impact)

Auf der Wirkungsebene zeigen die Daten in Dimension 1 Anzeichen für positive Trends auf struktureller Ebene, darunter ein besserer Zugang zu Programmen der sozialen (Gesundheits-)Sicherung für die Bevölkerung, die Einrichtung dynamischer Registrierungsinstrumente und eine verbesserte Governance der Programme der sozialen (Gesundheits-)Sicherung in Pakistan, auch wenn die Auswirkungen auf das gesamte Land erst mit der Zeit sichtbar werden. Darüber hinaus zeigt Dimension 2, dass das Modul plausibel zu diesen Veränderungen beigetragen hat, da beide Hypothesen zeigen, dass das Modul wirksame Mechanismen zur Entwicklung eines anpassungsfähigen Systems der sozialen Sicherung eingesetzt hat, um die Abdeckung und die Leistungen für gefährdete Bevölkerungsgruppen zu verbessern (H4) und die Verbindungen zwischen den Provinzen und zur Bundesregierung zu verbessern (H5). Allerdings liegen nur wenige Informationen über die Verbesserung des Schutzes gefährdeter Haushalte auf der Ebene der Begünstigten vor, abgesehen von einem besseren Zugang zu Programmen der sozialen (Gesundheits-)Sicherung, da die Veränderungen noch relativ jung und noch nicht sichtbar und messbar sind. Darüber hinaus haben die Auswirkungen der Covid-Pandemie, die Wirtschaftskrise, die Sicherheitsprobleme in einigen Teilen des Landes und die durch den Klimawandel verursachten Schocks die jüngsten Erfolge Pakistans bei der nachhaltigen Armutsbekämpfung erschwert. Schließlich stellt die Evaluierung in Dimension 3 keine weiteren negativen unbeabsichtigten Auswirkungen fest, sondern lediglich eine positive: Die Ergebnisse des Moduls wurden genutzt, um den Zugang zu einer erweiterten Gesundheitsversorgung (z. B. Neugeborenen-Intensivstationen (NICUs), Kardiotokographie-Geräte (CTG) und Katheterlabore) weiter zu verbessern

Effizienz

Beginnend mit der **Produktionseffizienz** zeigt die Evaluation, dass die Modulressourcen sinnvoll auf die fünf Outputs verteilt wurden und die Instrumenten- und Personalkosten den beobachteten Ergebnissen angemessen sind. Die Zuordnung der Kosten zu den Indikatoren ist ebenfalls plausibel, da die meisten Output-Indikatoren (über-)erfüllt wurden. Es wurden einige Herausforderungen identifiziert (z. B. Herausforderungen aufgrund politischer Unruhen, die zu Verzögerungen bei der Durchführung der Kommunikationskampagnen führten, oder die Herausforderung, die richtigen nationalen Teammitglieder in das Team zu integrieren), jedoch lässt die zuletzt im Jahr 2024 vorgelegte Änderungsvorschlag die Evaluierung davon ausgehen, dass alle Indikatoren bis zum Ende des Moduls erreicht werden können. Insgesamt lässt sich feststellen, dass die Verteilung der Kosten auf die Outputs im Einklang mit den Schwerpunkten des Projekts steht, d. h. der Schwerpunkt liegt auf der Erhöhung der Registrierungskapazitäten (Output 4), der Verbesserung der Koordinierung (Output 3) und der Verbesserung der Vertrautheit der Begünstigten mit Programmen der sozialen Gesundheitssicherung (Output 1). Die Effizienz des Moduls und seines Ansatzes zeigte sich angesichts der Veränderungen im Kontext (z. B. Überschwemmungen im Jahr 2022), da es dem Team gelang, sich erfolgreich an die sich ändernden Bedingungen anzupassen und seine Ressourcen auf die Notfallmaßnahmen des Partners BISP auszurichten. Die **Allokationseffizienz** wird ebenfalls als erfolgreich bewertet. Die Evaluierung kommt zu dem Ergebnis, dass es dem Modul gelungen ist, seine Aktivitäten wirksam in Ergebnisse umzusetzen. Dies zeigt sich in den gemessenen erwarteten Gesamtergebnissen. Zwar besteht möglicherweise ein gewisses Potenzial zur Kostenminimierung, um die anhand der Modulzielindikatoren gemessenen Ergebnisse zu erreichen, da diese trotz der verbleibenden Zeit und des verbleibenden Budgets größtenteils erreicht werden, doch war das Modul generell darauf ausgerichtet, die Ergebnisse sogar über die Modulergebnisse hinaus zu maximieren. Die Effizienz der Mittelzuweisung ergibt sich aus der Identifizierung von Möglichkeiten für skalierbare Wirkungen und neue Kooperationen – wie beispielsweise die Zusammenarbeit mit dem BISP-Programm, dem größten Anbieter von sozialen Geldtransfers in Pakistan –, allerdings wurden Möglichkeiten für Kooperationen mit deutschen DC-Projekten nur in begrenztem Maße identifiziert.

Nachhaltigkeit

Die Evaluierung kommt in Dimension 1 zu dem Schluss, dass die direkten Zielgruppen über starke Kapazitäten verfügen, die sie nutzen können, insbesondere technische, institutionelle und technologische Kapazitäten und Instrumente. Die institutionellen Kapazitäten sind zwar derzeit stark, könnten jedoch an eine einzelne Person gebunden sein, was sich im Laufe der Zeit auf ihre Nutzung auf der Ebene der gesamten Institutionen auswirken würde. Die indirekten Zielgruppen zeigen vielversprechende Beispiele für Kapazitäten, die im Laufe der Zeit genutzt werden könnten, die jedoch noch nicht vollständig bestätigt werden können. Dimension 2 zeigt, dass das Modul eine solide Grundlage für eine nachhaltige soziale Sicherung in Pakistan geschaffen hat, und zwar durch Strategien zur Unterstützung der institutionellen (z. B. integrierter Nachhaltigkeitsansatz, Einbeziehung von Interessengruppen, bedarfsorientierter Ansatz), technologischen (z. B. Entwicklung von Instrumenten und Systemen zur Stärkung der Akteure auf mehreren für die Sektoren relevanten Ebenen) und finanziellen Nachhaltigkeit (z. B. Anpassung der Aktivitäten an die Kapazitäten der Partner und Pilotansatz). Allerdings mangelt es an einer systematischen Überwachung und Dokumentation der Nachhaltigkeitsergebnisse. Wie in Dimension 3 erläutert, stellen Risiken wie die Politisierung der Sektoren der sozialen (Gesundheits-)Sicherung, Personalfluktuation, Finanzierungsengpässe und Herausforderungen im Bereich der Datensicherheit zwar erhebliche Herausforderungen dar, doch hat das Modul erfolgreiche Strategien zur Bewältigung einiger dieser Herausforderungen (z. B. politische Arbeit, Integration der Nachhaltigkeit in alle Aktivitäten seit deren Beginn, Einbeziehung von Stakeholdern u. a.) eingeführt, und die laufenden institutionellen Reformen bieten vielversprechende Wege, um die Dauerhaftigkeit der Ergebnisse des Moduls sicherzustellen.

Gesamtbewertung

Unter Berücksichtigung der oben beschriebenen Evidenz wird die „Unterstützung der sozialen Sicherung einschließlich im Krankheitsfall“ in Pakistan (PN 2019.2140.2) mit 88 von 100 Punkten als „erfolgreich“ bewertet. Die Stärken und Schwächen des Moduls werden im Folgenden dargestellt.

Tabelle: Bewertung der OECD DAC Evaluierungskriterien

Kriterium	Punkt-zahl (von 100)	Bewertung 1 (sehr erfolgreich) bis 6 (gänzlich erfolglos)	Kriterium	Punkt-zahl (von 100)	Bewertung 1 (sehr erfolgreich) bis 6 (gänzlich erfolglos)
Relevanz	92	Stufe 1: sehr erfolgreich	Impact	93	Stufe 1: sehr erfolgreich
Kohärenz	73	Stufe 3: eingeschränkt erfolgreich	Effizienz	86	Stufe 2: erfolgreich
Effektivität	91	Stufe 1: sehr erfolgreich	Nachhaltigkeit	91	Stufe 1: sehr erfolgreich
Gesamtbewertung				**88**	Stufe 2: erfolgreich

Schlussfolgerungen sowie Erfolgs- und Misserfolgsfaktoren

Erfolgsfaktoren sind beispielsweise die folgenden:

- ☑ Erfolgsfaktor 1: Der systemische Ansatz des Moduls in Bezug auf soziale (Gesundheits-)Sicherung bietet umfassende Unterstützung, indem er die Förderung politischer, technischer und technologischer Kapazitäten kombiniert, um die Abdeckung und den Schutz der ärmsten Haushalte zu verbessern.
- ☑ Erfolgsfaktor 2: Im Einklang mit dem systemischen Ansatz arbeitete das Modul auch mit Systemakteuren zusammen, um durch seinen mehrstufigen Ansatz die Reichweite und Wirkung zu erhöhen.
- ☑ Erfolgsfaktor 3: Das Modul stützte sich auf ein anpassungsfähiges Design für die Entwicklung des Sektors der sozialen (Gesundheits-)Sicherung, um die wichtigsten Hindernisse für die Verbesserung der Angebots- und Nachfragebedingungen für die Umsetzung von Programmen zu beseitigen.
- ☑ Erfolgsfaktor 4: Das Moduldesign war klar auf Nachhaltigkeit ausgerichtet.
- ☑ Erfolgsfaktor 5: Das Modul setzte sein technisches und administratives Personal strategisch ein, um eine reibungslose Umsetzung der Aktivitäten zu gewährleisten und Verzögerungen zu begrenzen.
- ☑ Erfolgsfaktor 6: Das Modul war auf konkrete Ergebnisse und Veränderungen ausgerichtet und nicht nur auf die Erfüllung von Indikatoren, was zu innovativen Maßnahmen führte.
- ☑ Erfolgsfaktor 7: Das Modul konnte auch auf die langjährige Arbeit der GIZ im Bereich der sozialen Gesundheitssicherung in Pakistan zurückgreifen, um frühere Ergebnisse und vertrauensvolle Beziehungen zu wichtigen Akteuren zu nutzen und weitere Kooperationen auszubauen.
- ☑ Erfolgsfaktor 8: Die Qualität des systemischen Ansatzes des Moduls wurde durch die Auswahl proaktiver Partner, die gut in die ausgewählten Sektoren integriert sind, weiter gestärkt.
- ☑ Erfolgsfaktor 9: Die Präsenz mehrerer Geber, die an den thematischen und geografischen Schwerpunkten des Moduls interessiert sind, bietet die Möglichkeit, auf bestehenden Ergebnissen und Instrumenten aufzubauen und die Produkte an andere Zielgruppen weiter zu verbreiten.

Misserfolgsfaktoren sind beispielsweise die folgenden:

- ☒ Misserfolgsfaktor 1: Es gab einige Mängel bei der Konzeption und Verwaltung des Research Hubs, welcher die Nutzung von Forschungsergebnissen für die Politikgestaltung fördern sollte (im Rahmen von Output 2).

- ☒ Misserfolgsfaktor 2: Das M&E-System könnte verbessert werden, um dem systemischen Ansatz des Moduls besser gerecht zu werden, Veränderungen im Bereich der Nachhaltigkeit zu reflektieren und zu einem soliden Bestandteil eines starken Modulsteuerungsmechanismus zu werden.
- ☒ Misserfolgsfaktor 3: Das größte externe Risiko in Pakistan betraf das politische Umfeld (Kompartimentalisierung von sozialer (Gesundheits-)Sicherung, noch in der Entwicklung befindliche Governance-Struktur zwischen den Provinzen und der nationalen Regierung, politische Unruhen).

Empfehlungen

1. Das GIZ-Modul und künftige GIZ-Projekte sollten ihr Monitoring- und Evaluierungssystem (M&E) konsolidieren, um die Qualität und Verfügbarkeit der Monitoringdaten zu verbessern. Insbesondere sollte sichergestellt werden, dass die starke Nachhaltigkeitsorientierung des Moduls im M&E-System zum Ausdruck kommt.

2. Das GIZ-Modul und künftige GIZ-Projekte sollten die Koordinierung mit der KfW weiter verbessern.

3. Das GIZ-Modul und künftige GIZ-Projekte sollten Informationen und Erfahrungen aus ihren innovativen Maßnahmen zusammenstellen.

4. Das GIZ-Modul und künftige GIZ-Projekte sollten weiterhin Verbindungen zwischen ihren wichtigsten Stakeholdern herstellen, um das Dienstleistungsangebot und die Koordinierungsbemühungen im Einklang mit dem integrierten Ansatz der GIZ und des Moduls zu ergänzen.

5. Das GIZ-Modul und künftige GIZ-Projekte sollten die Einrichtung eines Fortgeschrittenenkurses zum Thema soziale Sicherung in Betracht ziehen.

6. Zukünftige GIZ-Projekte sollten die Übernahme des anpassungsfähigen Designs des Moduls in Betracht ziehen.

7. Zukünftige GIZ-Projekte und andere GIZ-Projekte sollten die Nutzung der Verwaltungsstelle des Regionalbüros West systematisch prüfen.

8. Zukünftige GIZ-Projekte sollten die Kohärenz zwischen sozialer Sicherung und sozialer Gesundheitssicherung stärken, um den Informationsfluss zu verbessern und Komplementaritäten, Synergien, Wirksamkeit und Wirkung zu erzielen.

9. Zukünftige GIZ-Projekte sollten die Bundes- und Provinzregierungen weiterhin dabei unterstützen, ihre Rahmenwerke für Sozialschutz und Sozialgesundheitsschutz zu konsolidieren.

10. Zukünftige Projekte sollten die Unterstützung für evidenzbasierte Entwicklung verstärken, insbesondere auf politischer Ebene im Bereich der sozialen Sicherung.

The project at a glance

Pakistan: Support to social protection including social health protection

Project number	2019.2140.2
Creditor reporting system code(s)	16010: Social Protection (100%)
Project objective	The population in the target regions is better protected against poverty caused by health issues and climate-induced crises.
Project term	1 October 2020 – 30 September 2025
Project value	EUR 17,370,054, of which EUR 2,370,054 is co-financed by the Gates Foundation (GF)
Commissioning party/co-financier	German Federal Ministry for Economic Cooperation and Development (BMZ)
Lead executing agency	Ministry of Finance, Revenue and Economic Affairs
Implementing partner organisations (in the partner country)	GFA Consulting Group, Oxford Policy Management, Nutrition International, Center for Economic and Policy Research
Target group(s)	Direct target groups: Ministry of Poverty Alleviation and Social Safety (MoPASS), Benazir Income Support Programme (BISP), Ministry of National Health Services, Regulations and Coordination, Department of Health, Government of Khyber Pakhtunkhwa, Department of Planning & Development, (Social Welfare, Special Education & Women Empowerment Department, Zakat and Ushr Department), Government of Khyber Pakhtunkhwa, Punjab Social Protection Authority (PSPA). Other direct target groups are: Health Services Academy (HSA), National Rural Support Programme (NRSP), Social Policy and Development Centre, Institute for Management and Sciences (IMSciences), Sustainable Development Policy Institute (SDPI), Pakistan Institute of Development Economics (PIDE), and Aga Khan University. Indirect target groups: BISP staff (compliance monitors and ushers and supervisors) and the general population with a focus on the poorest-of-the-poor households (those eligible for BISP) especially groups not adequately covered by social protection systems (women and people with disabilities), estimated at 10 million households, 65 million individuals.
Development cooperation (DC) programme	Social Protection in Pakistan
Implementing organisations of the DC programme	Deutsche Gesellschaft für Internationale Zusammenarbeit (GIZ) GmbH, Kreditanstalt für Wiederaufbau (KfW)
Organisation responsible for implementing and coordinating the DC programme	GIZ
Reporting year of CPE	2024
Sample year of CPE	2022

1 Evaluation objectives and questions

This chapter aims to describe the purpose of the evaluation, the standard evaluation criteria, and additional stakeholders' knowledge interests and evaluation questions.

1.1 Evaluation objectives

Central project evaluations (CPE) of projects commissioned by the German Federal Ministry for Economic Cooperation and Development (BMZ) fulfil three basic functions: they support evidence-based decisions, promote transparency and accountability, and foster organisational learning within the scope of contributing to effective knowledge management. The Deutsche Gesellschaft für Internationale Zusammenarbeit (GIZ) GmbH structures the planning, implementation and use of evaluations to optimise the contribution made by the evaluation process and the evaluation findings to these basic functions (GIZ, 2018a). Projects evaluated as part of the CPE process were selected in a two-stage procedure. In a first step, projects were selected by means of a regionally stratified random sample. In a second step, a criteria-based selection was made, whereby the sample was supplemented by evaluations selected according to specific knowledge interests. The technical cooperation (TC) programme module called **Support to social protection including social health protection, Pakistan (PN 2019.2140.2)** (henceforth: the module or SP-SHP) was selected by random sampling. The module is expected to run until September 2025 and this report can therefore be understood as being part of a mid-term evaluation.

Regarding the **feasibility of the evaluation**, several internal and external factors were identified:

- The evaluation team partially mitigated the risk of lost information as a result of staff turnover in the module team by organising a series of interviews with the 'Auftragsverantwortlicher' (AV, person responsible for the commission) before he left the module and GIZ in August 2024. In addition, despite the delayed start of the CPE, the evaluation team and the evaluation unit agreed to maintain the original timeframe for the submission of the inception report to ensure the participation of the AV.
- Data availability on the national trends at impact level are limited, in part due to the relatively new development of the social (health) protection sector in Pakistan. The evaluation team refers to secondary data to the extent possible and uses evaluation data to provide indications.
- Lastly, considering the module will finish in September 2025, some activities are still ongoing. The results of some activities were therefore not expected to be observable at the time of the evaluation, in particular with respect to Output 5. For these ongoing activities, the assessment of effectiveness, impact and sustainability are therefore based on a plausibility analysis.

1.2 Evaluation questions

The module was assessed based on standardised evaluation criteria and questions to ensure comparability by GIZ. This is based on the Organisation for Economic Co-operation and Development (OECD) Development Assistance Committee (DAC) evaluation criteria (updated 2020) for international cooperation and the evaluation criteria for German bilateral cooperation (BMZ, 2021): **relevance**, **coherence**, **efficiency**, **effectiveness**, **impact** and **sustainability**.

Specific assessment dimensions and evaluation questions have been derived from this given framework by BMZ. These assessment dimensions and analytical questions are the basis for all CPEs in GIZ and can be found in the **evaluation matrix** (see Annex). In addition, contributions to the 2030 Agenda for Sustainable Development and its principles are also taken into account as well as cross-cutting issues such as gender, the

environment, conflict sensitivity and human rights. Moreover, during the inception mission, stakeholders expressed specific knowledge interests for this evaluation (see **Fehler! Verweisquelle konnte nicht gefunden werden.**). While these fall within the scope of the evaluation and are covered by the standard evaluation dimensions and respective questions, these specific aspects were pursued during the data collection and evaluation mission. Additionally, considering the volatile context of the country, specific questions regarding context and conflict sensitivity were applied (see Annex Evaluation Matrix). Lastly, considerations of the predecessor project were accounted for. At the time of the end of the evaluation, a follow-on module proposal was expected to be submitted to BMZ.

Table 1: Knowledge interests by main evaluation stakeholder groups

Evaluation stakeholder group	Knowledge interests in evaluation/additional evaluation questions	Relevant section in this report
GIZ module team, other GIZ members	• How is the module integrated into the broader donor and development context (both German (KfW) but also other donors, most notably WB)? • Is the module effectively leveraging financial cooperation measures? • How does social protection address multidimensional poverty? • How does social protection, especially social assistance, support graduation out of poverty? • How useful is the results matrix and how can it be effectively leveraged? • What are the relevant indicators to consider? • What is the effectiveness of the training measures?	4.3 Coherence 4.3 Coherence 4.5 Impact 4.5 Impact 4.2 Relevance and 4.4 Effectiveness 4.2 Relevance and 4.4 Effectiveness 4.4 Effectiveness
BMZ	• Evaluation of cooperation with Financial Cooperation and other donors • Reform readiness of Pakistani partners in the sector • Take-aways for further engagement in the sector	4.3 Coherence, 4.5 Impact and 5 Conclusions

2 Object of the evaluation

This chapter aims to define the evaluation object, including the theory of change, and results hypotheses.

2.1 Definition of the evaluation object

The SP-SHP module originally encompassed a period from 1 October 2020 to 30 September 2023 and is geographically focused on the provinces of Khyber Pakhtunkhwa (KP) and Punjab. Two change offers were submitted in 2022 and 2024, which ultimately redefined the module duration from 1 October 2020 to 30 September 2025 and increased the budget from EUR 7,000,000 to EUR 17,370,054. This budget increase was to account for an adjustment of the module to developments in the implementation of the Benazir Income Support Programme (BISP) of the Pakistani government, and a co-financing agreement signed with the Gates Foundation (GF) to improve social protection services for the nutrition of adolescent girls. The co-financing from GF (EUR 2,370,054) started in December 2023 and was agreed to be approximately matched by BMZ. Therefore, previously committed funds under the current module (approximately EUR 1,700,000) were reallocated towards this component, while BMZ is committed to providing the remaining EUR 1,000,000 contribution in a sequel phase of SP-SHP for a pilot project called Leveraging Pakistan's Social Protection Programmes for Adolescent Girls' Nutrition (referred to as SOPRAN), which is planned to start in April 2026.

This led to the amendment of a target indicator and an output as well as the inclusion of a new output in the scope of the order. Co-financing from GF, implemented from 24 November 2023 onwards, is part of the evaluation object. The module built on the experience of the previous project of the same name (PN 2015.2186.3; for more details on the predecessor module, please refer to Section 4.1). At the time of the end of the evaluation, a proposal for a follow-on project, planned to start in April 2026 after the current module ends and due to run for three years until the end of 2028, has been submitted to BMZ, through the standardised integrated programme allocation 2025 procedure.

Structural poverty continues to be a challenge for socio-economic growth and welfare in Pakistan, a situation only exacerbated by the COVID-19 pandemic, economic crisis, inflation, and the effects of catastrophic natural disasters. Politically, the country has faced instability, with fluctuating governance and policy directions impacting the consistency and implementation of social protection programmes. A significant proportion of the population living below the poverty line lack social safety nets, leaving them exposed to financial shocks from illness, accidents, unemployment, climate risks and extreme weather events. In particular, healthcare costs pose a major burden, pushing many families deeper into poverty. Furthermore, inflation, unemployment, the devaluation of the Pakistani Rupee, and the increased risk of natural disasters such as flooding due to climate change have further increased the need for robust social protection systems. The fragmentation of social protection programmes has led to inefficiencies, duplications and gaps in coverage, with only a fraction of the population benefiting adequately from these initiatives. Gender disparities with regard to social protection remain a significant concern. Due to patriarchal systems, men often decide which health and social services women can utilise. Thus, women, especially in some rural areas, are largely excluded from public life and have greater difficulty accessing social protection programmes. These barriers to access are particularly problematic as women typically have a higher risk of poverty but simultaneously are often the main and targeted beneficiaries of programmes (e.g. BISP). Knowledge of social protection programmes and their eligibility criteria is often not widespread among the population. The policy and regulatory environment have seen attempts to address these challenges, notably through the Ehsaas programme, which aimed to consolidate existing social safety nets and address multidimensional poverty, launched in 2019 (although this was discontinued with the change in government), and through other strategies that have yet to be endorsed. Despite its ambitious goals, implementation of Ehsaas and other continuing initiatives like BISP and the Sehat Sahulat Programme (SSP) has been hampered by coordination challenges, underfunding, and inclusion and exclusion errors (e.g. likely fraudulent behaviour of service officers signing up their families or limited coverage in hard-to-reach areas). In 2022, the country experienced devastating floods which affected over 33 million people, displaced communities, destroyed infrastructure worth billions of dollars, and exposed the vulnerabilities in Pakistan's emergency response and social protection systems, underscoring the urgent need for more resilient and inclusive policies. The **module aims to respond to these challenges**, contributing to improving the conditions for the targeted and effective implementation of **social protection programmes** so that the population in the selected regions is better protected against poverty caused by health issues and, to a certain extent, to protect them in the face of climate-induced crises (GIZ, 2020a). The project was implemented in a fragile context. There is still a nationwide risk of terrorist attacks in Pakistan, particularly in KP and Balochistan, that can lead to measures being interrupted or cancelled or access to individual regions being temporarily restricted (GIZ, 2020a). At the end of 2022, the Islamist Pakistani Taliban unilaterally declared an end to a ceasefire agreed with the government and called on their fighters to carry out attacks across the country. Since then, there have been increasing numbers of terrorist and suicide attacks, leaving numerous people dead and injured, particularly in parts of the provinces of KP and Balochistan. Other Islamist terrorist groups, such as the Islamic State of Pakistan and Baloch nationalists, are also increasingly carrying out attacks in these regions (Auswärtiges Amt, 2025).

Overall module structure

The **module's objective** is to improve the supply- and demand-side conditions for the implementation of social protection programmes. The **political partner** of the module is the Ministry of Finance, Revenue and Economic

Affairs. **Implementing partners** include the GFA Consulting Group, Oxford Policy Management, Nutrition International and the Center for Economic and Policy Research; these actors are involved in activities ranging from organising capacity-building events to the implementation of cross-sectoral activities aiming at improving food security as part of social protection programmes.

Direct **target groups** the module is working with are key actors implementing social protection, including social health protection, in Pakistan especially in KP and Punjab (and most recently Balochistan), as well as the federal level, which in addition to the capital Islamabad covers the administrative territories of Azad Jammu and Kashmir (AJK) and Gilgit-Baltistan (GB). The governmental actors include the Punjab Social Protection Agency (PSPA); within the Planning and Development Department Khyber Pakhtunkhwa, the Public Policy – Social Protection Reforms Unit (Social Welfare, Special Education and Women Empowerment Department, Zakat and Ushr Department); Punjab Health Initiative Management Company (PHIMC); Ministry for Poverty Alleviation and Social Safety (MoPASS); and the Ministry of Health Services, on the provincial and federal level. Most specifically, the module works with the federal SSP (covering Islamabad, Punjab, AJK and GB) and the Social Health Protection Initiative (SHPI) in KP, as well as the BISP, as Pakistan's largest social protection/assistance programme. The National Database and Registration Authority (NADRA), through its involvement in the social protection programmes via the dynamic National Socio-Economic Registry (NSER) for BISP, is also part of the module's direct target groups. Other direct target groups include the Health Services Academy (HSA), National Rural Support Programme (NRSP), Social Policy and Development Centre (under the research hub), Institute for Management and Sciences (IMSciences) (among other things, under the research hub), Sustainable Development Policy Institute (SDPI) (under the research hub), and the Pakistan Institute of Development Economics (PIDE).

The indirect target groups and final beneficiaries at the impact level are BISP staff (compliance monitors and ushers and supervisors) and the general population with a focus on the poorest-of-the-poor households (those eligible for BISP) especially groups not adequately covered by social protection systems, estimated at 10 million households and 65 million individuals, including women, adolescent girls, and people with disabilities.

The module adopts a multilevel approach for its interventions, including for its capacity-building measures. As such, it aims to strengthen capacities to implement social protection programmes by: (a) strengthening the capacities of professionals who are responsible at the national, federal, provincial and district level from public authorities (macro) and SP-SHP professionals (meso level); (b) strengthening organisations on core and support processes of selected social protection programmes, including for the improved communication and registration of beneficiaries of SP programmes at district/tehsil level (micro level); (c) reinforcing coordination mechanisms and data exchange; and (d) supporting the institutionalisation of health insurance and cross-level dialogue processes.

The following paragraphs describe the module's results on the output, outcome and impact level as well as how the different results are causally linked. As part of the inception phase, the evaluators and the module team reviewed the module's **results model**. An updated results model (updated in 2024) already existed. The existing results model was a simple representation of the module logic. However, considering the theory-based background for this CPE, it was necessary for the evaluation team to further detail the expected results chain, starting from activities for each output, outcome and impact. Therefore, a more detailed results model was developed by the evaluation team based on this existing model, preliminary interviews and analysis of module documents. It was reviewed with the module team as part of the Results Model Workshop, and relevant contribution hypotheses were selected for investigation by the evaluation team. The updated results model is shown in Figure 1 and shows the connection of (I) impacts all the way down to (A) activities. For a definition and demarcation between outputs, outcomes and impacts, please refer to Table 2.

Table 2: Results level terminology

Terminology	Definitions
Results	GIZ understands results as the intended or unintended, positive or negative changes in a situation or behaviour as the direct or indirect consequence of an intervention. Results are mapped in a systemic GIZ results model and implicitly include impacts, outcomes and outputs. The OECD DAC and BMZ also define results as the overall term for impacts, outcomes and outputs. The BMZ definition is in line with OECD DAC.
Impacts	Impacts are the higher-level development results (which is in line with OECD DAC).
Outcomes	Outcomes are the changes that occur as a consequence of the use of an intervention's outputs. They correspond more or less to the achievement of the project objective and include both the use of the outputs delivered by the project by the intermediaries and target groups, and the direct benefit.
Outputs	Outputs describe the changes and conditions achieved by the project's activities, such as products, goods, services, acquired technical skills and knowledge, institutions and regulations.

Output and outcome level. The module encompasses the following outputs that are expected to contribute to the MO described above:

- Output 1: The SHPI in KP, SSP in Punjab and in the federally administered territories are strengthened.
- Output 2: Greater evidence-based decision-making is made.
- Output 3: Social protection capacities for coordination in the provinces are reinforced.
- Output 4: The mechanisms of BISP for interacting with the target groups are more efficient.
- Output 5: The implementation of a framework for the cross-sectoral integration of social assistance and nutritional support for adolescent girls has begun in selected parts of the country.

Output 1 focused on institutionalising approaches in social health protection to strengthen the implementation capacities of the federal SSP stakeholders including Punjab and federally administered regions and the SHPI stakeholders in KP (in the following referred to as SHPI programmes). The main **activities** implemented by the GIZ module team started with the development of baseline studies for SHPI KP and federal SSP that were used to underline the needs for development in terms of communication and capacity development (R1). On this basis, two main activities were expected to be conducted: on the one hand, a communication strategy to raise awareness on performance assessment was developed and piloted (R2). Following the pilot, the communication strategy was intended to be operational (R3). On the other hand, the module also expected to develop training measures on selected key skills for practitioners in social health insurance (R4). Once developed, it was expected that the curricula of national educational institutions would integrate the Social Health insurance practitioner training programme (R5). By integrating the programme, this was expected to improve the competences of SSP stakeholders (R6). This dual approach was also replicated by the federal programme in the province of Balochistan in an effort to further contribute to reinforcing the federal government of Pakistan. Together, it was expected that the increased communication and capacities would contribute to strengthening the SHP programmes in KP, Punjab and federally administered territories (Output 1). In turn, through increased communication on the SHP programmes to its target groups and the greater capacity of the service provider of SHP programmes, the module expects that a greater proportion are familiar with the benefits and processes of the health insurance schemes and can therefore make better use of the insurance (R16). Therefore, this should contribute to improving both the supply- and demand-side conditions for the implementation of social protection programmes (MO). The **first contribution hypothesis** selected for the investigation for this evaluation was that through the operationalisation of a communication strategy for awareness raising (R2, R3), the SHP programmes have greater capacities to increase the reach of the health insurance, strengthening the supply side (Output 1). Through the implementation of the communication strategies, a greater proportion of the population should become familiar with the SHP programmes such that the demand side is strengthened (R16). Thereby, the module aimed to contribute to improving the supply- and demand-side conditions for the implementation of the social protection programmes (MO). The **key underlying assumptions** were that SSP and SHPI provided sufficient financial and human resources for the federal and provincial implementation of the communication strategy. In addition, despite the Pakistani government's focus

on increasing the reach of health insurance, issues of service quality and effectiveness of SSP and SHPI remain on the political agenda. The staffing situation of the institutions working in the field of social protection (i.e. the availability of adequately qualified staff) enables the participation of specialists in training programmes. **Risks** for having achieved the output are that despite the Pakistani government's focus on increasing the reach of health insurance, issues such as the financial sustainability of the programme might challenge its implementation and efforts for its expansion. In addition, the institutional stakeholders lack the confidence to act and sensitise the target groups to their entitlements. Lastly, due to political instability, different provinces have different mechanisms for implementing SHP schemes. Some services are limited to public sector hospitals with the intention of limiting the benefit package.

Output 2 focused on improving the evidence-based decision-making of relevant actors in the field of social protection in the provinces of KP, Punjab (but also Balochistan) and at the federal level**.** The **main activities** implemented by the GIZ module team were based on pre-existing results from the previous module with the same name, SP-SHP, in the fields of capacity-building, information systems and operational research. Based on partners' demands, the module therefore focuses on three areas of intervention. First, regarding capacity-building, the module expected to pilot the integration of the SSP Central Management and Information System with other social protection programmes, such as with the BISP (R9). The aim was to achieve this by reinforcing the Monitoring and Evaluation (M&E) capacities of the SSP (at regional and federal level) to increase their evidence-based decision-making skills (R8). The capacity-building measure on M&E would be based on the establishment of a concept for Key Performance Indicators (KPIs) and dashboard, adaptable to the needs of the SSP (R7). Second, the module also expected that it would support the introduction of Standard Operating Procedures (SOPs) for an adaptable system for complaints management (R10, R11) that would allow the SSP to further improve the evidence available to SSP to take decisions on their programmes (Output 2). Thirdly, the module should also contribute to improving the evidence-based decision-making capacities of SP and SHP actors (Output 2) by supporting the introduction of a research hub (R12). It is expected that the research hub would produce and channel operational research that is analysed and utilised by relevant policy-makers (R13). In turn, the SHPI programmes, SP actors at federal and national level and the research institutions should disseminate the results of the research and provide evidence for effective decision-making of policy-makers (R14). The underlying hypothesis is that through the improvement of M&E systems and skills, together with operational guidance for a complaints mechanism and a research hub to develop operational research, evidence is generated and made available to decision-makers, thereby improving the capacities of SHP and SP actors for evidence-based decision-making (Output 2). This should further contribute to the use of empirical knowledge for dialogue on the strategic development of systems processes to increase the proportion of people that have knowledge of the benefits of SSP and the SHPI health insurance system as well as SP programmes (R16). This should also improve the processes and package of benefits available to the population (R17). In turn, the greater knowledge and capacity to adapt the health insurance scheme and SP programmes should support improvement of the conditions on the supply and demand side for the implementation of the social protection programmes (MO). The **second contribution hypothesis** that was selected for further investigation during the evaluation is that through the development of an adaptable concept for M&E, based on KPIs and a dashboard (R7), together with the development of capacities on the system (R8), the SSP actors have better capacities to analyse the data of the SSP beneficiaries, adapt their programme and integrate their national health insurance with other SP programmes (R9). This should contribute to improving evidence available and the coverage of needs at a regional and federal level, thus allowing for evidence-based decision-making (Output 2). The evidence generated should be used for strategic dialogue and planning (R27). This therefore contributes to the improvement of the supply- and demand-side conditions for the implementation of social protection programmes (MO). The **key underlying assumptions** were that decision-making processes within the SSP, e.g. for the adoption of the communication strategy and the M&E system, are sufficiently efficient to enable the relevant outputs to be utilised during the module term. In addition, it was expected that the Ministry of Health would create guidelines and SOPs for the development of an M&E system, supplemented by proposals for the qualitative improvement and system-wide harmonisation of complaints and feedback mechanisms. Lastly, the module must adopt a flexible approach to answer

changing needs. Available technical capacities for the implementation and maintenance of the M&E system and dashboard are further critical assumptions. **Risks** for achieving the output are that heavy bureaucracy and staff turnover can delay implementation and the results.

Output 3 focused on strengthening the capacities for better coordination of social protection programmes in the regions of KP and Punjab and at the federal level. The **main activities** implemented by the GIZ module team were the development of training courses, the establishment of coordination mechanisms and the implementation of process analyses to develop software for beneficiary identification. First, the module oriented one of its interventions at the policy level, by supporting the development of national frameworks that provide a clear vision and definition of social protection programmes. To this end, the module was expected to support two coordination mechanisms: 1) within and across provinces, which includes the participation of heads of departments, and 2) at the national level (R18). Once operational, these coordination mechanisms would enable the development of two policy frameworks: 1) an integrated and inclusive social protection strategy and operational plan at the provincial level (R19). This provincial framework established clear responsibilities and actions for a five-year period. 2) The strengthening of a national framework for social protection and poverty alleviation (R20). These frameworks would seek to bring social protection programmes under a common umbrella, which would contribute to improving the coordination capacities of social protection programmes (Output 3). The second area of intervention focused on capacity-building. It was expected that the design and implementation of a training course on social protection (on design, implementation and evaluation of social protection programmes) (R21) would improve the capacities of social protection professionals (R22). Once the first training event had taken place, it was expected that the module would support integration of the course into a national training institution to roll it out independently (R23) to facilitate the dissemination of the course at all levels of the administration and the sustainability of the activities. This would ultimately contribute to establishing a common basis for the implementation and evaluation of social protection programmes and to enhance the capacities of social protection actors for coordination (Output 3). The third area of intervention focused on digitalisation to improve beneficiary data exchange within provinces and with the federal government. To this end, it was expected that the module would conduct analysis on social protection system processes (i.e. on normative principles, financial flows and IT structures) (R24) in order to develop (R25) and use (R26) a concept and operational software for the reinforcement of the Beneficiary Registration System (BRS) in the KP province that would have the capacity to be integrated with other systems for intra-province coordination. The reinforced data sharing was expected to contribute to greater coordination within provinces and with the federal level (Output 3). In turn, the greater coordination capacities, frameworks and systems should improve the capacities of the actors for the analysis of system and impact processes to be used for strategic dialogues and iterative planning for SP systems building (R27). This should ultimately contribute to improving the supply- and demand-side conditions for the implementation of social protection programmes (MO). The **key underlying assumption** was that the provincial and federal levels develop solid trust in order to support intra-province coordination. The other assumption is that there is sufficient capacity to integrate the digital systems. The module also relies on the fact that the staffing situation of the institutions working in the field of social protection (i.e. the availability of adequately qualified staff) enables the secondment of specialists to training programmes. **Risks** for achieving the output are that political standstill can create unfavourable conditions for cooperation during the module period.

Output 4 focused on increasing the efficiency of the mechanisms of BISP for interacting with the target groups at sub-district (tehsil) level and national level. **The main activities** implemented by the GIZ module team (together with BISP) included providing support to complement the digitalisation of the BISP communication processes with its beneficiaries, and supporting the establishment of (mobile) registration centres to improve the process used with the new dynamic NSER. With regards to the digitalised communication processes, the module intended to complement the already existing one-way communication of the BISP with its beneficiaries to upgrade it to two-way communication. This was done by creating communication tools such as a hotline and social media systems and the distribution of smartphones to beneficiary committees to implement an SMS service (R28). This was expected to support the communication campaigns of BISP (R29) to communicate

more efficiently to refine the targeting of its beneficiaries (Output 4). The second area of intervention was expected to focus on the establishment of mobile and static registration centres (R32). For mobile centres, the module would provide vans and mobile human resources, with capacity-building, to reach those people who are not able to go to existing registration centres (R30). For the static centres, the module would support the centres with equipment (such as ticket machines and megaphones) and support access to social worker positions in the centres for BISP beneficiaries (R31). Together, the two interventions would be expected to improve the efficiency of the communication of BISP with its beneficiaries (Output 4). In turn, this should support the registration of beneficiaries into the dynamic NSER under BISP and other social protection programmes (R33). The increased registration numbers should therefore contribute to improving the connection and information-sharing between supply and demand sides in social protection programmes and improve the responsiveness of the programmes to the recipients' needs (R34). Ultimately, the better communication and easier registration processes should contribute to improving the supply- and demand-side conditions for the implementation of social protection programmes (MO). The **third contribution hypothesis** that was selected for further investigation is that through the support to establish (mobile) registration centres (R31, R32), the module should contribute to improving the efficiency of the communication of BISP with its beneficiaries by increasing its outreach in more remote areas such as Balochistan, parts of Sindh and KP (Output 4). In addition, this should support the registration of beneficiaries into the dynamic NSER under BISP and other social protection programmes (R33). R34, discussed above, also applied to this hypothesis.
The **key underlying assumptions** were that by adopting a transformative approach, the module ensured that conditions for registration were enhanced by strong engagement from the site supervisors and contact persons for the target groups. In addition, the ability of target group members to use the BISP communication system was not restricted by drastic price increases for telecommunications. Moreover, the financing of dynamic registration in permanent registries is secured in the long term. Another assumption was that the political priorities for the implementation of BISP will not change significantly. Finally, it was also expected that international development partners continue their support for BISP. The main **risks** for achieving the output were the degradation of the security situation in the programme provinces and districts, the financial sustainability of the programme, and the political direction and further devolution of social protection programmes, including BISP, to the provinces in line with the 18th amendment of the Pakistan constitution.

Output 5 focused on the implementation of a framework for the cross-sectoral integration of social assistance and nutritional support for adolescent girls and their communities. The implementation of this framework has begun in selected parts of the country at sub-district (tehsil) level and national level. **The main activities** implemented by the module (together with co-financier GF) were the support to develop a concept and to implement a pilot that links social protection programmes to food security and nutrition and to conduct capacity development measures. Based on the interest expressed by BISP in expanding its instruments of social protection, the module relied on the pre-existing educational component of the social protection programme, Taleemi Wazaif, to develop an innovative cross-sectoral concept for conditional cash support linking social protection with improved nutrition, nutrition education and food fortification with a focus on adolescent girls (R35). It was then expected that the concept would be piloted to target girls being enrolled in school to receive iron folic supplements and to help them better understand nutrition challenges (R36). In parallel, the participants also received capacity development measures to be enabled to eventually become agents of change in their communities (to a large extent, depending on the region, sometimes within households and sometimes in entire communities) (R37). This concept and the capacity measures were then expected to consolidate a framework for the cross-sectoral integration of social assistance and nutritional support for adolescent girls (Output 5). In turn, the module intended to contribute to the objective of Output 4 to increase the efficiency of BISP's mechanisms for interacting with the target groups. Ultimately, better communication should contribute to improving the supply- and demand-side conditions for the implementation of social protection programmes (MO). The **key underlying assumptions** were the same as for Output 4 considering it relies on the same partner structures. The risks for this output were related to the sensitivity of the intervention in relation to the specific target group and more community-based work, since providing supplements to young

girls requires careful preparation and communication with families and communities (to ensure the supplements are well understood and accepted).

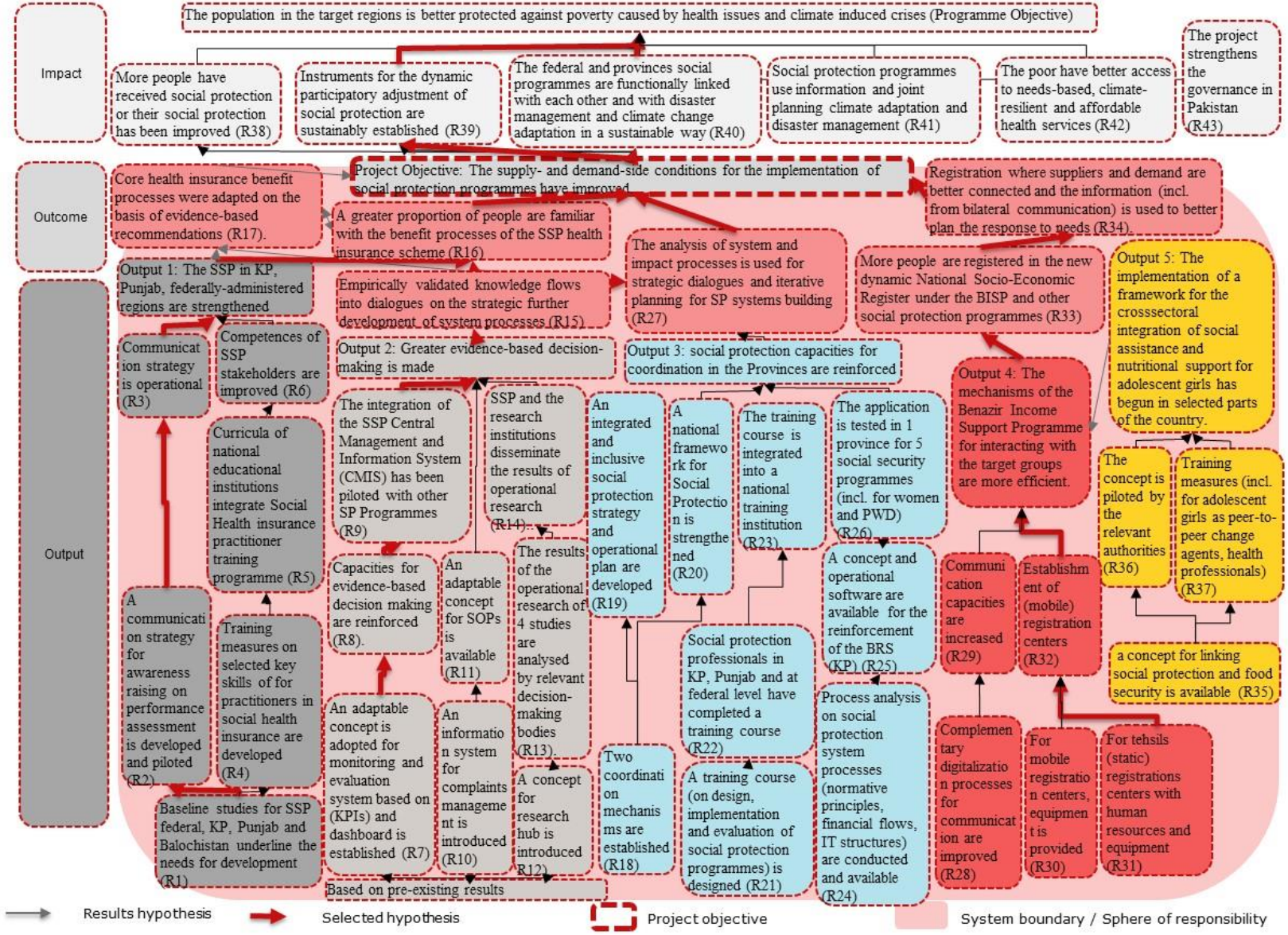
Impact level. The improved supply- and demand-side conditions for the implementation of social protection programmes (**MO**) could then lead to more people receiving social protection; moreover, the social protection they receive should be improved (**R38**, DC programme objective) through enhanced coverage and benefits. Eventually, the module contributes at impact level to BMZ markers Gender Equality (GG) 1 and Poverty Orientation (AO) 2. The **fourth hypothesis** selected for the evaluation is that by supporting the dynamic tool for NSER, which should serve as the basic registry for all SP programmes and for some adaptive instruments for different risks, the module should contribute to improving the supply and demand conditions for the implementation of SP programmes (MO). In turn, the module contributes to sustainably establishing instruments for the dynamic participatory adjustment of social protection (**R39,** DC programme objective 2), and also to the following BMZ markers: GG 1, AO 2 and Participatory Development/Good Governance (PD/GG) 1. The module should therefore contribute to improving the coverage and benefits for SHP, thus contributing to better protection of vulnerable households against health risks (programme objective). The **fifth hypothesis** selected for further investigation was that by developing data-sharing agreements within provinces and with the federal government (**R27**), the module was expected to contribute to developing a federal social protection programme and should therefore contribute to functionally linking the federal- and province-level social programmes with each other and with disaster management and climate change adaptation in a sustainable way (**R40**, DC programme objective 3); it should also contribute to the above-mentioned BMZ markers (GG 1, AO 2 and PD/GG 1). The aim of the module was therefore to improve the governance of social protection and to provide better protection for vulnerable households against health risks (programme objective). By improving information-sharing and coordination, the module also contributed to establishing the basis for social protection programmes to use information and joint planning of climate adaptation and disaster management (**R41**, DC programme objective 4); it also contributed to BMZ markers GG 1 and AO 2. By supporting better capacities from the supply and demand sides, the module also contributes to the poorest of the economically disadvantaged people having better access to needs-based, climate-resilient and affordable health services (**R42**, DC programme objective 5) as well as to BMZ marker AO 2. Additional impact also includes strengthening of the governance in Pakistan (R43), which contributes to BMZ marker PD/GG 1. All together, these results are expected to contribute to the programme objective: the population in the target regions is better protected against poverty caused by health issues and climate-induced crises. The **key underlying assumptions** were that use of the systems and processes lay the foundation for the development of strategies to better reach the target groups, focusing on those that are active in the informal sector and have so far been insufficiently recorded by many social protection programmes. The improved system will also help to reduce inclusion and exclusion errors. In addition, the production and use of operational knowledge is expected to lead to uptake from decision-makers and the establishment of nationwide guidelines and instructions. Lastly, support for the poorest of economically disadvantaged people was expected to be developed in relation to their nutritional situation, with a particular focus on adolescent girls from poorest-of-the-poor households and vulnerable households. The main **risk** for the contribution of the module to the identified impact areas is that although the financing of dynamic registration in the NSER and eventually also provincial registries is secured in the long term, there is the chance that target group members will not be able to use the BISP outreach and communication system due to drastic price increases in telecommunications services. In addition, the degradation of the security situation in the programme provinces and districts could be a risk for results at impact level, and changes to climatic conditions could affect the capacities of local actors and significant changes in political priorities. Further key risks were that the dynamic registry (to expand the coverage of SP) relied on active participation of beneficiaries, which could have proved particularly challenging in rural, remote or other difficult-to-reach areas. The module aimed to address this challenge through the mobile registration units and improved mobile communication options as well as functional communication systems in BISP. Similarly, the benefits of the SSP and SHPI were closely linked to the accessibility and quality of empanelled hospitals (i.e. a hospital that is officially recognised and approved by an insurance company to provide medical services) and therefore the Pakistani health infrastructure. The registration process is

generally linked to NADRA registration, which implied that about every fifth person was excluded (based on the 2017 census). This risk, however, was mitigated through the integration of BISP registration with NADRA such that bureaucratic barriers were reduced.

Finally, and according to its proposal, the module aimed to contribute to results regarding **cross-cutting themes,** in particular **human rights and gender safeguards**. First, the module was expected to make advances in human rights, considering that the availability and accessibility of social protection services is a human right that is addressed through this measure and through which people in vulnerable life situations are integrated into social systems. The improved access to health services through the module specifically contributes to the realisation of the human right to health. The poverty-oriented design of the systems in the sense of 'leave no one behind' contributes to the realisation of the human right to health and social protection for all, in line with the **Agenda 2030**. With regards to **gender**, the guarantee of equal access contributed to equal opportunities as a fundamental human rights principle. The creation of a uniform database for beneficiaries of social protection programmes and specific target group-oriented programmes, especially for people who were exposed to particularly high poverty risks (e.g. people with disabilities), meant that access to these programmes was made easier and more transparent. In general, the measure raised awareness among stakeholders for a gender-sensitive, inclusive and accessible implementation of social programmes and the citizens' centres were able to become a point of contact for corresponding advisory services.

System boundary. The results within the system boundary (depicted graphically by a red background in Figure 1) were expected to stem from technical and organisational capacity development in key partner organisations. Beyond the system boundary, it was expected that the module outcomes would contribute to providing better conditions on the supply and demand sides of social protection programmes to be implemented sustainability and effectively. For the module outcome, which lies partially within the boundary, some impact is expected from the project while external factors may help or hinder the result (see above). One key external factor outside of the system boundary is the availability and continuity of the social protection programmes; this is outside the module's immediate control. This includes the extent to which their continuation is subject to financial sustainability. The continuity of the different social protection programmes is a key underlying assumption across different results hypotheses.

Figure 1: Current results model (June 2024) adapted during evaluation



3 Evaluability and evaluation process

This chapter aims to clarify the availability and quality of data and the process of the evaluation.

3.1 Evaluability: data availability and quality

This section covers the following aspects:

- availability of essential documents,
- monitoring and baseline data including partner data, and
- secondary data.

Availability of essential documents

Most of the essential documents were available to be assessed during the evaluation and are listed in the reference section of the report (see list of references). These essential documents were largely assessed to be of sufficient quality. However, no centralised M&E existed for the module (see below in monitoring data). In addition, no capacity development strategy was developed. Also, considering the relatively new nature of the developments of the SP and SHP sectors in Pakistan, no other national frameworks than the ones supported by GIZ were available.

Monitoring and baseline data including partner data

The module's results-based monitoring can be best characterised as an informal and decentralised system. Part of the reason for this is staff turnover but also the dynamic adaptation the module underwent in response to the flooding in 2022. Staff turnover and the current temporary lack of a dedicated M&E officer for the module might also play a role in this regard. Excel tools were used to keep track of some indicators, though these likely relate to the predecessor module or programme objectives and are no longer actively used. For Output 5, there is a shared monitoring tool (results framework and tracker) from the co-financier GF which will be used by the module for this component. However, for the main part, the monitoring is decentralised, and output-specific information unsystematically relies on various documents, reports and data from stakeholders and partners as well as research institutes to measure its outputs and outcomes. The use of administrative data from partners, such as BISP or the SHP programmes and insurance company, should be noted. This ensured the module used high-quality data sources for information on the module's final beneficiaries. The module monitors risks and unintended results in most of its activities as a feedback mechanism (Int_16). For instance, focus group discussions (FGDs) with beneficiaries were conducted during communication activities implemented by NRSP. Another example is the regular monitoring of the satisfaction of staff hired to implement or support the implementation of the dynamic registration.

The evaluation team also reviewed the module's **baseline data** to the extent feasible. Of the four MO (outcome) indicators, two have baseline values above zero. The indicators with a baseline value above zero are the share of beneficiary households who are deemed to know the benefits and processes of the SHP set at 20% as well as the number of people registered through the dynamic NSER set at six million at baseline. Since the module joined to support the dynamic NSER not from the start and because the SHP was implemented and people were aware of it before the module's activities, partly due to the predecessor project, the use of values above zero is appropriate. The remaining outcome and output indicators that have baseline values of zero focus on the outputs that can be clearly linked to the project's activities and in case of the outcomes can be

linked to the module's output and measure related changes in processes and policies respectively. The evaluation team used **the monitoring data** of the module and the documents which they are based on. The module team shared an updated version of its current monitoring data for the evaluation.

Secondary data

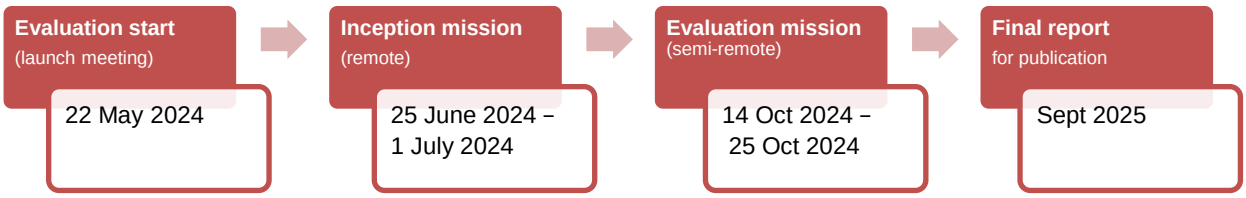
In order to assess the module's relevance, effectiveness, impact and sustainability, the evaluation mostly relied on primary data (interviews, FGDs and group interviews) as well as monitoring data (including partner data mentioned above) and studies conducted as part of the module's activities. Also, the evaluation team made use of policy strategies used in the module which are deemed to be beneficial to foster greater supply and demand processes for the social protection system. Throughout the evaluation, sources from national or regional statistical offices as well as documents from other relevant actors (such as KfW and the World Bank (WB)) including related studies on social protection were considered if needed. This data was triangulated whenever possible with sources considered accurate and reliable.

3.2 Evaluation process

This section covers the following aspects:

- milestones of the evaluation process,
- involvement of stakeholders,
- selection of interviewees,
- data analysis process,
- roles within the evaluation team,
- (semi-)remote evaluation,
- context and conflict sensitivity within the evaluation process, and
- security management in the evaluation process.

Figure 2: Milestones of the evaluation process



Involvement of stakeholders

The evaluation team followed a participatory approach that fostered ownership of the evaluation results and provided the basis for learning that can be used in future interventions. The participatory approach implied that the evaluation team described the purpose of the evaluation to interviewees and considered the issues that stakeholders would like to see addressed. It also meant that the evaluators were transparent about how the evaluation findings were derived from the data and gave stakeholders the opportunity to comment on the evaluation results. In this regard, a debriefing workshop with the module team was conducted after the evaluation mission and the module team will receive a draft of the evaluation report to provide comments. In line with the participatory approach, all interviewees were informed about the purpose of the evaluation when they were contacted for an interview. They also received an interview guide prior to the meeting and the invitation to share relevant information that goes beyond the questions in the guide. As part of the inception phase, the evaluation team held preparatory discussions with the GIZ evaluation unit and the AV. Exploratory interviews were conducted with module staff as well as with additional GIZ representatives, including the group leader for Bangladesh and Pakistan, the responsible technical and methodological department staff, and the GIZ person responsible for the sustainable economic programme, to which the module was previously

assigned, to deepen the evaluation team's understanding of the module and come to a common understanding of aspects to be addressed in the evaluation. The evaluation unit and the module manager had the opportunity to comment on the inception report prior to the evaluation mission. According to our participatory approach, all interview partners, including external actors, were informed about the objective of the evaluation when they were contacted for an appointment. Further, they received an interview guide before the meeting.

Selection of interviewees

The list of stakeholders interviewed can be found in Table 3. The list was based on a joint selection and prioritisation with the GIZ module team. The selection/prioritisation considered the importance of the suggested persons for the module (degree of involvement and knowledge of the module) and their availability. Therefore, the final sample of interviewees/FGD participants was not a representative sample, but a purposive sample composed of those persons who could provide the most useful information. The evaluation team also aimed to select a diverse representative sample of participants (including women). The geographical focus of the evaluation was on the provinces of KP and Punjab and federal partners, considering aspects of representativeness of module activities, access and location of relevant interview partners. As it was methodologically difficult to involve the indirect target group (the general population) in the assessment, their participation and therefore data directly obtained from them by the evaluation team was limited. The evaluation team responded to this limitation by discussing the indirect target group and the module's impact on them in most interviews during the evaluation mission and then triangulated the results.

Table 3: List of evaluation stakeholders and selected participants

Organisation/company/target group	Overall number of persons involved in the evaluation (including gender disaggregation)	No. of interview participants	No. of focus group participants	No. of workshop participants	No. of survey participants
Donors and other development actors	6 (3m ,3 f)	6 (3m ,3 f)			
German Federal Ministry for Economic Cooperation and Development (BMZ), GF, Asian Development Bank (ADB), WB					
GIZ	16 (9m, 7f)	16 (9m, 7f)			
GIZ module team, GIZ Cluster Coordinator, GIZ Country Representative, GIZ Technical and Methodological department, GIZ Regional West Bureau, GIZ Country Director					
Political partner	1 (1m)	1 (1m)			
Ministry of Finance, Revenue and Economic Affairs					
Other German cooperation projects	3 (2f, 1m)	3 (2f, 1m)			
KfW project responsible for Healthcare financing II and Healthcare financing III; GIZ sectoral project: Development and integration of social security systems; GIZ ASP					
Government cooperation partners (direct target group)	14 (14m)	14 (14m)			
PSPA, Planning and Development Department Khyber Pakhtunkhwa, Social Welfare Department Khyber Pakhtunkhwa, federal SSP, SHPI, PHIMC, BISP, MoPASS, specialists and managers of Social Protection Programmes (Social Protection course participants)					
Implementation partners and other direct target groups	12 (10m, 2f)	12 (10m, 2f)			
GFA Consulting Group, HSA, NRSP, SDPI, PIDE, Oxford Policy Management, Nutrition International Pakistan, Center for Economic and Policy Research					

Organisation/company/target group	Overall number of persons involved in the evaluation (including gender disaggregation)	No. of interview participants	No. of focus group participants	No. of workshop participants	No. of survey participants
Other actors	4 (4m)	4 (4m)			
NADRA, external social protection researchers, State Life Insurance Cooperation of Pakistan (SLIC), former director of PSPA					
Final beneficiaries/indirect target groups (sum)	**8**		**8**		
BISP staff (compliance monitors and ushers and supervisors) in KP and Punjab	2 (2m)		2 (2m)		
Leadership for Universal Health Care (L4UHC) course participants	4 (3m, 1f)		4 (3m, 1f)		
Mobile BISP van	2 (2m)		2 (2m)		
Note: f = female; m = male					

Data analysis process

Within the framework of the module evaluation, the **data collection** process included the following methodologies: document analysis, monitoring data analysis, secondary data analysis, qualitative interviews (individual/group), focus groups, and a theory of change workshop. In terms of the principles that guided the data collection, the evaluation team sought to ensure the reliability, credibility and validity of the data collected. The evaluation team also aimed to protect the privacy of the people involved. In that sense, it collected the minimum amount of personally identifiable information necessary for the required data; the interview guides, shared prior to the meetings, also included information on data protection. Through the **continuous analysis** of the module documents in the evaluation phase, the knowledge base for the evaluation mission was expanded. This was achieved through the strategic analysis of the various documents provided by the module team, following the questions in the evaluation matrix. All interviews and FGDs were documented through protocols. The guides were drawn up and consolidated by the evaluators in preparation for the evaluation mission. These were coded to indicate relevant evaluation questions, and key statements were summarised at the end of the protocols. Specific guides were drawn up for each type of stakeholder interviewed. The monitoring data was analysed qualitatively against the module indicators and limited descriptive quantitative analysis was provided, based on need and relevance for the analysis. The data from the interviews, document analysis and monitoring data was aggregated and documented in the evaluation matrix (developed during the initial phase). At the end of the evaluation mission, the evaluation team organised an internal summary meeting to discuss how to interpret the data. **Triangulation** of data and methods took place at several points during data collection and analysis. Firstly, the evaluation team ensured that the researchers were triangulated by reflecting on the results of the interviews at the end of each day of the evaluation mission. Secondly, when synthesising and analysing all data after the evaluation mission, the evaluators conducted an internal synthesis meeting to exchange ideas on how to interpret the data (researcher triangulation) and whether the different methodological tools used produced compatible analytical results (data/method triangulation). Triangulation with module stakeholders took place at the workshop held to present the results of the evaluation mission (if relevant). This workshop brought together the module team, with whom it was then be possible to discuss the recommendations. The participation of the module partners (policy and implementation) was agreed with the module team. Evaluators communicate transparently in cases where no clear joint conclusions can be drawn from the data.

Roles within the evaluation team

The evaluation team is composed of one international evaluator and a local evaluator. Their profiles complement each other in an ideal way: the international evaluator mostly provides methodological evaluation expertise and background knowledge on the specific requirements of German DC, as well as on decentralisation and good governance. The local evaluator from Pakistan in turn provides specific sectoral expertise in the field of social protection as well as evaluations in the Pakistan context. Together, the evaluators reflected on the findings from the documents and interviews in light of the specific country context of Pakistan. Tasks in this evaluation were divided according to the specific knowledge of the evaluators. While the international evaluator was responsible for the overarching project management and for setting up the evaluation design, drafting the data collection tools and planning the remote missions, the local evaluator made an important contribution in the preparation of the evaluation mission by co-conducting interviews and providing input on the local context of Pakistan. She also provided comments on the data collection tools and selection of interview partners. Jointly, the evaluation team analysed the documents provided by GIZ and reflected on the interview results against the indicators in the evaluation matrix. Furthermore, both evaluators shared responsibility for documenting the interview results. An exception are those interviews that were held in German or via telephone before/between the inception and evaluation missions (e.g. with BMZ or GIZ headquarter staff). The responsibility for reporting (inception report and final report) lies with the international evaluator and is supported through backstopping and specific thematic expertise by the local evaluator.

Semi-remote evaluation

Based on the terms of reference of the module and discussions between the evaluators, the evaluation unit and the module team, a semi-remote approach was chosen for the evaluation mission: while the international evaluators attended interviews virtually from Germany, the local evaluator attended interviews and FGDs on site (where possible/appropriate). The evaluation team used common software/applications, such as MS Teams or Zoom, to meet the stakeholders' preferences for specific platforms (including taking into account unstable internet connections and offering phone options instead). In all cases, researcher triangulation was employed to ensure that methods for reaching and surveying contacts fit the respective target groups.

Context and conflict sensitivity within the evaluation process

The evaluation was conducted in a context that is considered fragile. The evaluation team therefore continuously reflected on conflict sensitivity (do no harm) to avoid (indirect) negative outcomes, mitigate and address risks, and avoid unintentionally harming counterparts and stakeholders. Table 7 details risks and mitigation measures, also to ensure that the team acted in a culturally appropriate manner.

4 Assessment according to OECD DAC criteria

The evaluation matrix (provided in the Annex) provides a detailed overview of the evaluation dimensions and analysis questions that will be examined under each evaluation criterion, including indicators, available data sources, planned data collection methods, evaluation design and expected evidence strength, based on the availability of sources of evidence. In the following, a narrative description sums up key aspects of what will be analysed regarding the OECD DAC evaluation criteria, as well as additional questions pertinent to the interest of the main stakeholders of this evaluation.

4.1 Impact and sustainability of predecessor projects

This section analyses and assesses the impact and sustainability of the predecessor project: Support to social protection, including social health protection, Pakistan (PN 2015.2186.3).

Summarising assessment of predecessor project

Through its pivotal role in the establishment of the SHP programmes, the predecessor SP-SHP module had a significant impact for the population in Pakistan through improved access to health services. The SHP programmes, with eligibility for all officially registered people in the provinces of KP, Punjab and federal-administered territories, cover expenses for inpatient services in empanelled hospitals. A total of 12 million patients have already benefited from the SHP programmes since 2015. The successful engagement with key stakeholders further fostered an enabling environment for the current SP-SHP module which was able to build on the established reputation of GIZ as a trusted partner with expertise in the areas of social protection and health financing. By continuing its support to the SHP programmes, the current module directly builds on the established relationships and expertise as well as the results of the predecessor project. Beyond the SHP programmes, the current module further expanded the results of the single-window services (OWO) initially piloted as part of the predecessor module and generated additional insights which can inform future reforms towards more digitalisation and less fragmentation for more access and efficiency of social protection in Pakistan.

Analysis and assessment of predecessor project

The predecessor module was implemented from July 2016 until September 2020 before the current module commenced. The predecessor module focused on (i) establishing four centres (single-window services) that would integrate different social protection processes and services, and (ii) increasing the number of people accessing healthcare through the SSP and SHPI in Punjab and KP while ensuring that (iii) both male and female beneficiaries benefit equally from the insurance. Since the module under evaluation is partly a continuation of the predecessor project, the projects' impact and sustainability are closely interlinked. The predecessor project's impact and sustainability were assessed via analysis of module documents and secondary data from the SHP programmes (federal SSP and SHPI in KP) and semi-structured interviews.

The impact and scale of the GIZ-supported SHP programmes in Pakistan is huge and a key component of the country's strategy towards Universal Health Coverage (UHC). All officially registered members of the population of the provinces of KP, Punjab and the federally administered territories are eligible for the SHP programmes. Since 2015, the SHP programmes have benefited over 12 million patients (Int_62). These programmes cover expenses for inpatient treatments in empanelled hospitals, including over 30,000 cancer and over 70,000 dialysis patients, thereby providing access to health services for the poorest-of-the-poor households (Int_62). While utilisation remains below expectations (Int_20), the scale of the SHP programmes worth EUR 45 million per year cannot be understated (Int_4).

The following paragraphs look at the sustainability of the predecessor module, improvement of the processes and capacities of the providers of the SHP programmes, the extent to which it fostered an enabling environment for the activities of the current module and the implementation of SHP programmes, and the extent to which the single-window services approach was used to inform the current module or processes.

The predecessor SP-SHP module introduced gender-sensitive monitoring of the use of SHP programmes to ensure that both women and men benefit from the programmes. The current SP-SHP module continues to support the development and implementation of the monitoring system. Based on the shared monitoring data from 2022, SHP use continues to remain gender-balanced such that both men and women benefit in similar proportions. Since financial constraints are one factor for the prevalent trend of home births, the SHP

programmes could work to significantly increase the number of hospital births, especially for poorest-of-the-poor households (Int_38). Through this, the SHP programmes have the potential to address a key health risk for women in Pakistan. Pakistan has one of the highest maternal mortality rates, such that access to healthcare is critical for pregnant women. While the data on health impacts is scarce and it is too early to draw conclusions, it is plausible to expect that the introduction of the SHP programmes could reduce maternal mortality rates in the province of KP (Int_38).

The predecessor and current SP-SHP modules have been key in institutionalising the social health protection programmes in Pakistan. The predecessor module supported the first programmes from their inception almost 10 years ago (Int_4). The predecessor supported the provinces in piloting and expanding approaches and institutionalising the social health protection programmes (Int_2). Before the predecessor module, the concept of social health protection was unfamiliar and there was widespread doubt among policy-makers that health insurance could work (Int_36, 62, 8). Now, by contrast, there is a broad consensus among policy-makers to continue the SHP programmes (Int_36, 46). The SP-SHP predecessor module played a key role in these developments through its engagement and advocacy efforts (Int_62). Further, the technical assistance strengthened the IT infrastructure and operations, thus increasing the efficiency of the SHP programmes (Int_62). Together with the KfW, the predecessor module is seen as the key driver behind the SHP programmes, and this would not exist without the engagement of German development cooperation (Int_56). The predecessor and current SP-SHP modules have helped to shape the health sector which exhibits increased engagement and collaboration between insurance companies, healthcare providers and government entities, thereby advancing Pakistan's health sector (Int_46).

The predecessor module's critical role in establishing the SHP programmes strengthened GIZ's reputation and position, paving the way for the effective implementation of current and future modules in Pakistan. The German development cooperation's project-based approach challenges continuity and limits its ability to lobby and support long-term policy reform (Int_56). For instance, despite its influence, the SP-SHP module was unable to prevent the politically motivated expansion of the SHP programmes beyond the poorest parts of the population (Int_56). While this expansion enlarged the SHP programmes to their current scale, the predecessor module advocated against this expansion since the lack of co-payments raises doubts about whether the current scale can be sustained and securing financing remains one of the key challenges for the provinces and federal government. Nevertheless, due to its long and continued engagement through the predecessor module and earlier projects, partners value and trust the relationship with GIZ and recognise its expertise (Int_12, 38, 40, 56). Through the predecessor module, GIZ was able to position itself as an important actor with a high level of technical expertise in the areas of social protection and health financing (Int_40, 56).

In addition, the single-window service pilots from the predecessor project allowed the SP-SHP programme to continue its efforts towards a more integrated and digitalised provision of social protection in Pakistan to improve its efficiency and access. The single-window services approach represents a first step towards digitalisation and reducing the complexity of the fragmented social protection landscape in Pakistan (Int_4, 5). The SP-SHP module continued the expansion of the single-window services, building on the results of its predecessor. However, due to a shift in political priorities and a lack of funding, further pursuing the single-window services was mostly abandoned by the current module when it instead focused its support on the dynamic NSER. Although it was mostly abandoned, the potential of the single-window services approach was recognised, and learnings were included in social protection strategies and could further inform future developments in this direction (Int_4, 5, 23, 26, 34).

Methodology for assessing predecessor project

Table 4: Evaluation matrix for the assessment of predecessor project

Predecessor project: assessment dimensions	Basis for assessment	Evaluation design, empirical methods and data sources	Data quality and limitations
Impact of predecessor project	This dimension is considered achieved if the impacts of the SP-SHP are still visible, in particular in terms of: • utilisation of the SHP programmes, i.e. the number of claims and share of beneficiaries who used the insurance, and • the gender equity in use, i.e. the extent the use differs between male and female beneficiaries.	**Evaluation design:** The evaluation analyses the impact of the previous project by following the questions in the evaluation matrix. **Empirical methods and data sources:** • Analysis of project documents of predecessor project. • Semi-structured interviews with stakeholders (project team and partners). • Analysis of secondary and implementation data of the SHP programmes.	Because of the high degree of continuity (in terms of personnel and content), there is familiarity in parts of the project team with the previous phase as well as within the structures of the direct target group and cooperation partners. Through established data-sharing channels with the SHP programmes, the data continued to be available for the relevant indicator(s). **Overall data quality for this assessment dimension:** strong
Sustainability of outcomes of predecessor project	The evaluation analyses the sustainability of the results of the predecessor, in particular in the following areas: • improvement of the processes and capacities of the direct target groups, especially with respect to the provision of the SHP programmes, • extent to which learnings from the (abandoned) single-window approach are used in the current project's activities or the processes of the social protection programmes, and • the extent the results of the predecessor project fostered an enabling environment for the activities of the current project (and similar projects) and the implementation of the SHP programmes.	**Evaluation design:** The evaluation analyses the sustainability of the previous project by following the questions in the evaluation matrix. **Empirical methods and data sources:** See above.	See above. **Overall data quality for this assessment dimension:** strong

4.2 Relevance

This section analyses and assesses the relevance of the project. It is structured according to the assessment dimensions specified by BMZ (see Annex).

Summarising assessment and rating of relevance

Table 5: Rating of OECD DAC criterion relevance

Criterion	Assessment dimension	Score and rating
Relevance	Alignment with policies and priorities	30 out of 30 points
	Alignment with the needs and capacities of the beneficiaries and stakeholders	30 out of 30 points
	Appropriateness of the design	12 out of 20 points
	Adaptability – response to change	20 out of 20 points
Relevance total score and rating		Score: **92 out of 100 points** Rating: Level 1: highly successful

Overall, the relevance of the module is assessed as **highly successful**. In particular, the module aligns with key international frameworks (in particular with Sustainable Development Goals (SDGs) 1, 3 and 5) and German policies and priorities (e.g. BMZ's 2023 German development policy with Asia, BMZ's core area strategy on health, social protection and population dynamics and the German Feminist Development Policy). At national level, although there are limited formalised government-led frameworks for social protection and social health protection considering the relatively new nature of the sectors, the module nonetheless aligns with the priorities observed at the level of key actors in the sector (support in the governance of the SP-SHP programmes in Pakistan, support of key actors, like HSA, and support of systems like the Zakat system). In addition, the module is well aligned with the needs of the direct and indirect groups in terms of capacity development (for direct groups in terms of technical and organisational means and indirect groups through the reinforcement of the entire SP-SHP system to improve their protection) in addition to financial support, policy framework, technological/digital capacities, communication strategies and networking for the direct target groups. Furthermore, the module was developed on a solid design, was built on a strong multilevel approach, had the right targeting of sectors to answer the needs of the target groups, deployed relevant and timely activities to achieve the objectives, and selected the right partners. Some weaknesses in the traceability of the module were observed, such as the lack of details in the results model and centralised M&E. In addition, the evaluation also finds two weaknesses in the concept of the project, in particular regarding the conceptualisation of Output 2 and the remaining compartmentalisation of the SP and SHP topics. Lastly, the module maintained a high level of relevance by continuing to adapt to emerging needs, e.g. by cooperating with BISP to address the need for a dynamic registry for SP and with GF to pilot an innovation in existing social assistance programmes. **In total, the relevance of the module is rated as highly successful (level 1), with 92 out of 100 points.**

Analysis and assessment of relevance

The relevance criterion analyses the extent to which the project's objectives were aligned with policies and priorities and consistent with stakeholders' needs and capacities as well as the extent to which the module was appropriately designed to meet them. The criterion also assesses the project's adaptability to change. The methodological approach adopted for the analysis of this dimension is included in Table 6.

Relevance – Dimension 1: Alignment with policies and priorities

At a global level, the module contributed to the Agenda 2030 and the SDGs. SDG 1 No Poverty and SDG 3 Good Health and Well-Being were both addressed through the project's main objective of improving the supply- and demand-side conditions for the implementation of social protection programmes. Output 1 aimed to strengthen the supply-side capacities of the SSP by operationalising a communication strategy, which would increase demand by familiarising a larger segment of the population with the programme, thereby aligning with the above-mentioned SDGs. Outputs 2 and 3 focused on improving evidence-based decision-making and enhancing coordination capacities of relevant actors in social protection across KP, Punjab, Balochistan and federally administered territories, supporting progress towards SDGs 1 and 3. Enhancing the efficiency of BISP was the aim of Output 4, which supports the establishment of (mobile) registration centres and improves digital communication processes, fostering two-way communication with beneficiaries and strengthening the connection between the supply and demand side, ultimately contributing to the same SDGs. As Output 5 focused on integrating social assistance and nutritional support for adolescent girls, it also contributes to SDG 5 Gender Equality, in addition to SDGs 1 and 3.

Regarding the priorities of the German government, at the time of its conceptualisation and implementation, the module aligned closely with the policies of BMZ. Specifically, the module aligned with one of the five cornerstones outlined in the **BMZ's 2023 German development policy with Asia**, namely Cornerstone IV – Harnessing the opportunities of social protection as an investment in the stability and future of Asian societies. This alignment was achieved through the project's efforts to enhance evidence-based

decision-making processes, to strengthen coordination capacities, and to develop a comprehensive communication strategy aimed at improving the supply-side capacities of the SHP programmes (MO, Outputs 1, 2, 3). Additionally, Output 4 of the module aligned with the BMZ policy sub-item – Spotlight on digitalisation of Cornerstone IV – by implementing digital two-way communication between the administrators and the beneficiaries of BISP. The module also closely aligned with the **BMZ's feminist development policy.** The implementation of the framework for integrating social assistance and nutritional support for adolescent girls (Output 5) particularly aligned with field of action 1 of this policy by increasing access to resources and expanding social protection coverage for women and adolescent girls. This was achieved by improving both the supply- and demand-side conditions for social protection programmes, thereby contributing to the plausible expansion of coverage and the quality of social protection schemes in Pakistan. Finally, the module is closely aligned with **BMZ's core area strategy on health, social protection and population dynamics**, in particular the area of social protection. Through its objective, the module contributes to the goal of increasing the number of people who are adequately covered by social protection including social health protection as well as enhancing digitalisation and collaborating with other international actors.

Lastly, at national level, given the relatively new nature of the SP and SHP sectors, the existence of formalised frameworks before the start of the module was limited. However, the evaluation finds that the module has **aligned well with existing systems and efforts of the federal and provincial governments**.

First, by capitalising on the conducive national policy environment, the module's efforts to assist federal and provincial institutions in policy and institutional development align with **Pakistan's 1973 constitutional provisions.** Specifically, Article 38 emphasises the promotion of social and economic well-being of the people, positioning the module within the broader historical context of Pakistan's approach to social welfare. Article 38 mandates that the state ensures equitable access to essential services and provides social security to those unable to earn a livelihood. The project's main objective of improving both supply- and demand-side conditions for the implementation of social protection programmes aligns with the objectives laid out in the constitution, which were not achieved due to funding issues, shifting goalposts and political instability (Khan,, Arshi Rasheed 2007: 316-318). In addition, in Pakistan, **social protection system, social assistance and health programmes have been devolved to the provinces and should predominantly be managed at the provincial level.** However, the federal government assumes the role of a provincial authority in regions such as Kashmir and GB, where it administers these programmes. Instead of developing a country-wide strategy first and implementing it afterwards, programmes are developed and implemented at the regional level (Int_15, 4, 5). Hence, strengthening the capacities for better coordination of social protection programmes, as outlined in Output 3 of the module, captures the national policy environment and aligns with the significant need for standardisation within Pakistan's welfare system. Moreover, Pakistan has historically relied on the **Zakat system** to address welfare needs. While Zakat delivers social provision to a certain extent, it suffers from misallocation of funds to high-income households and is mostly financed by low-income households, which hinders its effectiveness (Khan, 2007: 2-4). By supporting the Zakat Department in KP with digital systems (Output 3), the module therefore aligns with the established systems of welfare in Pakistan.

Second, KP's provincial government introduced the **SHPI, back then as SSP and later Sehat Card Plus**, in 2015, which aimed to provide free health insurance coverage to poorest-of-the-poor households, marking an important step towards achieving UHC (Khan et al., 2013). Additionally, the **Health Services Academy (HSA)**, founded in 1988 as a training institution, was granted autonomous status under the Ministry of National Health Services Regulation and Coordination, positioning it as a key player in the research and teaching of public health. Through the implementation of a monitoring system and the operationalisation of a communication strategy (Outputs 1 and 2), the module aligns with efforts to expand access to health services provided by regional governments, such as KP, through medical professionals educated by HSA.

Overall, the module is aligned with international and German policies and priorities and with the institutional set-up of SP, SHP and Pakistan at the time of the project. In addition, although the existence of a formalised

SP-SHP framework was limited at the start of the module, it nonetheless aligned with the tendencies observed at the level of key actors in the sector. Therefore, relevance dimension 1 – alignment with policies and priorities – scores **30 out of 30 points.**

Relevance – Dimension 2: Alignment with the needs and capacities of the beneficiaries and stakeholders

The second evaluation dimension of relevance deals with the suitability of the module design to match the specific needs of its different target groups.

The first **direct target group and cooperation partners** included the **specialists and managers responsible for the effective implementation of social protection and social health protection programmes at federal and provincial level**. At federal level, these include SSP, BISP, MoPASS, NADRA, and the State Life Insurance Company (SLIC). At the level of the provinces targeted by the module, namely Punjab and KP but also Balochistan, the actors targeted are PSPA, PHIMC and KP Ministry of Health, within the SHPI, within the Planning and Development Department Khyber Pakhtunkhwa the Public Policy – Social Protection Reforms Unit and the Social Welfare, Special Education and Women Empowerment Department, and the Zakat and Ushr Department.

First, the module contributed to responding to the **capacity development needs of** specialists and managers from SP-SHP programmes through technical and organisational capacity development at an individual level (e.g. on SP and SHP) and at institutional level (e.g. technological, streamlining processes such as contracts and SOPs). As such, the module supported **various technical capacity-building efforts at individual level** through workshops, training, exposure visits and tailored curricula. For example, the module supported the development of a health insurance management module (i.e. L4UHC) and training 135 individuals in areas such as procurement, benefits package development, and healthcare financing (Int_46, 4, 8, 20, 36, 29, 62). Other examples include developing and implementing a course by supporting local service providers to design and deliver these courses that integrate SP-SHP topics (FGD_1, 4, 26). The module also supported governmental institutions, **at institutional level, with technological and organisational capacity development**. For example, the module addressed gaps in institutional capacity through the reinforcement of core processes like operational manuals, targeted support (e.g. IT consultants for digital registries, monitoring systems and data analysis) and communication strategy in KP and the federal level including Punjab to address the demand-side challenge (lack of community awareness) (Int_4, 8, 11, 40).

Second, the module also answered the **digitalisation needs of this first target group**. In line with KP's priority for digitalisation, the module supported the integration of **technology-driven solutions** into social protection governance. A first example is the module's support to develop Management Information Systems (MIS) for the Social Welfare Department and Zakat and Ushr departments, to improve transparency and targeting of social protection programmes in KP (Int_20, 4). Another example is the support to introduce an **online system for disability certificates**, the first of its kind in Pakistan (Int_20, 4, 9 ,10). Lastly, at national level, the module also promoted the **exchange of digitised social protection and social health protection data** between provinces. For example, the module supported advocacy and technical efforts to link BISP with Balochistan and with NADRA (Int_4, 7, 62, 11, 9, 10).

A third key aspect was **answering the need for financial and resource support.** The data indicates that the module supported its partners with equipment, such as office equipment (e.g. for partners such as SHPI, SSP and BISP), but also with financial support to ensure human resources could be deployed for the planned interventions that the partners could continue covering for afterwards (such as staff members at static and mobile registrations) (Int_40).

The module answered this direct target group's need for SP-SHP **policy frameworks and evidence-based decision-making**. As such, the data indicates that the module supported the development of Pakistan's first-ever SP conference and National Poverty Alleviation policy, as well as KP's first social protection policy (Int_4, 9, 10, 28). In addition, the module was key in supporting the brokering of a data-sharing agreement between BISP and SHP in Balochistan, enabling better coordination across systems (Int_4, 5, 8, 9, 11, 17, 66, 68 62). Lastly, the module emphasised the need to integrate evidence-based practices to address the key gap of lack of evidence for policy, through the development of several MIS, the development of dedicated capacities and the launch of the research hub gathering national and international expertise to conduct targeted research to be used for political decision-making. The gathering of several actors also addressed the disconnect between policy-makers, implementers and researchers, and was designed to raise policy-makers' awareness of the value of evidence and research (Int_9, 10, 54, 20, 67).

Moving on to the **second direct target group**, the module answered the need for capacity development of the **insurance companies, in this case SLIC**. For example, the module helped develop an operational manual and their data management processes, a monitoring tool based on KPIs, which provides real-time admissions and claims data, and a system for complaints to facilitate evidence-based decision-making. The SHP programmes are fully digitalised, and the module provided IT consultants to facilitate data analysis and reporting (Int_36, 13, 62).

Regarding the other direct target group (i.e. HSA, NRSP, Social Policy and Development Centre, IMSciences, Aga Khan University, SDPI and PIDE), the module helped with capacity development to support their role in the programme. For instance, the module created a pool of SP-SHP expertise, for example through SP courses and targeted support to carry out their activities. For example, HSA was supported to integrate the SHP module into its standard training (Int_4, 8, 46). The module also supported their networking and outreach efforts to identify needs and share results, for example by organising workshops and ensuring their role is responsive to needs (Int_55, 69).

Moving on to the **indirect target groups and final beneficiaries**, i.e. BISP staff (compliance monitors and ushers and supervisors) and the general population with a focus on the poorest-of-the-poor (those eligible for BISP), estimated at 10 million households made up of 65 million individuals, the module was aligned with their needs. With regards to BISP staff, the module supported them with capacity development for data collection and equipment, such as vans (Int_11, 4, 9). Regarding the general population with a focus on the poorest-of-the-poor, the module's support to the reinforcement of the entire SP-SHP system that answers the needs of the poor population, focused on nutritional support and aimed to strengthen the registration system through NSER and enhanced frameworks and ensure availability of robust data for decision-making (Int_4).

Overall, the module is well aligned with the needs of the direct and indirect groups in terms of capacity development; in addition, it is well aligned for the direct target groups in relation to financial support, policy framework, technological/digital capacities, communication strategies and networking. Therefore, relevance dimension 2 – alignment with the needs and capacities of the beneficiaries and stakeholders – scores **30 out of 30 points.**

Relevance – Dimension 3: Appropriateness of the design

This dimension assesses the appropriateness of the project's objective and results model with its outputs, activities, instruments and assumptions, as well as the implementation strategy. In this regard, the results model and underlying assumptions, as described in Section 2.1, serve as the central basis for the evaluation.

In terms of traceability, the evaluation looked at the results model developed for the project and the corresponding M&E system in place. A **results model** already existed that included a basic representation of output objectives and MOs, which **generally seemed plausible**. However, the existing results model lacked details on milestones at activity level, a complete set of milestones at output level and expected intermediary

steps at outcome level as well as milestones at impact level. In addition, the existing results model was not regularly updated to reflect the latest changes in the module concept. The general status of the existing results model therefore required revision by the evaluation team for the purpose of the evaluation. The joint refining, together with the module team, allowed the evaluation team to obtain a clear picture of all expected milestones at activity, output, outcome and impact levels, as well as to clearly integrate Output 5, which was included through a modification offer in 2022. This process also allowed the evaluation team to jointly select five plausible results hypotheses to be tested during the evaluation.

Beyond the lack of specific details of the results model, the traceability of the module was also weakened by the **decentralised structure of the module's M&E system** (see also Section 3.1). The decentralised nature of the M&E system made the tracking of expected/unexpected results unsystematic and created difficulties in obtaining a solid overview of advances. For instance, delays suffered in Output 1 and the lack of observable results for Output 1 should be mitigated by a strong M&E system that would allow the module team to track processes and (mid-term) results, which was not the case for the project. Nonetheless, the module still has a good oversight of their activities through individual efforts to track the interventions under Output 1. In addition, despite the strong orientation of the module towards sustainability, the M&E system did not include a measure to trace these results. The decentralised structure of the M&E system hinders good knowledge management (especially in case of staff turnover) and blurs the lines of responsibility on knowledge management. Finally, the evaluation finds some measurement gaps and multi-directional logic between outputs and module indicators. For instance, output indicator 1.1, the operationalisation of the communication strategy, was due to be informed by MO indicator 2 which measures the knowledge of beneficiaries linked to communication campaigns. In this case, the MO indicator is only indicative of a larger potential of the module's output. Further, the underachievement of output indicator 2.2, measuring the consideration of operational research studies, did not hinder the achievement of MO indicator 1 (evidence-based adaptations of core processes of the SHP programmes). Especially in this case, the output is not expected to always lead to tangible outcomes, because ex ante research is inherently uncertain such that tangible outcomes cannot be guaranteed. Finally, Output 5 is not clearly linked to the MO indicators such that the achievement of MO indicators does not depend on the implementation of Output 5.

Looking at the project's components, the **choice of the project's interventions in each of the sectors constituted a strength of the module design**. The module followed a sectoral approach, spreading across SP and SHP, which was viewed as appropriate when working on the early developments of such sectors in Pakistan. In this regard, the evaluation data indicates that the focus on these sectors was considered especially relevant by the public partners who have stronger incentives to develop strong processes and systems to answer the needs of the poorest-of-the-poor in Pakistan (Int_4; see also dimension 1 above). The evaluation also finds that **integration of the two key sectors** was adapted to implement activities and interventions that contribute to achieving the MO. The data further indicates that the selection of two sectors allowed the module to be flexible in its activities and interventions to address the needs of its partners through multiple activities, including innovative ones, such as Output 5 combining social protection and nutrition for young girls, and to face evolving priorities (Int_4, 11, 62; see also dimension 4 below). In this regard, the historical engagement of GIZ in support of SHP and SP in Pakistan since 2015 has facilitated this flexibility as the module builds on pre-existing relationships and on the reputation of GIZ among governmental and non-governmental actors in the sectors, and looks to capitalise on significant results (Int_4, 5, 12, 36, 69, see also Section 4.1). For instance, GIZ's early focus on health systems research and reforms in KP laid the foundation for broader engagement across other provinces, including Punjab, Balochistan and AJK. Through continuous support, over the years, GIZ has demonstrated its capacity to adapt and contribute significantly to Pakistan's evolving social protection needs as well as German DC priorities, such as moving away from bilateral cooperation on health to integrate into a broader focus on social protection (Int_4, 5). Therefore, the decision to combine SP and SHP activities within the module, while initially seen as a temporary solution, underscores GIZ's pragmatic approach to navigating political changes. The hybrid nature of these initiatives also facilitated

co-financing opportunities with the EU in the other GIZ module, ASP, enabling GIZ to extend its reach to underdeveloped regions like Sindh and Balochistan (Int_4).

However, the refining of the results model contributed to shedding light on the **compartmentalisation of the sectors** targeted by the module. In line with successive modification offers (GIZ, 2022a; 2024a), the module has gradually integrated the social health protection sector into its design. While the evaluation team identified theoretical synergies between the two sectors, there appeared to be a compartmentalisation of the sector, with limited concrete realisations of synergies and mutual effects observed. This can be explained by external and internal factors. Externally, national actors from SP and SHP agencies are working in silos. For instance, SP and SHP agencies face challenges in data sharing that would be beneficial for their respective mandates due to the limited incentive to encourage data sharing (Int_4, 7, 62, 11, 9, 10). This lack of incentive for cross-agency synergies is also worsened by the lack of alignment between different provinces which would, in theory, also contribute to developing overarching systems that could be adopted on a national level. However, the data shows that, in Pakistan, political affiliations still play a strong role in preventing interprovincial cooperation (Int_4, 5). In addition to external contextual factors, this compartmentalisation is also reflected internally in the module team. The data indicates that the module team is structured in two separate teams that each deal with the interventions in the SP and SHP sectors, respectively (Int_4, 5, 8). The evaluation finds that the work in silos creates little opportunity for traction between the two teams dealing with SP and SHP to find opportunities for collaboration.

The evaluation also finds that although most of the activities and interventions are adequate for the MO and flexible enough to adapt to the needs of the partners, **the design of Output 2 (research for policy evidence) remains an area in need of improvement.** The data indicates that Output 2 was too ambitious. On the one hand, some data indicates that the concept of an institutionalised research hub that clearly informs evidence-based policy-making might have been too pioneering for Pakistani actors. In particular, data suggests that the strong disconnect that exists between policy-makers, implementers and researchers is heightened by the fact that research is oriented towards long-term development issues with uncertain outcomes (Int_9, 10, 54, 20, 28). On the other hand, the implementation mode of Output 2 also shows some flaws since the contract awarded to an external consultancy firm would have been better awarded to a structure with a stronger political footing in Pakistan in order to increase the buy-in of the partners and traction of the political stakeholders to bring the research results into policy decision-making processes. The obtainment of results was further complicated by delays in the management of Output 2, which led to delays in the research being done and materialisation of results. These conditions contributed to the underachievement of this output, since the results of the research could not inform policy in the expected way. For example, the PHIMC went ahead without the results of the research. Nonetheless, the module offers another perspective of the successful adoption of the development approach in the context of Output 5. In this output, the integration of the research component around a specific topic is expected to be successfully used for policy decision-making (Int_4, 54, 67, 29).

The evaluation further points to the appropriate **multilevel approach** of the project. In the context of developing SP and SHP sectors and actors, the module has worked in a relevant way across multiple levels. First, the data shows that the module has adequately worked with actors at the federal, provincial and tehsil levels. For instance, at federal level, the module collaborated with the Ministry of Health and Poverty Alleviation, at national level with BISP, and at provincial level with SHPI and PSPA departments, as well as working at tehsil level. This multilevel approach was reflected across all of the project's outputs through activities that involved actors at federal level that have a national reach, such as BISP and ministries, combined with actors at provincial level that have the authority to make decisions at meso level, with a vision to improve the systemic preconditions necessary for the improvement of the supply and demand side of social protection.

In conclusion, the module has a solid foundation, built on a strong multilevel approach, the right targeting of sectors to answer the needs of the target groups, the deployment of relevant and timely activities to achieve their objectives, and the selection of the right partners. Some weaknesses in the traceability of the module

were observed, such as the lack of details in the results model and a centralised M&E. In addition, the evaluation also finds two weaknesses in the project's strategy, mainly regarding the missed opportunity to find more concrete synergies between SP and SHP, but also regarding the conceptualisation of Output 2 and the remaining compartmentalisation of the SP and SHP topics. Therefore, relevance dimension 3 – appropriateness of the design – scores **12 out of 20 points.**

Relevance – Dimension 4: Adaptability – response to change

This last dimension for relevance analyses whether changes in the general conditions (e.g. the political, security and regulatory context) took place, and to what extent these changes have been taken up by the module. Aligned with the assessment of other dimensions for this criterion, the evaluation examines if the adaptations reflect appropriate adjustments to the changing module conditions.

Overall, the module adapted **appropriately and successfully to the changing political and social environment**, as well as to emerging opportunities. The module demonstrated adaptability in navigating changes in political priorities, which disrupted planned activities. For instance, delays in district selection due to political turmoil were mitigated by selecting alternative districts (Int_27). These political dynamics required SP-SHP to remain flexible and responsive, enabling it to maintain its relevance in an ever-changing environment.

Furthermore, the **COVID-19 pandemic posed significant challenges to module implementation**. The module quickly responded to the pandemic by shifting from in-person to online formats for capacity-building activities through rapid adjustments, including reformatting course materials to ensure readability and effectiveness in a digital environment (Int_55). Additionally, the pandemic also prompted the digitalisation of BISP processes, which the module supported, enabling targeted cash transfers and providing the foundation for dynamic registries. These adaptive measures not only addressed immediate needs during the pandemic but also established mechanisms for more efficient service delivery in the long term (Int_4, 5).

The 2022 flood that hit Pakistan significantly altered the module's trajectory, creating an urgent need to address widespread poverty and acute socio-economic challenges. Through an adaptation of Output 4 (i.e. moving away from the single-window service approach to dynamic registry), the module played a critical role in supporting BISP's changing priorities to expand its processes and digital infrastructure to enable rapid assistance (Int_4, 9, 11; GIZ, 2022a). The introduction of dynamic registries, originally piloted during COVID-19, was scaled up to provide immediate support to flood-affected populations. Additionally, the module supported tehsil-level offices where beneficiaries could verify their status and enrol quickly. To address the demand, the module introduced female social case workers ('Ashas'), beneficiaries from the programme who acted as case workers and queue managers. Initially a short-term measure, the initiative's success led to its expansion to every Union Council, creating a sustainable support system for citizens (Int_4, 5).

Another significant shift in the module's focus was driven by **new collaboration opportunities**, such as with the GF. An adaptation to Output 5, including co-financing from GF, was made to address malnutrition, a topic intricately linked to poverty (GIZ, 2024a; Int_4, 9). The concept of Output 5 relied on combining extra conditional cash transfers for families whose daughters participate in BISP nutrition education programmes. This intervention includes nutritional supplements like iron provided in schools and incentives to send girls to school (Int_4, 8, 9).

Similarly, the module adapted to specific needs as they arose. For example, when working on registrations through the single-window services approach, it became clear that people with disabilities faced significant barriers (Int_10). To ease these barriers, **an app** was developed **that allows people with disabilities to conveniently schedule appointments** for an examination at the hospital which is needed for them to be eligible for certain social protection schemes (Int_10).

In conclusion, the project's adaptability is assessed as very good in the context of emerging needs and priorities of national and international partners. Therefore, relevance dimension 4 – adaptability – response to change scores **20 out of 20 points.**

Methodology for assessing relevance

Table 6: Evaluation matrix for OECD DAC criterion relevance

Relevance assessment dimensions	Basis for assessment	Evaluation design, empirical methods and data sources	Data quality and limitations
Alignment with policies and priorities	Extent to which the module design is aligned with key frameworks. **Pakistan's, Germany's and international strategic frameworks and sectoral documents:** • Global: 2030 Agenda, in particular, SDG 1 (No Poverty), SDG 3 (Good Health and Well-being) and SDG 8 (Decent Work and Economic Growth). • Pakistan: The seven 'pillars' of Pakistan in Vision 2025 are assigned to the goals of the 2030 Agenda. The module contributes to poverty reduction, improved access to health services (both under Pillar 1) and inclusive sustainable growth (Pillar 2). The module closely relates to the KP and Punjab Social Protection Policies of 2022. • Germany: Sector concept on Social Protection (2009); Position paper: Shaping equitable development with social protection (09/2017); Sector concept: Health in German development policy (2009); Position paper: Global health – An investment in the future (2019); Discussion paper: A systems perspective on Universal Social Protection (2019).	**Evaluation design:** The evaluation design followed the questions from the evaluation matrix. Specific areas of interest are considered in Table 1. **Empirical methods:** • Document analysis on progress reports, study reports, relevant strategic documents of the KP and Punjab provincial governments, data and documentation on NSER, and relevant accessible policies. • Interviews with module staff. • Interviews with relevant direct target groups, i.e. provincial- and federal-level SP and SHP public actors and other providers of SP-SHP programmes, as well as implementing partners.	A relevant challenge regarding data quality/reliability was that no sectoral policy developed by the country was available at the time of the evaluation due to the ongoing development of the sectors' frameworks in Pakistan. **Overall data quality for this assessment dimension:** good
Alignment with the needs and capacities of the beneficiaries and stakeholders	Extent to which the module's design is aligned with the needs of its target groups. It also considered to what extent deescalating factors/connectors and escalating factors/dividers (cf. Tables 7 and 8) are considered in the module design.	**Evaluation design:** The evaluation design followed the questions from the evaluation matrix. **Empirical methods:** • Document analysis of progress reports, study reports, relevant strategic documents of the KP and Punjab provincial governments, data and documentation on NSER, and relevant accessible policies. • Interviews with module staff. • Interviews with relevant direct target groups, i.e. provincial- and federal-level SP and SHP public actors and other providers of SP-SHP programmes, as well as implementing partners. • FGDs with indirect target groups, i.e. beneficiaries of SP and SHP programmes.	A relevant challenge regarding data quality/reliability and limitations was that access to final beneficiaries was difficult due to logistical constraints. Overall data quality for this assessment dimension: **good**

Relevance assessment dimensions	Basis for assessment	Evaluation design, empirical methods and data sources	Data quality and limitations
Appropriateness of the design	Relevance in this dimension is achieved if the module's results model and module offer adequately address the baseline conditions identified at the outset of the module and if the selected results hypotheses offer a realistic and plausible pathway towards reaching the intended objective and impact, given available resources and consideration of external influences and risks. The contribution hypotheses for this evaluation were jointly selected with the module team based on their relevance to the main results of the module and availability of data.	**Evaluation design:** To assess the plausibility of the hypotheses of the results model, the evaluation team assessed the model's fit to contextual assessments. It further analysed the synergies among outputs that were expected to lead to the achievement of the MO. **Empirical methods:** • Document analysis on progress reports, study reports, relevant strategic documents of the KP and Punjab provincial governments, data and documentation on NSER, and relevant accessible policies. • Interviews with module staff. • Interviews with relevant direct target groups, i.e. provincial- and federal-level SP and SHP public actors and other providers of SP-SHP programmes, as well as implementing partners. • Results model workshop held in June 2024.	The module proposal and modification offers, the results matrix and the module team offered detailed information on the module's theory of change. Comparing this information with the progress reports and feedback from stakeholders outside the module as well as state-of-the-art literature and hands-on experience (e.g. inputs from the local consultant) delivered good-quality information on the fit of the module design for achieving the intended results. **Overall data quality for this assessment dimension:** strong
Adaptability – response to change	Extent to which the module's results model and other steering instruments have been adapted to changing contextual factors over the course of the module, such as natural disasters and political changes.	**Evaluation design:** To assess the adaptability to change, the evaluation team compared the module's proposal, change offers and any changes to the module's central steering documents and assessed the extent to which the module adapted to changing conditions. **Empirical methods:** • Document analysis on progress reports, study reports, relevant strategic documents of the KP and Punjab provincial governments, data and documentation on NSER, and relevant accessible policies. • Interviews with module staff. • Interviews with relevant direct target groups, i.e. provincial- and federal-level SP and SHP public actors and other providers of SP-SHP programmes, as well as implementing partners. • FGDs with indirect target groups (i.e. beneficiaries of SP and SHP programmes).	The quality of data was strong due to the variety of perspectives and the possibility to triangulate data. **Overall data quality for this assessment dimension:** strong

Conflict sensitivity in the project design

Table 7: Dividers/escalating factors in the project context

Which dividers/escalating factors were identified in the project context?	Addressed by the project?	If addressed, how is it considered by the project design?
Discrimination and limited access to essential (state) services for women, ethnic and religious minorities, refugees and scheduled castes, particularly in education, healthcare and economic opportunities.	Yes	An integrated digital bidirectional communication system was developed, primarily benefiting women as BISP beneficiaries, with consideration of gender literacy differences and inclusive training for staff, along with supporting exchange opportunities for girls and women through beneficiary groups or digital platforms.
Vulnerability to climate change and related disasters. The poorer population is the most exposed group to disasters, suffering higher relative losses to their assets and livelihoods.	Yes	The module adapted its design and implementation to react to the devastating floods of 2022 by supporting the implementation of emergency aid measures and adapting technical support to changing partner needs.
Corruption, patronage structures and organised crime affect all levels of public administration.	Yes	The module supports BISP with the development of a digital backbone, which fosters accountability and creates obstacles for corruption in the social protection sectors. BISP cash transfers improve livelihoods of households and thus reduce the opportunity for criminal activities.
Digitalisation efforts crucial to accessing public services are minimised by insufficient government funds and corruption in the public sector.	Yes	The module supports digital solutions such as a bidirectional communication system between BISP and its beneficiaries, grievance redressal mechanisms, beneficiary registration systems, and applications for social protection systems.
Vulnerability to violent extremism and religious radicalisation in the regions of KP and Punjab.	No	

Table 8: Connectors/deescalating factors in the project context

Which deescalating factors/connectors were identified in the project context?	Addressed by the project?	If addressed, how is it considered by the project design?
Interest in increasing the effectiveness and efficiency of social protection programmes.	Yes	The module prioritises the improvement of supply- and demand-side conditions for the implementation of social protection programmes, particularly for people at risk of poverty.
Promotion of gender equality and access of women and girls to social services.	Yes	The module addresses the gender gap in social protection by strengthening BISP, which specifically targets women to facilitate their access to social services. It promotes the inclusive design of training for administrative staff and supports the establishment of exchange opportunities for girls and women.

4.3. Coherence

This section analyses and assesses the coherence of the project. It is structured according to the assessment dimensions specified by BMZ (see Annex).

Summarising assessment and rating of coherence

Table 9: Rating of OECD DAC criterion coherence

Criterion	Assessment dimension	Score and rating
Coherence	Internal coherence	30 out of 50 points
	External coherence	43 out of 50 points
Overall score and rating		Score: **73 out of 100 points** Rating: **Level 3: moderately successful**

The internal coherence of SP-SHP is assessed as mixed, with good potential at both the programme level and in collaboration with the financial cooperation elements. SP-SHP effectively utilised synergies with ASP (started 08/2023), within the programme (i.e. shared partners, joint events and integration of SP-SHP activities into ASP), but did not establish a systematic approach to capitalise on synergies with other GIZ-implemented modules. Furthermore, collaboration with KfW was limited, due to differing implementation approaches and lack of structured mechanisms for coordination (e.g. joint planning), leading to missed opportunities for synergy. **The external coherence of the module shows examples of good cooperation practices that have led to concrete opportunities for complementarities, with national partners and international organisations.** Collaboration with international organisations such as the WB, ILO and ADB has been largely successful, demonstrating effective coordination mechanisms (i.e. monthly agency meetings and sector groups) and alignment of objectives. It has helped strengthen social protection coordination, notably through initiatives like national conferences with MoPASS, BISP, ADB and WB, laying the foundations for the first joint SP approach in 2023, and the Cash Transfer Coordination Group, together with the ASP module in late 2023. Additionally, the module has fostered strategic collaborations with organisations like GF. The co-financing and collaboration with GF in nutritional support for adolescent girls serves as a positive example of coordinated efforts, characterised by consistent information-sharing and the effective use of synergies. Lastly, the module actively engaged in coordinated crisis response efforts. **In total, the coherence of the project is rated as moderately successful (level 3), with 73 out of 100 points.**

Analysis and assessment of coherence

The coherence criterion analyses the extent to which a development intervention is compatible with other interventions in the country, sector and institution, using the methods presented in Table 10 below.

Coherence – Dimension 1: Internal coherence

Internal coherence refers to the project's synergies and links with other German cooperation projects.

Various systems of coordination exist to reinforce the internal coherence of the module in the search for alignment and complementarity to add strategic value. **Within the GIZ DC programme Social Protection in Pakistan**, the SP-SHP has created concrete synergies with the other TC modules and with the financial cooperation. First, **the TC module – Adaptive Social Protection** (ASP) (PN: 2023.2098.4) – has leveraged the SP-SHP module's partner relations, for example with social welfare departments in KP, Punjab and MoPASS, to strengthen pro-poor social policies. The ASP module has also integrated and expanded on some of the core foci of the SP-SHP module, such as the integration of disaster management and climate-adaptive approaches in social protection systems (Int_17). This alignment ensures coordinated contributions to impact-level social protection objectives (Development Cooperation Programme Social Protection Pakistan 2023) (see also 4.5 on Impact) and represents a facilitating factor for sustainability (see also 4.7 on Sustainability). The two TC modules also joined forces to organise a social protection conference in Bhurban in 2022 and established the Cash Transfer Coordination Group to coordinate efforts with partners on cash transfer activities (Int_13). The two TC modules are currently managed by the same module lead, share technical and administrative staff, and hold joint team meetings (Int_17). The data also shows that the SP-SHP module has maintained some communication with the **financial module** within the DC programme. For example, with the FC module Health Financing II (BMZ 2013 66 228), the SP-SHP maintained regular technical exchanges and in-person meetings (GIZ, 2023). In addition, the evaluation finds that **some communication channels are used** between KfW and the SP-SHP module, such as quarterly meetings and punctual invitations to each other's activities. For example, one interviewee noted that the two agencies extended invitations to debriefing workshops (Int_67). Another example revolved around the recommendation of participants in GIZ capacity-building activities by KfW, which were taken into account (Int_4). However, this does not seem to have become common practice since in general, other interviewees noted that GIZ and KfW did not generally participate in each other's activities. However, although data indicates that both modules support the same partners in KP in

implementing and expanding health insurance schemes, there is no indication that particular efforts were provided to find synergies.

Beyond the contribution to common objectives in the context of the DC programme, **the synergies with KfW in general remain limited**. In general, the lack of concrete synergies can be explained by two factors, the first one being that not all GIZ and KfW activities are on the same scale of implementation, with GIZ being involved at different levels of the decentralised governments (see multilevel approach in 4.2 above), combining a variety of intervention approaches (policy works, capacity-building and systems development) compared to KfW (Int_20). This difference in work approach can complicate communication. The second factor is that there is no joint planning exercise for both institutions (Int_4). Despite the lack of systematic harmonisation and search for complementarities, the evaluation finds opportunities for synergies that remain to be seized, e.g. by targeting some of the same regions and supporting similar priorities of the Pakistani government for the reinforcement of its health system. For example, while the KfW financed an operation to pilot health cards in selected districts of KP and GB, SP-SHP was also supporting the entire province of KP (Int_4, 20). However, this set-up was not conducive to systematic cooperation between the projects. The lack of alignment was complicated by the policy context; implementing a finance-heavy module in a province where the policy framework was not fully established could have benefited from closer collaboration considering the policy work by the SP-SHP module in KP (Int_4). Moreover, the coordination required between the two projects was not effectively managed, as it was largely voluntary, including among consultants (Int_4). Despite the above, by **collaborating with the same partners**, KfW and GIZ have kept in close contact. For example, both organisations collaborated with Oxford Policy Management to implement capacity-building measures. In this context, both organisations put specific efforts into coordination, especially to select the topics to be addressed during the respective capacity development measures (Int_20). However, although both organisations were **involved in similar interventions,** no concrete opportunities for synergy or complementarity were sought. For example, research plays a crucial role in both organisations, but the evaluation identified potential duplication of research activities related to cash transfer systems (Int_20).

Lastly, the module was also part of several clusters in Pakistan, without leading to compounding effects for international coherence and synergies. The data reveals that significant developments have occurred within GIZ in Pakistan, particularly regarding the integration of SP into various thematic clusters. SP often spans multiple clusters due to its diverse linkages, including its recent inclusion in the climate change cluster, alongside the economic development cluster. Despite the participation of the SP-SHP module in the various cluster meetings, little noticeable effect has been observed in the data collected (Int_4, 12). Inversely, participation in multiple structures was perceived as an inefficient use of personnel. The later integration of the module into the governance cluster has yet to bring concrete examples of synergies.

In conclusion, the internal coherence of the SP-SHP is assessed as mixed, with good potential at both the programme level and in collaboration with the financial cooperation. SP-SHP effectively utilised synergies with ASP, within the programme, (i.e., shared partners, joint events and integration of SP-SHP activities into ASP) but did not establish a systematic approach to capitalise on synergies with other GIZ-implemented modules. Furthermore, collaboration with KfW was limited due to differing implementation approaches and lack of structured mechanisms for coordination (e.g. joint planning), leading to missed opportunities for synergy. Nevertheless, the evaluation identified several opportunities that remain to be seized, such as working more closely on sites targeted by both organisations, with partners common to both, and by selecting similar fields of activities (e.g. research). In addition, while the module has been integrated into multiple GIZ thematic clusters, this has not yet translated into significant strategic gains, with participation in numerous structures sometimes perceived as an inefficient use of resources. **Overall, coherence dimension 1 – internal coherence – scores 30 out of 50 points.**

Coherence – Dimension 2: External coherence
This dimension considers other actors' interventions in the same context, and addresses complementarity, harmonisation and coordination with others, as well as the extent to which the intervention adds value while avoiding duplication of efforts. A key aspect of external coherence concerns the coordination between the intervention and other donors' activities.

The module **complemented partners' activities, such as BISP.** The data indicates that **the module participates in coordination mechanisms developed by partners**. On the one hand, through participation in institutionalised mechanisms like BISP's monthly agency meetings involving WB, United Nations and ADB, the module ensures alignment and avoids duplication of efforts while fostering complementarity with other donors in the efforts to support BISP (Int_57). Additionally, sector group meetings, such as those focused on health and social security, further support the coordination of development actors and enhance the module's coherence (Int_57). In this context, for example, the evaluation underlines the role of GIZ and UNICEF in creating the Cash Transfer Coordination Group, hosted at BISP, which brings together 15 development partners, including WB, UNICEF and the World Food Programme, along with governmental agencies like MoPASS (Int_4, 11). However, the evaluation uncovered a potential risk to the effectiveness of BISP-led coordination processes due to changes in BISP personnel (Int_57). The module also participates in another key communication channel: the national-level steering committee, led by the Ministry of Health, which includes participants from GIZ, WB, the Foreign, Commonwealth and Development Office, and World Health Organization (WHO) (Int_5). On the other hand, efforts to align activities are not limited to mechanisms established by BISP but also include bi-annual collective meetings involving both financial and technical actors in Pakistan (Int_57).

The module was also found to be aligned with other donors and international projects. First, in the health sector, the module is aligned with WB and ADB. GIZ has emerged as a leading actor in the health sector in Pakistan, a position that has led other donors and international organisations to consult with them before launching their own projects (Int_5). One key example is the launch of a capacity-building series by WB for which SP-SHP was invited to deliver lectures on health financing challenges. While ADB and WB also provide technical advisory services, similarly to GIZ, their roles have limited overlap. WB and ADB hold a donor and monitoring role and develop KPIs, whereas GIZ is focused on implementation and project development (Int_3). To ensure better complementarity, GIZ initiated a dialogue with WB and included them in a workshop with other international developmental actors (Int_6, 14). A key achievement in the context of social protection coordination was GIZ's organisation of a national conference for MoPASS with ADB and WB, to increase outreach efforts to their own partners. The conference, attended by senior-level representatives from all invited organisations, concluded with the drafting of a 10-point declaration, co-authored by BISP and GIZ, marking a significant step forward in aligning efforts between the participants (Int_4). Another important collaboration of the SP-SHP module has been **with ILO**, in the context of their support to SSP. In this context, ILO was responsible for conducting specialised actuarial work for SSP, while GIZ facilitated the process by providing project funding and ensuring effective communication with SSP (Int_4). This collaboration highlighted the complementary strengths of both organisations, with ILO bringing in its expertise in actuarial analysis in the pursuit of GIZ's MO.

In addition, the evaluation identified examples of closer collaboration between international organisations during crises (e.g. pandemics and floods). The data shows that GIZ has actively collaborated with other development partners in response to these challenges. For instance, during the COVID-19 pandemic, GIZ provided technical advice and conducted clinics jointly with SPACE and the Foreign, Commonwealth and Development Office (Int_9, 10).

Lastly, within the framework of Output 5, cooperation with GF highlights a strategic approach to enhancing coherence by aligning nutrition-focused interventions for adolescent girls with existing social protection systems in Pakistan. Communication was initially informal and irregular but has transitioned to structured bi-weekly

meetings to enhance coordination (Int_65). While challenges such as cash flow management and integrating partner organisations with limited GIZ experience remain, the collaboration reflects a significant effort to align the SP-SHP with GF's expertise in nutrition, creating synergies that promise to improve outcomes for vulnerable populations in Pakistan (Int_65).

In conclusion, the external coherence of the module shows examples of good cooperation practices that have led to concrete opportunities for complementarities, with national partners and international organisations. The collaboration with other international organisations such as WB, ILO, Foreign, Commonwealth and Development Office, and ADB has also been largely successful, demonstrating effective coordination mechanisms (i.e. monthly agency meetings and sector groups) and alignment of objectives. It has played a leading role in strengthening social protection coordination, notably through initiatives like the Cash Transfer Coordination Group and national conferences with MoPASS, ADB and WB. Additionally, the module has fostered strategic collaborations with organisations like GF. The co-financing and collaboration with GF in nutritional support for adolescent girls serves as a positive example of coordinated efforts, characterised by consistent information-sharing and the effective utilisation of synergies. Lastly, the module actively engaged in coordinated crisis response efforts. Points are deducted in light of the risks of staff turnover for the continuity of the coordination platform hosted at BISP and the organisational challenges that materialised during the collaboration with GF. Overall, **coherence dimension 2 – external coherence – scores 43 out of 50 points.**

Methodology for assessing coherence

Table 10: Evaluation matrix for OECD DAC criterion coherence

Coherence assessment dimensions	Basis for assessment	Evaluation design, empirical methods and data sources	Data quality and limitations
Internal coherence	The extent to which the module was planned to complement and then deliberately complemented other German development cooperation interventions (or vice versa) thereby (i) achieves greater development outcomes and impacts than these interventions could achieve on their own; and/or (ii) realises efficiency gains, i.e. the development results are achieved with less resources than would otherwise be needed. Other GIZ projects: • Predecessor: Social protection including social health protection (PN 2015.2186.3) • ASP in Pakistan (PN 2023.2098.4) • Developing and integrating social protection systems (PN 2017.2045.7) KfW projects: • Health financing in Pakistan, II & III (PN 2013.6622.8 and PN 2017.6730.0) BMZ programme: • DC Programme Social Protection in Pakistan	**Evaluation design:** To assess this dimension, the evaluation team mapped the objectives of other projects with a view of analysing potential synergies, overlaps and trade-offs. The evaluation team assessed the module's objective and implementation regarding GIZ's governing principles and standards. This second step was implemented as a cross-cutting topic across all evaluation criteria. **Empirical methods and data sources:** • Review of documents from selected other interventions, including project proposals, as well as of the programme proposal. • Interviews with module staff. • Interviews with staff of other relevant projects from the German DC.	The quality of data was strong due to the variety of data and the already existing synergies that facilitated access to potential interview partners. **Overall data quality for this assessment dimension:** strong

Coherence assessment dimensions	Basis for assessment	Evaluation design, empirical methods and data sources	Data quality and limitations
External coherence	The extent to which the project considered other actors' interventions in the same context, and addressed complementarity, harmonisation and coordination with others, as well as the extent to which the intervention added value while avoiding duplication of efforts (in particular assessing the use of existing systems and structures, in particular for Monitoring, Evaluation, Accountability and Learning (MEAL) purposes). It is understood to have been achieved if the module does not duplicate efforts of other actors' interventions and if potential synergies are realised. According to the subsidiary principle, the module shall not replace the work of the partner institutions. Other key actors in social protection: • WB • ADB • GF	**Evaluation design:** The analysis follows the evaluation questions from the Annex; triangulation of methods and sources (see below). **Empirical methods and data sources:** • Analysis of websites and documents of development partners. • Semi-structured interviews with project management and staff of development partners.	No challenges. **Overall data quality for this assessment dimension:** strong

4.4. Effectiveness

This section analyses and assesses the effectiveness of the project. It is structured according to the assessment dimensions specified by BMZ (see Annex).

Summarising assessment and rating of effectiveness

Table 11: Rating of OECD DAC criterion effectiveness

Criterion	Assessment dimension	Score and rating
Effectiveness	Achievement of the (intended) objectives	30 out of 30 points
	Contribution to achievement of objectives	26 out of 30 points
	Quality of implementation	18 out of 20 points
	Unintended results	17 out of 20 points
Overall score and rating		Score: **91 out of 100 points** Rating: Level 2: successful

In dimension 1, the evaluation confirms that all four MO indicators are on track for full achievement by the end of the module in September 2025: new processes were successfully implemented (e.g. developing guidelines, updating hospital panel selection criteria, and standardising family unit definitions); beneficiary knowledge of SHP programmes improved significantly; KP provincial authority developed its first social protection strategy; and dynamic NSER registrations had reached 16.9 million households by October 2024. In dimension 2, the evaluation finds that the module used the right strategies and mechanisms to obtain its results. The operationalisation of communication strategies contributed to increasing the knowledge beneficiaries on SHP programmes (H1). Further, by supporting BISP in establishing 647 tehsil centres and introducing mobile registration vans, the module contributed to improved beneficiary registration and information-sharing in social protection (H3). Finally, the contribution analysis also indicates that the module contributed to improved evidence-based decision-making and institutional capacities but faced challenges (gap in the level of buy-in of evidence-based decision-making in Pakistan and high staff turnover). In dimension 3, the data indicates that the quality of implementation of the module is generally high; relying on strong steering mechanisms and a collaborative approach ensured strategic alignment and effective interventions. Its multilevel partner selection

reinforced structural strength, inclusivity and cooperation. However, GIZ's heavy bureaucratic processes caused delays in implementation despite well-managed administrative procedures. Finally, dimension 4 demonstrates that the module had unintended positive results, notably a data-sharing agreement between the Balochistan Health Card Programme (BHCP) and BISP, influenced by its work in KP. It also addressed emerging needs by developing an app to assist people with disabilities in accessing social protection and incorporating sexual harassment awareness into its training. While monitoring of unintended results was fragmented, the module adapted to new opportunities. **In total, the effectiveness of the project is rated successful (level 2), with 91 out of 100 points.**

Analysis and assessment of effectiveness

Effectiveness – Dimension 1: Achievement of the (intended) objectives

The assessment of effectiveness dimension 1 examines whether the module **achieved** its objectives on time and in accordance with the MO indicators (outcome indicators) agreed upon in the inception report. Table 12 outlines the assessment of the project's outcome indicators, including an assessment of whether the indicators can be considered specific, measurable, achievable, relevant and time-bound (SMART).

Table 12: Assessed project objective and objective indicators (outcome level)

Project objective according to the (last modification) offer: 2024		
Project's objective indicator according to the (last modification) offer	**Assessment according to SMART criteria**	**Operationalisation of indicator in evaluation** (esp. if necessary for measurement or understanding)
OI 1: Three core health insurance processes were adapted on the basis of evidence-based recommendations. Base value (May 2020): 0 Target value: 3 Current value as assessed by evaluation team (January 2025): 3 Achievement in % as assessed by evaluation team (January 2025): 100% Source: analysis of SOPs and contractual documents with regard to adjustments.	**Specific:** Yes. Note that the results matrix specifies both core processes (drafting of contracts with healthcare providers, patient intake, complaint and feedback mechanisms) and evidence-based recommendations (those based on analysis of monitoring data and/or applied research). **Measurable:** Yes. **Achievable:** Yes. **Relevant:** Yes. **Time-bound:** Yes. While the module ends in September 2025, the target was set for December 2024 and thus its achievement can be assessed at the time of the evaluation as most of the module implementation towards this indicator should be concluded. As a milestone, official draft documents of SOPs with a plan of adoption are also considered for two of the three processes. **The indicator is SMART but for the evaluation a milestone has been set.**	
OI 2: The proportion of the target group in three selected districts who know the benefits and processes of the SSP increased from 20% to 65%. Base value (modification offer): 20% Target value: 65% Current value as assessed by evaluation team (January 2025): 87% Achievement in % as assessed by evaluation team (January 2025): 100% (138%).	**Specific:** Yes. **Measurable:** Yes. In principle this is measurable; however, the selection of interviewees and thus the representativeness of statistics is unknown. **Achievable:** Yes. **Relevant:** Yes. **Time-bound:** Yes. **The indicator is SMART.**	Note that the baseline value reported in the modification offer differed from the one reported in the endline assessment. Therefore, the target was adjusted in line with the same percentage point increase as in the modification offer. Achievement is measured as change relative to the baseline.

Project objective according to the (last modification) offer: 2024		
Project's objective indicator according to the (last modification) offer	**Assessment according to SMART criteria**	**Operationalisation of indicator in evaluation** (esp. if necessary for measurement or understanding)
Source: preliminary results of the endline assessment of the communication campaigns based on baseline and endline surveys.		OI 2: The proportion of the target group in three selected districts who know the benefits and processes of the SSP increased from 25% to 70%. Base value (January 2025): 25% Target value: 70% Current value as assessed by evaluation team (January 2025): 87% Achievement in % as assessed by evaluation team (January 2025): 100% (138%). Source: preliminary results of the endline assessment of the communication campaigns based on baseline and endline surveys.
OI 3: A provincial government has developed a strategy for the development of an integrated social protection system based on the evaluation of the BRS and taking into account the legal framework, financing issues, the necessary IT structures and gender equality. Base value (May 2020): 0 Target value: 1 Current value as assessed by evaluation team (January 2025): 1 Achievement in % as assessed by evaluation team (January 2025): 100% Source: analysis of strategic documents of the provincial governments in KP or Punjab.	**Specific:** Yes. **Measurable:** Yes. **Achievable:** Yes. **Relevant:** Yes. **Time-bound:** No; while the target was set to May 2024 in the most recent module offer, it also suggests that this is expected to be achieved by June 2025. To account for this, a milestone is set to allow this indicator to be fulfilled if an official draft of a strategy document fulfils the conditions. **The indicator is SMART but for the evaluation a milestone has been set.**	
OI 4: The number of people registered in the new dynamic NSER for benefits under BISP and other social protection programmes has risen from 6 million to 22 million. Base value (January 2023): 6 million households Target value: 22 million households (milestone for the evaluation: 16.5 million households) Current value as assessed by evaluation team (October 2024): 16.9 million Achievement in % as assessed by evaluation team (October 2024): 68% (milestone) (104%) Source: evaluation of the NSER data with regard to the number of people eligible for benefits from BISP and other programmes of the households registered for social protection.	**Specific:** Yes. **Measurable:** Yes. **Achievable:** Yes. **Relevant:** Yes. **Time-bound:** No; the target is expected to be reached in September 2025 and is not expected to be reached within the evaluation period. Thus, a milestone was set at 16.5 million households based on a linear approximation of progress. Note that currently there is some uncertainty about the baseline value, such that both baseline and milestone values might need to be adjusted. **The indicator is SMART but for the evaluation a milestone has been set.**	

As shown in Table 12, the evaluation team concludes that all four MO indicators are expected to be fully achieved by the end of the project.

MO indicator 1, which measures the adaptation of core health insurance processes, was **fully achieved (100%)**. Achievement of this is based on analysis of contractual and supporting documents from the SHP

programme and SLIC shared by the module team. The data first shows that the insurer, SLIC, developed operational manuals (Int_62). Second, the hospital panel selection criteria were updated for the federal SSP and assessment forms were developed. These assessments are used to ensure that hospitals covered by the SHP programme meet necessary criteria to provide quality healthcare. Third, the definition of the unit of family was standardised and aligned between SLIC, NADRA and the federal SSP to facilitate the SSP, as misaligned definitions caused delays in reimbursements (Int_8). Finally, more generally, the reinforcement of the SHPI data management and monitoring tool in KP supports the SHPI to adapt its processes based on real-time admission and claims data (see effectiveness dimension 2 for more details).

MO indicator 2, which measures the knowledge of beneficiaries of the SHP about these programmes, was also **fully achieved (138%)**. This indicator is evaluated based on an endline assessment of the communication campaigns conducted by NRSP (HSA, 2025). The survey measured the respondents' knowledge on 14 items[1] and categorised respondents as well informed if they indicated knowledge on 40% of the items. Using this indicator, the endline survey results show that while an average of 25% of respondents were well informed at baseline, a total of 87% were well informed at endline in the districts in which the NRSP communication campaigns were conducted. This is true across most items, in particular regarding eligibility (except for SMS codes to check eligibility), documentation requirements, and coverage (except for coverage of health facilities). The reported baseline value for this indicator (25%) differs from the baseline value according to the modification offer (20%). Therefore, the evaluation team based the assessment of the MO indicator achievement on the target percentage point change outlined in the modification offer. This target was a 45 percentage point increase, but the actual change from baseline to endline is considerably higher at 62 percentage points. This resulted in an overachievement of this MO indicator of 138%.

MO indicator 3 measures whether a provincial government developed a strategy for the development of an integrated social security system based on the evaluation of the BRS, which was **fully achieved (100%)**. There are two relevant provincial strategies to consider for this indicator: the Punjab Social Protection Policy 2022 and Khyber Pakhtunkhwa Social Protection Policy 2022. In particular, the strategy for KP mentions GIZ's contribution in its acknowledgements section, though without reference to the BRS. Further, the strategy identifies the lack of integration and legal frameworks, the lack of unified databases of programmes and beneficiaries, and funding as key challenges. The strategy aims to address these challenges through policy principles aligned with recommendations from the BRS evaluation, thereby indicating the link to the evaluation of the BRS as intended by the MO indicator. For the strategy in Punjab, there are fewer, if any, indications of a contribution from the module as intended by the results matrix. Therefore, this strategy is not counted towards this indicator.

MO indicator 4, which measures registrations to the NSER, is currently **partially achieved (68%)**, though it fully achieved the **milestone** set for the evaluation (104%). Considering the long remaining duration of the module relative to the timing of the evaluation mission, with almost one year remaining, a milestone was set for this indicator. The baseline value for the indicator from January 2023 was 6 million households registered with the NSER according to the change offer. The target was set to 22 million households corresponding to 16 million additional households registered over a 33-month period by September 2025. Assuming progress towards this indicator is constant, by October 2024, the time of the evaluation mission, the evaluation team expected about 16.5 million households to be registered. On this basis, the evaluation finds that the MO indicator was fully achieved, with 16.9 million households registered in October 2024, according to information received from BISP during the evaluation mission. If progress towards this MO indicator continues, it is **expected to be fully achieved by the end of the module**. While progress towards registrations might slow down since the people most in need will register early and there are fewer and fewer non-registered households, measures are planned and due to be implemented to improve accessibility of registration to counteract any potential slowing down in progress. For instance, the module introduced mobile registration

[1] The 14 items include: whether they are eligible, coverage portability, methods to check eligibility, checking remaining balance, documentation required for eligibility, documentation required for treatment, SMS codes to check eligibility, financial coverage, package coverage, service coverage, which facilities are covered, coverage of family members, who generally is eligible, and owner of the programme.

vans to support the registration of particularly hard-to-reach populations which has already resulted in the registration of 400,000 households (Int_11). Further, the GIZ sister module ASP, building on the results of the SP-SHP module, continues to work on improving access by expanding the registration from the administrative level of tehsil to the Union Councils (Int_11). This expansion will decrease the distance to the registration centres and thereby further improve access to the registry and contribute to the achievement of this indicator.

In conclusion, effectiveness dimension 1 – achievement of the (intended) objectives – scores **30 out of 30 points.** Points are based on the average (expected) achievement of MO indicators.

Effectiveness – Dimension 2: Contribution to achievement of objectives

To assess effectiveness dimension 2, the evaluation team analysed how the project's activities and outputs contributed to the attainment of the MO (more information on the output level is available in the section on production efficiency). As outlined in Table 16, a contribution analysis was used to assess the dimension. As a methodological guideline, the evaluation team worked in accordance with the six steps established by John Mayne (2001). Together with the module team, hypotheses linking output and outcome levels were selected for closer examination (WS_1) and are listed in Tables 13, 14 and 15. The hypotheses were selected based on the team's interest and (expected) availability of data, with the aim to cover different areas of the module's activities and results. Hypotheses 1 and 2 focus on the demand- and supply-side strengthening of the SHP programmes, whereas hypothesis 3 focuses on support for another key actor in the direct target group: the social protection programme BISP.

Table 13: Effectiveness – Assessment of results hypothesis 1

Results hypothesis 1 (activity – output – project objective)	Through the operationalisation of a communication strategy for awareness raising on performance assessment (R2, R3), the social health protection programme actors have greater capacities to administer and deliver the national health insurance scheme (Output 1). This should contribute to improving the familiarity on the benefits and processes of the SHP programmes of a greater proportion of the beneficiaries (R16) and thus contribute to improving their capacities to demand social protection programmes (MO).
Main assumptions and necessary context factors	• SSP provides sufficient financial and human resources for the nationwide implementation of the communication strategy. • The communication strategy is effective.
Risks and unintended results	• Despite increasing uncertainties in the issuing of visas to international experts, counselling and implementation processes took place without significant delays. • Despite the Pakistani government's focus on increasing the reach of health insurance, issues of service quality and effectiveness of SSP remain on the political agenda. • The institutional stakeholders lack confidence to raise the awareness of the target groups to their entitlements (e.g. as an increased utilisation of services would imply larger costs).
Alternative explanation(s)	• KfW, other donors and government initiatives at federal and provincial level as well as word-of-mouth are expected to contribute to the familiarity of the beneficiaries with the SSP's benefits and processes.
Assessment of hypothesis	Confirmed

The first results hypothesis is **confirmed** based on the evidence.

First, the data confirms that the module **contributed to the operationalisation of a communication strategy through the implementation of communication campaigns**. The communication campaigns were implemented by partner NRSP in five target districts[2] between March 2022 and December 2023, together with the social health protection programmes (NRSP, 2024). The module team and NRSP worked out the right communication channels and strategies to run the communication campaigns through advertisements (e.g.

[2] The selected districts of Mardan, Abbottabad, Muzaffarabad, Kotli and Gilgit include districts from both KP and the federally administered territories, thereby informing both SHP programmes.

banners or billboards, mobile floats, announcements through mosques, street theatres, and awareness sessions in villages and Union Councils) (Int_8; NRSP, 2024).

Second, through the communication campaigns, the baseline and endline survey data shows that **the module contributed to greater familiarity of beneficiaries with the benefits and processes**. As discussed under effectiveness dimension 1, there has been a large increase in the knowledge and awareness of beneficiaries within the districts in which the communication campaigns took place. Compared to the baseline estimates that 25% of beneficiaries were knowledgeable, there was a 62% increase in the share of knowledgeable beneficiaries in intervention districts (HSA, 2025). By the endline, the share of beneficiaries who know about benefits and processes of the SHP programmes had more than tripled. Comparing the intervention districts to districts in which the intervention was not implemented, the endline assessment further shows that the increase in knowledge between baseline and endline was significantly more pronounced in intervention districts. In comparison districts, the share of knowledgeable beneficiaries is estimated at around 55%, compared to 87% in intervention districts (HSA, 2025). This strongly indicates that the communication campaigns contributed to the observed increase in knowledge among beneficiaries.

Continuing to build on these results, **the module is expected to finalise a more comprehensive communication strategy** (OP 1.1) together with the SHP programmes **to further contribute to the knowledge of beneficiaries at a larger scale** (Int_8). The willingness-to-pay study conducted under Output 2 already indicates an increase in the willingness-to-pay from communication activities (Int_5). Using the evidence collected through the endline assessment, the module will be further enabled to inform a broader communication strategy based on the evidence of what went well and the associated costs (Int_8). In addition to informing the communication strategy, the module contributed to enabling its sustained implementation. Through discussion and consultations, the module supported a PC-1[3] which should institutionalise communication within the SHP programmes by including a 10% addition to the premium for the purpose of communication activities, ensuring that these can continue beyond the activities implemented by the module (Int_5).

Through increased knowledge of the beneficiaries, the module contributes to the demand-side conditions for the implementation of the SHP programmes. The SHP programmes can only achieve their impacts if its beneficiaries use the insurance to cover the costs of their inpatient treatments. Therefore, utilisation of the programmes is a prerequisite for impact. However, from the beginning of their implementation, the utilisation of the SHP programmes was below expectations assumed in the planning stages of the SHP programmes (Int_8), and independent analyses found that the use can be considered low compared to different benchmarks, for instance from similar programmes in other countries (ILO, 2022; Stelter et al., 2022). In its evidence-based approach, the module therefore conducted two baseline assessments to identify key barriers for accessibility and utilisation of the SHP programmes including an assessment of the beneficiaries' awareness and knowledge of the benefits and processes (Int_8; PIDE, 2021; IMSciences, 2021). These baseline assessments found low awareness and knowledge among beneficiaries and identified these as barriers to accessibility and utilisation (Int_8; PIDE, 2021; IMSciences, 2021). Therefore, the module is expected to contribute to the demand-side conditions for the implementation of the SHP programmes through operationalisation of a communication strategy based on evidence from successful communication campaigns implemented by the module.

[3] Government spending for projects in Pakistan is typically requested and approved through PC-1 forms. In this case, this means that with the approval of the PC-1 form, the government formally agreed to the increase in premiums for the purpose of communication efforts. Note that the provincial and federal governments finance the SHP programmes by paying the premiums of the enrolled population.

Table 14: Effectiveness – Assessment of results hypothesis 2

Results hypothesis 2 (activity – output – project objective)	Through the development of an adaptable concept for M&E, based on KPIs and a dashboard (R7), together with the development of capacities on the system (R8), the social health protection programme actors have better capacities to analyse the data of the SHP programme beneficiaries and to integrate their social health insurance with other SP programmes (R9). This should contribute to improving evidence and improve the coverage of needs at a provincial and federal level (Output 2). The evidence generated should be used for strategic dialogue and planning (R27). This therefore contributes to the improvement of the supply conditions for the implementation of social protection programmes (MO).
Main assumptions and necessary context factors	• Decision-making processes within the SHP programmes, e.g. for the adoption of the communication strategy and the M&E system, are sufficiently efficient to enable the relevant outputs to be utilised during the module term. • The Ministry of Health is creating guidelines and SOPs for the development of an M&E system, supplemented by proposals for the qualitative improvement and system-wide harmonisation of complaints and feedback mechanisms. • The module needs to adopt a flexible approach to answer changing needs. • Technical staff capacities for the implementation and maintenance of the M&E system and dashboard are available.
Risks and unintended results	• Heavy bureaucracy and staff turnover can delay the implementation and the results.
Alternative explanation(s)	• Other donors and government initiatives at federal and provincial level could also contribute to some of the results expected by the module.
Assessment of hypothesis	rather confirmed

The second result hypothesis is **rather confirmed**. To assess the module's contribution to the improvement of supply-side conditions for the implementation of social protection programmes through improving evidence use for strategic dialogue and planning, the analysis starts by outlining the module's support in terms of: (i) monitoring tools based on KPIs, (ii) capacity development, and (iii) the research hub. Then key risks and unsatisfied assumptions as challenges to the contribution of the module efforts are discussed. The analysis then concludes with discussion of the improvements in evidence-based decision-making and the module's contribution.

First, the data confirms that the **SP-SHP module has contributed to the development of an adaptable concept for M&E, based on KPIs and a dashboard, which increased the capacities of SHP programmes to analyse the data of its beneficiaries**. The SP-SHP module supported the SHP programmes and the implementing insurance company, SLIC, with their **data management** processes, a monitoring tool based on KPIs, which provides real-time admissions and claims data, and a system for complaints to facilitate evidence-based decision-making. As a result, the SHP programmes are fully digitalised, and the module supported with IT consultants to facilitate data analysis and reporting (Int_13, 36).

In addition, the module supported the SHP programmes and practitioners **with capacity development** including the use of monitoring data and evidence-based decision-making. In its approach, the module's capacity development for SHP and SP implementers was aimed at mid-level managers who are both hands-on involved in the implementation but also influential in decision-making processes (Int_4, 8). This was successful as the Civil Service Academy (CSA) has included the courses introduced by the module in the curriculum for all mid-level managers who move to more senior levels (Int_4). The courses provided by SP-SHP, with over 200 participants over the years, filled a gap in the educational system, and new topics are continuously being added as the need for additional capacity-building arises (FGD_1; Int_8, 46). Starting out by introducing the concept of SHP and health insurance, in recent years the module added courses on data management, financial sustainability and health system stewardship to prepare for current and future challenges (FGD_1; Int_8). In addition to strengthening the SHP programme's capacities, the module also strengthened the capacity of the HSA who became an increasingly important institution in the capacity development of SHP (Int_46). Further, the module organised multiple exposure visits to showcase examples of successes and challenges towards UHC and SHP in various countries, which both introduced new ideas and facilitated decision-making to adopt best practices (Int_8, 20, 29, 36, 62).

Lastly, beyond specific results of the KPI-based monitoring system and related capacities, the data indicates that the module contributed, to a certain extent, to **improving evidence-based decision-making through the research hub** (Output 2). Relative to its ambitious goal of institutionalising a research hub which links policy-makers and SP implementers with researchers, this output was unsuccessful (Int_4, 16; WS_1). However, it resulted in some successful matchmaking between SP implementers and researchers, resulting in research projects that fostered dialogue between these parties (WS_1; Int_54). While dialogue on the political level generally remained a challenge in most research projects, some research studies sparked interest among stakeholders (Int_54). While there was no research hub, one of the participating institutions continued to regularly publish a call for policy research, resembling some features of the research hub (Int_28, 54). Still, a shift to more evidence-based decision-making and an institutionalised research hub is a longer-term project, well beyond the module's horizon (Int_16). Nevertheless, the module successfully produced four research studies (OP 2.2) and while the dissemination faced challenges, there were discussions of findings with decision-makers which constitute first steps towards more evidence use in strategic dialogue and planning.

Evidence-based decision-making remains a key gap in Pakistan and the efforts of the module therefore faced **several barriers** for furthering this goal (for details, see Section 4.2). Further, a **key challenge for the capacity development is high staff turnover** (Int_5, 20). If staff changes, then capacity can dilute (Int_20, 26). Compared to the health sector, especially SHPI in KP with long continuity of the same staff, this is more challenging in the social protection sector (Int_5). While the module's courses with the HSA are visited by the SHP implementers, other actors, such as government officials in other ministries or institutions, reportedly still have limited understanding of SHP (FGD_1).

Overall, despite challenges**, the module contributed to improvements in** terms of **evidence-based decision-making** among key actors. During module implementation, an increasing demand for research from policy-makers could be observed (Int_54). For example, in KP different stakeholders were brought together to identify their most pressing needs which resulted in a policy-oriented research agenda for social protection (Int_9, 10, 54). While progress might be limited, especially the capacities in terms of evidence-based decision-making, the SHPI in KP has improved and actors were steered in the right directions (Int_20, 67). During an FGD, SHP implementers for instance explained how they try to implement their learnings from the courses to develop a more financially sustainable approach (FGD_1). Through the module's support, the SHP programme in KP was enabled to analyse the insurance data in real time, and the data analysis supported by the module became a continuous practice in two steering structures to improve decision-making (Int_38). Similarly, the module improved the data management in Punjab, resulting in more accuracy which is key for their health programme monitoring (Int_29). Through the module, GIZ is recognised as a key actor strengthening the SHP institutions across Pakistan through technical expertise, support in digitalisation, the generation of evidence, and institutional capacity development (Int_46, 62). In conclusion, there are indications of the module's contributions to the improvement of supply-side conditions for the implementation of social protection programmes through improving evidence use for strategic dialogue and planning, but there are significant barriers such that the hypothesis is assessed as being only rather confirmed.

Table 15: Effectiveness – Assessment of results hypothesis 3

Results hypothesis 3 (activity – output – project objective)	Through the support to establishing (mobile) registration centres, the module should contribute to improving the efficiency of the communication of BISP with its beneficiaries by increasing its reach (Output 4). In addition, this should support the registration of beneficiaries into the dynamic NSER under BISP and other social protection programmes (R33). The greater registration should contribute to improving the connection and information-sharing between supply and demand sides in social protection programmes and ultimately improve the response to the needs (R34, MO).

Main assumptions and necessary context factors	• By adopting a transformative approach, the module ensures that conditions for registration are enhanced by strong engagement from the site supervisors and contact persons for the target groups. • The ability of target group members to use the BISP communication system is not restricted by drastic price increases for telecommunications. • The financing of dynamic registration in permanent registries is secured in the long term. • The political priorities for the implementation of BISP will not change significantly. • International development partners continue their support for BISP. • The registration is accessible to the eligible population which often face barriers due to distance and time.
Risks and unintended results	• Degradation of the security situation in the programme provinces and districts.
Alternative explanation(s)	• Other donors and government initiatives at federal and provincial level could also contribute to some of the results expected by the module.
Assessment of hypothesis	confirmed

The third results hypothesis is **confirmed** based on the evidence.

To assess the project's contribution towards the connection and information-sharing between social protection programmes and beneficiaries, first the extent to which the module contributed to the dynamic registration with BISP's NSER needs to be assessed. As discussed in Section 4.6 for OP4.2, the module supported BISP with setting up 647 tehsil centres across the country in which beneficiaries can now register with the NSER. Without this support, BISP likely would not have been able to implement the dynamic registry at this scale and that efficiently (Int_11, 40). Since the centres cover a smaller population in sparsely populated areas, the module further introduced mobile vans to facilitate the registration of hard-to-reach populations in Balochistan (Int_11). Through these mobile vans alone, about 400,000 households were registered by the time of the evaluation (Int_11). Overall, there are strong indications that the **module's support has contributed to the registration of 10.9 million additional households within the dynamic NSER**, as measured by MO indicator 4 compared to its baseline (see effectiveness dimension 1).

The module's support in this context was multifaceted to maximise results. First, it provided resources to furnish the centres. Second, beyond providing financial resources, the module directly hired the staff in the initial phases of its support. This not only facilitated the hiring process, which otherwise could be expected to be longer (Int_11, 40), but it also introduced the idea of hiring BISP beneficiaries for service provision at the tehsil centres (Int_4, 15). The idea of hiring female BISP beneficiaries was well received, as it not only supports the beneficiaries in potentially graduating out of poverty but also has the potential to improve access through staff being familiar with the programmes and barriers faced by the beneficiaries (Int_4). This is key, as the tehsil offices are the first point of contact between BISP and its beneficiaries (Int_11). For the communication with beneficiaries, there is a dedicated complaints manager at the tehsil centre to register complaints, and BISP introduced a dedicated call centre through which beneficiaries can make complaints which are then transferred to the headquarters (Int_40, 11). Third, it provided support to the digitalisation and IT infrastructure to manage the registration system (Int_11). One example of digitalisation improving accessibility is the introduction of a mobile app through which registered households can update their contact information and address (Int_11). Fourth, the module facilitated an agreement between BISP and NADRA such that the barrier of a missing birth certificate or expired Computerised National Identity Card (provided by NADRA) can be simultaneously addressed in the mobile vans (Int_11). Finally, the registration was simplified compared to the old NSER based on household surveys (Int_9, 10). The dynamic NSER is now based on the family as registered with NADRA instead of the household and a lot less information is gathered during the registration process (Int_11). While this makes the data more reliable and no longer requires the collection of information on household members, it can present a challenge for people not registered with NADRA (Int_11). At the same time, it provides an incentive for families to obtain birth certificates and register their children or themselves (Int_11).

The dynamic registration with NSER was a key development towards improved capacity for the supply side of social protection to respond to the needs of the population. The COVID-19 pandemic and the floods in 2022, which caused the shift towards the dynamic registry for BISP, are examples of the need for emergency aid in crisis situations which can be better facilitated with a more dynamic register of families in need (Int_4, 15, 16). The dynamic registry provides a better basis for the targeting of social protection programmes as it can be dynamically updated whenever the needs of the population change (Int_11, 23). Further, the dynamic registry is more digitalised with improved data management, and it is more up to date compared to previous versions of the NSER. One example and an additional result of the module is the data-sharing agreement between BISP and BHCP, (see effectiveness dimension 4).

In conclusion, effectiveness dimension 2 – contribution to achievement of objectives – scores **26 out of 30 points.** Points were deducted since not all hypotheses were confirmed and while the first hypothesis is confirmed, the module's full contribution is only expected since the operationalisation of the communication strategy is yet to be seen.

Effectiveness – Dimension 3: Quality of implementation

This dimension addresses the appropriateness of the project's chosen strategies, processes, cooperation approaches and steering structure for the achievement of its outputs.

In terms of **steering**, the module established strong structures **to steer the SP and SHP team** in an effective way to facilitate the results. As such, weekly team meetings were held with all SP and SHP colleagues including finance and contract positions, complemented by needs-based additional communication (Int_8). As such, the good steering of the module is illustrated in the continuous work of the team in the face of successive adjustments to the module concept and interventions, including supporting **learning and innovation** in the module (Int_4, 5). The successive adjustments led to developing innovative interventions, such as linking SP with nutrition (Output 5), supporting the deployment of mobile vans (Output 4), and the introduction of Moodle (an online learning platform) for HSA to offer courses (Output 3), but sometimes they were too innovative, like the research hub which fell short of achieving its highly ambitious results (Output 2) (Int_4, 11, 62). The weekly team meetings were also intended to be used to ensure coherence between the SP and SHP teams in the module (Int_8). However, as highlighted in Section 4.2, more connections should be made between the two teams leading the respective SHP and SP interventions. Also, the data shows some weaknesses in the steering mechanisms with implementing partners (see Section 4.6).

In terms of **strategy**, **the module has also developed and was involved in strong steering structures** to maintain an inclusive and consultative approach with its external stakeholders. With implementing and direct political target groups, the module has set up steering committees that hold regular periodic meetings to address strategic alignment of activities and share information on developments (Int_8, 9, 11, 16, 55). Partners further noted that the involvement of the module team in high-level steering meetings (i.e. with ministers) was appreciated for its approach focused on technical expertise in a setting where decision-making is more usually led by a political approach; it was also commended for its agility, responsiveness and flexibility to answer the needs of the partners (Int_15, 16, 28, 55). In addition, the module organises a yearly planning workshop with all module partners and external stakeholders (e.g. people in academia who have the capacity to design and deliver some of the module interventions) to identify key priorities of the partners and the extent to which the module can plan interventions to answer them (see also Section 4.2 on the analysis of the module's response to target groups' needs) (Int_8, 9, 15, 28). Joint efforts with partners to develop a solid strategy are also **reinforced by GIZ's long-standing reputation and historical expertise in Pakistan's social protection sector**. By leveraging its deep-rooted relationships, technical knowledge and adaptability, GIZ has contributed to establishing solid foundations for the module to continue advancing social protection in Pakistan with comparatively lower funds than other actors in the field (e.g. WB) (Int_4, 5, 12).

In terms of **cooperation**, the evaluation finds that the module's multilevel approach to select its partner and implementing partners (see also Section 4.2) has facilitated the availability of results at outcome and impact level, by fostering inclusivity, leveraging contextual strengths, and promoting provincial and federal collaboration. The first strength of the cooperation is the **selection of implementing partners with strategic footing** and networks in the SP and SHP sectors, and with key expertise for the implementation of interventions. For example, NRSP was chosen due to its strong community mobilisation footprint and access to key stakeholders, ensuring smooth implementation and facilitating linkages with health and social welfare departments (Int_27). Similarly, the decision to collaborate with HSA as a government training provider not only addressed capacity gaps but also contributed to the sustainability of social health protection training systems (Int_46, 4). In addition, GIZ's flexible approach to cooperation with partners such as Oxford Policy Management ensured the inclusion of emerging priorities like ASP in response to stakeholder needs (Int_55). Disparities in capacities were nonetheless noticed between partners and between regions (Int_29). For example, some implementing partners needed to improve their management skills to fulfil their role, such as SDPI in the research hub. The data explains that this is due to the fact that SP-SHP are not competence areas for SDPI and therefore required more time than anticipated to acquire (Int_4, 9). Second, understanding the fragmentation between the decentralised levels of public administration**, the module collaborated with political partners at federal and provincial levels**, which facilitated results at output, outcome and impact level. At the national level, collaboration with BISP was significant for the advances in the SP sector on a national scale (e.g. by adopting the dynamic registry and the ASP model) (Int_2, 3, 4, 66). However, tensions between national, federal and provincial programmes highlight structural challenges. For instance, some provincial governments can view national programmes like BISP as overlapping with their initiatives (for example, in creating their own provincial registry systems), which requires the establishment of mechanisms to strengthen coordination between different systems (Int_2, 7, 66).

In terms of **processes**, the data indicates some good practices to facilitate the results but also some areas for improvement, primarily discussed in Section 4.6. The data indicates that two specific mechanisms supported the module to obtain results. First, the decision to **use the central GIZ administrative unit**, named Regional West Bureau, contributed to effectively managing the contract and partners for all of Output 5, co-financed by GF. Second, by striking a **balance between international and localised service providers**, the module strived to ensure that the contents of its interventions were tailored to the local context of SP-SHP in Pakistan (e.g. course contents) while also circumventing the recurring issue of fixed local contracts to enable the formation of a qualified team to conduct the interventions. In addition, the data reveals that owing to the long-standing support of GIZ to Pakistan's SP-SHP sectors, some local organisations have remained as trusted partners of the successive GIZ projects. This trust was illustrated, for example, by the continuous support provided to strengthening HSA, making it into an institution with established departments and trained faculty in place (Int_4).

In conclusion, the evaluation finds that the strong mechanisms developed by the module for the steering of the SP and SHP module team and for a collaborative approach with its partners supported strategic alignment with the needs and design of appropriate interventions to obtain the expected results at output, outcome and impact level. Furthermore, the module's multilevel approach to select its partner and implementing partner further reinforced the strong structures in place by fostering inclusivity, leveraging contextual strengths, and promoting provincial and federal cooperation. Despite the implementation of good processes to facilitate the administrative and financial management of the module, interview partners underlined the heavy bureaucratic processes of GIZ that caused delays in the implementation of the module activities. Overall, effectiveness dimension 3 – quality of implementation – scores **18 out of 20 points.**

Effectiveness – Dimension 4: Unintended results

In terms of unintended positive or negative direct results, the **most remarkable positive result** is the **data-sharing agreement** between the province of **Balochistan** and BISP for the purpose of the implementation of

BHCP (Int_4, 5, 8, 9, 11, 17, 62, 66, 68). While the module was not directly working with the province of Balochistan, this achievement can be linked indirectly to the module's efforts through its support to SHPI in KP and BISP. As such, the achievements in KP sparked interest from Balochistan and the existing engagement of the module with BISP facilitated the cooperation between BISP and BHCP. Given the evident interest in seeking support to adopt the project's successful approach in Balochistan, the module also facilitated the deployment of mobile registration vans in Balochistan, through the intermediary of BISP (Int_4, 8, 9). This is a key result and in line with the module's objectives but unintended insofar as the province of Balochistan was not part of the module's scope. This example further showcases the module's adaptability and ability to seize opportunities to support social protection actors across Pakistan (see also Section 4.2).

In Balochistan, the module further demonstrated its adaptability by addressing specific regional challenges. The province's health insurance programme sought to replicate successful models from Punjab and KP but faced hurdles due to unavailability of the necessary NSER data. SP-SHP, in collaboration with BISP, deployed mobile registration vans to update beneficiary data and link health insurance systems with social protection mechanisms. This effort enabled better integration of services and ensured that beneficiaries could access treatment while updating their eligibility records simultaneously. Lessons learned from other regions were applied to create linkages between BISP, NSER and the provincial health programme, culminating in a data-sharing agreement that strengthened service delivery in Balochistan (Int_8).

However, while some module components were successful in including a mechanism to monitor adverse effects, such as the implementation of Output 5 through Nutrition International (Int_12), the module's overall monitoring can be described as fragmented and **unintended results were not systematically monitored**. The examples above illustrate positive examples of how the module mitigated risks and unintended negative results which had positive outcomes, bringing attention to these issues despite a lack of systematic monitoring.

Overall, effectiveness dimension 4 – unintended results – scores **17 out of 20 points** given the successful unintended effects, even though there was no systematic monitoring of unintended results.

Methodology for assessing effectiveness

Table 16: Evaluation matrix for OECD DAC criterion effectiveness

Effectiveness assessment dimensions	Basis for assessment	Evaluation design, empirical methods and data sources	Data quality and limitations
Achievement of the (intended) objectives	Effectiveness is understood as the extent to which outcome indicators are fulfilled. During the inception phase, the evaluation team has analysed whether the module's four objective indicators have met the SMART criteria. Three objective indicators are not time-bound; one other might not be but is expected to be time-bound. The indicators are primarily not time-bound as the evaluation is being conducted before the module's conclusion. Therefore, milestones are set for the indicators that are not time-bound based on a linear approximation of expected progress and by lowering the requirement for achievement (from adopted SOPs or policies to drafted but not yet adopted/approved SOPs or policies).	**Evaluation design:** The analysis followed the analytical questions from the evaluation matrix (see Annex). The module's own monitoring data is thereby subject to a plausibility assessment and data triangulation by the evaluation team. **Empirical methods:**	The data quality is good, as the quality of the indicators and their data sources are good. The key challenges and limitations lie with the availability of relevant data. **Overall data quality for this assessment dimension:** good

Effectiveness assessment dimensions	Basis for assessment	Evaluation design, empirical methods and data sources	Data quality and limitations
		• Document analysis on the latest results matrix (2024), progress reports, module monitoring data available, SOPs and contractual agreements of SHP programmes (if available), baseline and endline survey reports, study reports, relevant strategic documents of the KP and Punjab provincial governments, and data and documentation of NSER registrations. • Secondary data analysis of baseline and endline data if data but no reports are available. • Interviews with module staff. • Interviews with relevant stakeholders (SHP programmes, ministries of health, NSER, PIDE).	
Contribution to achievement of objectives	This looks at the extent to which the contribution of the activities and outputs to the achievement of the intended objective are demonstrated in the analysis. Following the participative approach of the evaluation, the selected results hypotheses have been selected together with the project team during the inception phase to make sure that the hypotheses are relevant for the module. The following results hypotheses were selected for a contribution analysis: • The improvement of communication strategies contributes to greater familiarity of the target groups with the SHP (hypothesis 1). • The operationalisation of integrated MIS relying on an adaptable concept for M&E contributes to increasing the evidence available for decision-making on SHP (hypothesis 2). • The establishment of static registration centres supports the deployment and use of the dynamic NSER of BISP to communicate with its target groups (hypothesis 3).	**Evaluation design:** A contribution analysis is used to analyse the extent to which observed (positive or negative) impacts can be related to the intervention. This analyses the extent to which the intervention has contributed to the observed results. The data sources listed below are used to examine causal hypotheses between inputs, outputs, outcomes and impacts in the results model and to construct a 'contribution story' to show whether the intervention was a relevant factor, possibly together with other (context) factors, for change. **Empirical methods:** • Document analysis of progress reports, SOPs and contractual agreements of SHP programmes (if available), baseline and endline survey reports, study reports, relevant strategic documents of the KP and Punjab provincial governments, and data and documentation of NSER registrations. • Secondary data analysis of baseline and endline data if data but no reports are available. • Interviews with module staff. • Interviews with relevant stakeholders (SHP programmes, ministries of health, NSER).	Data quality was good since a large part of the activities have been implemented and allowed for observable results. Nevertheless, considering the module will finish in September 2025, some activities were still ongoing. The results of the activities were therefore observable at the time of the evaluation. For these ongoing activities, the assessment of effectiveness, impact and sustainability was therefore based on a plausibility analysis. **Overall data quality for this assessment dimension:** good
Quality of implementation	Quality of implementation is understood to have been achieved if the module's steering decisions and employment of instruments align with the module's objectives. This includes:	**Evaluation design:** Quality of implementation was assessed as a cross-cutting theme throughout the evaluation and was discussed as part of the contribution analysis. As such, the evaluation team assessed the appropriateness of the module's chosen strategy, deployed instruments, cooperation approach, and steering structure for the realisation of its outputs. **Empirical methods and data sources:**	A relevant challenge regarding data quality/reliability was that some key documents (e.g. steering structure) were unavailable. **Overall data quality for this assessment dimension:** moderate

Effectiveness assessment dimensions	Basis for assessment	Evaluation design, empirical methods and data sources	Data quality and limitations
	• the extent to which the module team actively and effectively steered towards the achievement of the MOs, including (i) regular context and results analysis; (ii) timely responses to module and context developments, (iii) making use of synergies with other interventions; (iv) using the M&E system; and (v) involving all stakeholders, and • the extent to which the quality of implementation contributed to the success of the module. The analysis is structured according to the Capacity WORKS success factors (mandatory elements: stakeholder map, capacity development strategy, steering structure, operational plan) and other important factors as well as integration of co-financing/linkages.	• Document analysis of operational planning documents and progress reports. • Interviews with module management and staff as well as implementing partners.	
Unintended results	The assessment of this dimension is based on unintended results identified over the course of the evaluation. The module is understood to have operated effectively in this regard if positive unintended results were seized upon and negative unintended results were mitigated by the module. This includes the extent to which the module applied conflict-sensitive module management and monitoring (especially of risks and unintended effects).	**Evaluation design:** Unintended results were assessed iteratively throughout the evaluation process. Specifically, the analysis relied on findings across the impact and sustainability dimensions to assess whether additional unintended results occurred. Potential trade-offs among the intervention's dimensions (e.g. economic, social, ecological) was also considered. **Empirical methods:** • Document analysis, e.g. of the integrated peace and conflict assessment. • Interviews with module staff and direct target groups.	The module did not systematically monitor unintended results which limited the data availability. Overall data quality is expected to be moderate and not all unintended effects were identifiable. **Overall data quality for this assessment dimension:** moderate

4.5. Impact

This section analyses and assesses the impact of the project. It is structured according to the assessment dimensions specified by BMZ (see Annex).

Summarising assessment and rating of impact

Table 17: Rating of OECD DAC criterion impact

Criterion	Assessment dimension	Score and rating
Impact	Higher-level (intended) development changes/results	28 out of 30 points
	Contribution to higher-level (intended) development results/changes	35 out of 40 points
	Contribution to higher-level (unintended) development results/changes	30 out of 30 points
Impact score and rating		Score: **93 out of 100 points** Rating: Level 1: highly successful

At impact level, the data finds indications of positive trends at structural level in all impact areas. This includes greater access to SP and SHP programmes for people, establishment of dynamic registration tools and improved governance of the SP and SHP programmes in Pakistan, although the impact at the scale of the

entire country will need time to materialise. In addition, the module plausibly contributed to these changes, as both hypotheses show that the module used effective mechanisms to develop a system for social protection that can be adjusted, therefore improving the coverage and benefits for vulnerable populations for better protection (hypothesis 4) and to improve the interlinkages between provinces and with the federal government (hypothesis 5). Nevertheless, little information is available on the improvement of the protection of vulnerable households at the beneficiaries' level, apart from greater access to SP-SHP programmes, since changes are still relatively recent and not yet visible. In addition, the evaluation finds no further negative unintended impacts but one positive one was that the results of the module were used to further improve access to expanded healthcare (e.g. neonatal intensive care units, cardiotocography machines and Cath Labs). **In total, the impact of the project is rated highly successful (level 1), with 93 out of 100 points.**

Analysis and assessment of impact

Impact – Dimension 1: Higher-level (intended) development changes/results

Under impact dimension 1, the extent to which the intended overarching development results have occurred or were foreseen was analysed. The main impact areas targeted by the module are derived from the revised results matrix. Given the long chain of impact and the mid-term character of the CPE, the module's results are assessed through plausibility analysis, recognising that full potential will only be realised over time and is difficult to predict at this stage.

In the impact area, **more people have received social protection, or the extent of their social protection has been improved (R38),** the module was expected to contribute to BMZ markers GG 1 and AO 2. Data from the national SP programmes indicates an increase in the financial capacities of SP programmes and an increase in the number of people receiving social protection. In this case, the government's flagship social safety net programme displays the largest, to date, annual budget of PKR 360 billion (1 billion EUR) (0.58 percent of GDP) and coverage of 9 million households in 2023 (Guven et al., 2024a). Observations from the (evaluation) mission align with this trend since the data indicates that the institutional, technical and technological capacities of BISP have improved. For example, BISP headquarters now benefit from a data centre that was instrumental in assisting BISP transitioning from a periodic paper-based household survey to a digital and dynamic registry, namely NSER, increasing the reach to target groups (Int_40). In addition, reinforcement of NSER has also led to additional spin-offs, primarily in generating demand for digitalisation and leveraging technology to strengthen organisational processes. For example, BISP is developing a prototype for a cloud-based MIS for users of NSER data, which is expected to be piloted in collaboration with the provinces to assess its effectiveness before a potential scale-up. It is also converting its programmes, such as Waseela-e-Taleem (conditional cash transfer for education programme targeting very poor members of the population), into this cloud framework, marking a transformational shift to digitalisation and technology by a government agency. It is expected that the increase in accessibility for target groups already observed after the digital transformation of NSER will continue with the spin-offs (Int_40). Nonetheless, while the data suggests that social protection programmes are increasing their financial capacity and expanding coverage, recent policy shifts raise concerns about long-term sustainability. For example, the Prime Minister's National Health Programme, a health microinsurance scheme providing subsidised medical care to low-income families, covered 42 million families before being phased out in 2023, with only KP continuing this in 2024 (UNDP, 2024). Similarly, the Mera Pakistan Mera Ghar Programme (G-MSP), which supported affordable housing, was suspended in 2022 (Coleman et. al, 2024). These developments suggest fluctuating political priorities, which may challenge the consistency of social protection expansion.

In the impact area, **instruments for the dynamic participatory adjustment of social protection are sustainably established (R39)**, the module was expected to contribute to the DAC/BMZ identifiers 'poverty orientation' (AO-2). Data from the international studies indicate that they have established and use instruments for the dynamic participatory registry, which is used for the adjustment of social protection programmes. As such, data shows that NSER's utilisation has increased in the past few years with over 30 programmes and

institutions reportedly relying on this data for identification and targeting (Guven et al., 2024b) and currently covers over 35 million families, centralising data on household demographics and other socio-economic indicators that are used to determine eligibility for benefits (Int_40). Data from the evaluation mission also confirm this trend considering that since the development of NSER, the BISP has gained efficiency in dynamic inclusion, considering access to registration remains open, continuous and needs-based. This is perceived as a critical element for ASP systems responding particularly to climate crisis induced emergencies/shocks. The evaluation data further notes that BISP was able to further extend the dynamic NSER to tehsil level (sub-district) following the 2022 floods, strengthening the registration process through operationalising Dynamic Registration Centres in 647 tehsil offices and mobilising staff resulting in the registration of 16.9 households and 82.3 million individuals (Int_40).

In the impact area, **the federal and provinces social programmes are functionally linked with each other and with disaster management and climate change adaptation in a sustainable way (R40)**; furthermore, the module expected to contribute to BMZ marker PD/GG 1. In the absence of national statistics on the interprovincial coordination, the assessment is based on the observations collected during the evaluation mission. In this case, the data indicates some positive developments vis-à-vis collaboration and linkages between the provinces as well as federal and provincial institutions in a historically fragmented social protection ecosystem in the country. In this context, there are examples of cross-provincial learnings for programme design of the BHCP by arranging interaction with other provinces to exchange experience. The exchange enabled BHCP to design its social health protection programme for long-term sustainability by funding it through the non-development budget after reviewing the models at the federal and provincial levels. By leveraging on lessons from other provinces' experience, Balochistan became the first province to conduct a comprehensive baseline. Similarly, BHCP used the actuarial analysis of the SSP to estimate utilisation rates for its programme, demonstrating an example of shared learning and innovation in social protection policy-making (Int_4). Other examples of greater coordination include the NADRA and NSER collaboration. Through the development of the unified Business Rules for Beneficiary Identification, NADRA is providing verification services to BISP by leveraging the Computerised National Identity Card system and linking it with NSER. This collaboration with NADRA ensures that BISP's database remains accurate and complete and ultimately supports effective targeting by addressing coverage gaps (Int_7). Lastly, a coordination mechanism, the Pakistan Intersectoral Coordination Committee, was also established at MoPASS with a view to enabling collaborative and sustainable inter-institutional and provincial collaboration, enhancing the long-term efficacy of SP initiatives. At this stage in the evaluation, however, it is not possible to gauge its effective utilisation given the short time the mechanism has been set in place (Int_41).

While efforts to improve coordination between federal and provincial social protection programmes exist, gaps remain in linking them to disaster management and climate adaptation. The data shows that some initiatives aim at improving linkages between social programmes and disaster risk management, like the Crisis Resilient Social Protection Programme and the Pakistan Resilient and Accessible Microfinance Project, which aims to strengthen social protection by enhancing financial resilience to climate shocks. The project plans to establish a Climate Risk Facility to enable microfinance providers to offer recovery loans to those affected by floods and other climatic disasters (Zaheer, 2025; Wells et al. 2023). This was also confirmed by the evaluation data since the sister module ASP also focuses on integrating disaster management in social protection (Int_4, 17). However, accessibility remains limited. Uncoordinated provincial investments in Sindh, lack of emergency credit for climate-affected farmers in Punjab, and the absence of crop insurance in KP hinder effective support (Wells et al., 2023). Additionally, only 34% of farmers in semi-arid Punjab hold bank accounts, restricting access to compensation and adaptation measures (Wells et al., 2023). These challenges suggest that, while institutional collaboration is progressing, the functional integration of social protection with climate adaptation remains inconsistent, limiting its effectiveness in addressing climate risks sustainably.

In the impact areas**, social protection programmes use information and joint planning for climate adaptation and disaster management (R41), and the poor have better access to needs-based, climate-**

resilient and affordable health services (R42); the module was also expected to contribute to BMZ markers PD/GG 1 and AO 2. Secondary data on the joint planning of social protection and disaster management and the expectation that poor people have better access to affordable health services provide some limited indications of changes. On the one hand, some data suggests that, in KP the UHC led to reduce out-of-pocket expenditures (Waqas et al., 2024). This is also confirmed by the evaluation data. The whole (officially registered) population of the provinces of KP, Punjab and the federally administered territories are eligible for the SHP programmes. By covering expenses of inpatient services in empanelled hospitals, poor people have improved access to health services. Since 2015, the social health protection programme has directly benefited 12 million patients for whom healthcare expenses were (partially) covered, including 30,000 cancer patients and 70,000 dialysis patients (Int_16). In this context, the data demonstrates that the programme now handles the daily uptake of 125,000 admissions as well as expediting processes by introducing an e-claims system. On the other hand, other sources indicate the impact is limited by the low utilisation (Stelter et al., 2022). While there is no consistent data available yet on whether an increase has occurred or not, one study and the evaluation data already argue that awareness might be a barrier that needs to be resolved (see also Section 4.4) (Stelter et al., 2022). The successful implementation of SHP programmes has paved the way for sustainable social protection by changing political perspectives and thus generating political willingness to invest in such initiatives. Observations from the evaluation mission align with this trend considering the significant development of institutional capacity and strengthening of the SHP programmes was noted as a key condition to improving the coverage of the Pakistani government's flagship social health insurance initiative (Int_36). The evaluation data also indicates that the SHP programmes are now showcased as a model, challenging the policy perspective where large-scale social protection initiatives were deemed unfeasible and presenting policy-makers with demonstrable evidence of successful implementation of a health insurance programme with a significant national footprint (Int_36).

In addition, the digitalisation of the KP Zakat system to a fully digital platform was also a key factor to improving beneficiary targeting and reducing inefficiencies. An example illustrating improvement in targeting efficiency supported by accurate data availability was shared whereby the department reportedly used the digital platform data to identify households with more than three daughters eligible for the Jehaz Allowance (Marriage Assistance Grant). Other digitalisation efforts have brought about structural changes such as the Social Welfare Department creating an online referral and disability certification system, linked with NADRA for verification. Noting reductions in the lead time for certificate issuance as well as increased accuracy and transparency due to NADRA verification, the department officials reported that 82,000 certificates in KP had been issued through the online system (Int_20).

Second, data shows that the increased access to registrations and the planned deployment of additional ushers and coordinators have expanded the BISP services to reach remote areas, therefore enhancing service accessibility (Int_11, 17, 40). In addition, increased registration into SP programmes and NADRA also contributed to expanding accessibility of services.

Lastly, the impact area of **improved governance in Pakistan (R43)** was expected to contribute to BMZ marker PD/GG 1. Analysis of this dimension builds on the results in other impact areas, i.e. the positive evolution observed in terms of registration capacities (see results linked to R38 above), digitalisation transition (R41 and R42 above) and interlinkages and cooperation (R40 above) since they all support this positive development. Secondary data provides limited accounts of systematic large-scale changes in terms of improvement of the governance in Pakistan. However, some anecdotal evidence can be found, notably through the implementation of the UHC programme in KP. Here, one study finds that A Policy Board, as the oversight and governing body of the Social Card Programme, was established subsequent to passage of the UHC Bill 2022 and organisational structures have been put in place for the management of the programme (Sameen et al., 2024). In addition, evaluation findings also align with the improvement of the governance of the SP-SHP programmes in Pakistan, although limited to the direct target groups of the module. First, there are indications that the introduction of technical courses (SP and SHP courses) in national institutions like CSA and HSA have

reinforced the foundational knowledge of SP and SHP practitioners and decision-makers (Int_4, 41). The integration into the curriculum of national institutions also sets the foundation to continue building a critical mass of skilled practitioners within government systems (Int_4, 41). Second, there are indications that the reinforced institutional capacities of the provincial and federal governments have led to stronger decision-making processes. For example, the reinforced government institutions have now developed Pakistan's first-ever Social Protection Policy and a draft social protection strategy in KP which sets out the governance roadmap for social protection programming in the province (Int_20). Nonetheless, well-functioning governance in the long term remains subject to overcoming issues of financial re-allocation to the provinces, mistrust and political priorities.

Overall, most of the points are attributed to this dimension since there are some indications of positive trends at structural level, such as greater access to SP and SHP programmes, establishment of dynamic registration tools and improved governance of the SP and SHP programmes in Pakistan, identified in the selected impact areas. Some points are deducted since only anecdotal evidence can be found, especially in terms of governance. Overall, impact dimension 1 – higher-level (intended) development changes/results – scores **28 out of 30 points.**

Impact – Dimension 2: Contribution to higher-level (intended) development results/changes

For this dimension, the evaluation team assessed whether and how the intervention contributed to the intended overarching development results, through the analysis of two impact hypotheses. These were selected with the module team to reflect the MOs of (1) improved dynamic registration and (2) improved coordination between provinces. The assessment took the form of a plausibility analysis.

Table 18: Impact – Assessment of results hypothesis 4

Results hypothesis 4 (project objective – impact)	By supporting the dynamic tool for registration with NSER, which should serve as the basic registry for all SP programmes and for some adaptive instruments for different risks (R34), the module should contribute to improving the supply and demand conditions for the implementation of SP programmes (MO). In turn, the module also contributes to sustainably establishing instruments for the dynamic participatory adjustment of SP (R39). The module should therefore contribute to improving the coverage and benefits for SHP, thus contributing to better protection of vulnerable households against health risks (programme objective).
Main assumptions and necessary context factors	• The use of the systems and processes lays the foundation for the development of strategies to better reach the target groups, including with a focus on target groups that are active in the informal sector and have so far been insufficiently recorded by many SP programmes. The improved system will also help to reduce inclusion and exclusion errors.
Risks and unintended results	• The financing of dynamic registration in permanent registries is secured in the long term. • The ability of target group members to use the BISP communication system is restricted by drastic price increases for telecommunications. • Degradation of the security situation in the programme provinces and districts. • Significant changes in political priorities. • Fragmentation of the SP-SHP sectors. • Competing systems are developed, further reducing opportunities for coordination and synergies.
Alternative explanation(s)	Other donors and government initiatives at federal and provincial level could also contribute to some of the results expected by the module.
Assessment of hypothesis	confirmed

The evaluation can confirm hypothesis 4 since the data shows that the project contributed to sustainably developing instruments for the dynamic participatory adjustment of SP. However, at the time of the evaluation,

no data was yet available on the effects of the instruments on the increased protection of vulnerable households.

First, as explained in Section 4.4, the data confirms that **by establishing (mobile) registration centres, the module contributed to improving the efficiency of the communication of BISP with its beneficiaries and the number of beneficiaries being registered in the dynamic NSER** under BISP and other SP programmes. The data also confirmed that **the greater registration numbers contributed to improving the connection and information-sharing between supply and demand sides in SP programmes**.

The greater familiarity of the beneficiaries is facilitated by two mechanisms: on the one hand, the mere participation in module activities informs beneficiaries (Int_4), and on the other hand, for those not directly participating in the module activities, the module also effectively supported the communication efforts of SHP programmes since beneficiaries now have greater knowledge (see Section 4.4). Secondly, the data also shows that public institutions have integrated the registration system into their own institutions (see also Section 4.7), thus facilitating the comprehensiveness and reliability of data (Int_4, 11). For example, the data gives an example of the complementarities observed between NADRA and NSER since, if mandatory, beneficiaries need to have Computerised National Identity Cards to be considered at the BISP tehsil office (Int_4). This was further facilitated by the intensive advocacy effort that led to the waiver of NADRA birth registration fees in Sindh, with similar initiatives underway in Balochistan (Int_4, 13). In addition, the sustainability of the instruments is also expected to be ensured by the institutional capacities developed at the level of ushers and coordinators, which is further continued through the GIZ module ASP, now under the name ASP coordinators (Int_17). The data further confirms that **beneficiaries and institutions now have sustainably easier access to update the dynamic registration instruments. This provides a strong basis of updated data for the dynamic participatory adjustment of social protection. Nevertheless**, at the time of the evaluation, **not enough data was available on the improvements in relation to the response to the needs**. As highlighted during the evaluation, the effects of the efficient two-way communication on the adaptation of the SP programmes to answer beneficiaries' needs are yet to be seen. The data shows that quick adaptation to the 2022 floods to support the deployment of the rapid registration system led to a short implementation period that could allow for observable changes in the improvement of protection of vulnerable beneficiaries in the timeframe of the evaluation.

Third, **the data also indicates that there are some promising examples of increased coverage in the SP and SHP actors, in terms of service expansion, use of data for decision-making and strengthening of registration systems**. On the one hand, the data shows that the increased access to registrations and the planned deployment of additional ushers and coordinators have expanded the BISP services to reach remote areas, therefore enhancing service accessibility (Int_11, 17, 40). In addition, increased registration in SP programmes and NADRA also contributed to expanding accessibility of services. For instance, the data indicates that the waiver of fees for birth registrations led to a significant increase in birth registrations through the NSER process (Int_4, 13). On the other hand, the data also shows that stakeholders perceive an improvement in the analysis and use of SP and SHP data for strategic decision-making. For example, a decision support system, developed through the mapping of disease incidence, now aids in more effective planning and decision-making (Int_36).

Lastly, there is one positive example of the extent to which the greater registration and availability of data allows for greater adaptation to risks. For example, through the module support, the number of Cath Labs on the Sehat Card Plus panel increased from three in 2016 to 12 in 2025. This demonstrates the programme's ability to identify service gaps and address them effectively through targeted initiatives. The programme then catalysed demand for healthcare services, prompting private-sector hospitals to establish facilities in underserved areas. This expansion aligns with an evidence-based understanding of regional needs while simultaneously improving the quality of services provided by public hospitals. One of the programme's notable achievements has been encouraging a shift from home births to hospital births, previously limited by financial

constraints and access. This change, enabled by data-driven resource allocation, is expected to contribute to improved maternal and infant health outcomes, with the potential to reduce overall mortality rates in the long term (Int_13).

The data shows that the module has contributed to developing a system for SP that can be adjusted, therefore improving the coverage and benefits for vulnerable populations.

Table 19: Impact – Assessment of results hypothesis 5

Results hypothesis 5 (project objective – impact)	By developing the data-sharing agreements within provinces and with the federal government (R27), the module should contribute to improving the supply and demand conditions for the implementation of SP programmes (MO). In turn, the module expects to contribute to functionally linking the federal- and province-level social programmes with each other and with disaster management and climate change adaptation in a sustainable way (R40). The module should therefore contribute to improving the governance of SP, thus contributing to better protection of vulnerable households against health risks (programme objective).
Main assumptions and necessary context factors	• The use of the systems and processes lays the foundation for the development of strategies to better reach the target groups, including with a focus on target groups that are active in the informal sector and have so far been insufficiently recorded by many SP programmes. The improved system will also help to reduce inclusion and exclusion errors. In addition, the production and use of operational knowledge is also expected to lead to uptake from decision-makers and the establishment of nationwide guidelines and instructions.
Risks and unintended results	• Degradation of the security situation in the programme provinces and districts. • Degradation of climatic conditions beyond the capacities of local actors. • Significant changes in political priorities.
Alternative explanation(s)	Other donors and government initiatives at federal and provincial level could also contribute to some of the results expected by the module.
Assessment of hypothesis	rather confirmed

With regards to impact **hypothesis 5** (see Table 19), which focused on improved governance of SP, the data can **rather confirm** the hypothesis.

First, **the data confirms that support for establishing data-sharing agreements has laid the groundwork for improved governance of SP in Pakistan** by fostering collaboration and initiating systemic changes. In this context, a first example of the contribution of the module to the consolidation of the foundations for the improvement of the governance of SP in Pakistan is the facilitation of agreements between SHP programmes in Balochistan and Punjab; these exemplify progress in building interoperable systems that align provincial and federal priorities (Int_9, 10, 17). These agreements not only enhance targeting and reduce duplication but also foster trust among stakeholders, also demonstrated by early successes like linking BISP with the Balochistan health card, through the ASP module (Int_17). A second example is the integration of the NSER and NADRA registries. For example, as demonstrated above in hypothesis 4, the data indicates that the support for a dynamic social registry represents a critical step in integrating SP and disaster management, despite resistance and mistrust over data handling capacities (Int_9, 10). While barriers such as inadequate data governance frameworks and privacy concerns remain, the foundational agreements and capacity-building activities suggest that improved governance structures are emerging, laying the groundwork for future enhancements in protecting vulnerable households. **Nonetheless, at the time of the evaluation, there are no systematic indications that the improved governance has led to improved protection of vulnerable households.**

In conclusion, hypothesis 4 demonstrates that the module has contributed to developing a system for SP that can be adjusted, therefore improving the coverage and benefits for vulnerable populations for better protection. In addition, the other mechanism supported by the module at the core of hypothesis 5, namely the sustainable improvement of the interlinkages between provinces and with the federal government, is also confirmed.

Nevertheless, no changes in the improved protection of vulnerable households could yet be confirmed at the beneficiaries' level, therefore leading to a deduction of some points. Impact dimension 2 – contribution to higher-level (intended) development results/changes – scores **35 out of 40 points**.

Impact – Dimension 3: Contribution to higher-level (unintended) development results/changes

Impact dimension 3 looks at the extent to which positive or negative unintended effects occurred at impact level. This included an assessment of how the module dealt with such effects, eventually turning them into additional results.

The findings also highlight indirect contributions and unanticipated positive impacts of its interventions and engagement with various SP and SHP stakeholder institutions. The evaluation respondents particularly highlighted the impact of support to the social health protection programming on the overall quality and accessibility of health services across the country, especially evident in the scaling up of healthcare infrastructure. Notable examples included the increase in the number of Cath Labs from only three in the entire province of KP in 2016 to 12 in 2025 on the Sehat Card Plus panel. Similarly in Punjab, the number of hospitals offering neonatal intensive care units, cardiotocography machines and Cath Labs has surged, indicating social health protection initiatives as a key driver in expanding and improving healthcare infrastructure and quality of provision.

In conclusion, considering no negative unintended impacts were found and one positive one was, i.e. improved access to expanded healthcare (e.g. neonatal intensive care units, cardiotocography machines and Cath Labs), impact dimension 3 – contribution to higher-level (unintended) development results/changes – scores **30 out of 30 points**.

Methodology for assessing impact

Table 20: Evaluation matrix for OECD DAC criterion impact

Impact assessment dimensions	Basis for assessment	Evaluation design, empirical methods and data sources	Data quality and limitations
Higher-level (intended) development changes/results	Main impact areas derived from updated results model and DC programme: • More people have received SP, or the extent of their SP has been improved (R38). • Instruments for the dynamic participatory adjustment of SP are sustainably established (R39). • The federal- and province-level social programmes are functionally linked with each other and with disaster management and climate change adaptation in a sustainable way (R40). • SP programmes use information and joint planning on climate adaptation and disaster management (R41). • The poorest-of-the-poor households have better access to needs-based, climate-resilient and affordable health services (R42). • The module strengthens the governance in Pakistan (R43). Other impact areas • Qualitative assessment of the module's application of the leave no one behind principle. • BMZ markers, namely GG 1, PD/GG 1 and AO 2. • Analysis of the core problems of the indirect target group(s) under the criterion of relevance (cf. Section 4.2).	**Evaluation design:** To assess this dimension, the evaluation team focused on the impacts according to the updated results model. In this regard, the evaluation team established the state of higher-level (intended) development changes. **Empirical methods and data sources:** • Analysis of official statistical data and international data regarding SP and social health protection and access to health. • Document analysis of module and programme progress reports (pending) and results matrix. • Semi-structured interviews with direct target groups at national and local level, and with SP experts. • FGDs with indirect target groups.	Analysing higher-level development changes/results necessitates consideration of external influencing factors. Finally, only limited data on the coverage and use of SP was available. **Overall data quality for this assessment dimension:** good

Impact assessment dimensions	Basis for assessment	Evaluation design, empirical methods and data sources	Data quality and limitations
Contribution to higher-level (intended) development results/changes	Hypotheses selected for examining the contribution of the outcome (MO) to the achievement of overarching development changes (impact-level) relate to the following: • By enhancing the NSER to be the core registry for SP programmes and adapting it for various risks, the module contributes to improving implementation conditions, establishing dynamic participatory social protection instruments, and ultimately increasing SHP coverage and benefits to better protect vulnerable households against health risks. • By developing data-sharing agreements between provinces and the federal government to sustainably link social programmes with disaster management and climate change adaptation, the module contributes to enhancing social protection governance and improving the protection of vulnerable households against health risks.	**Evaluation design:** The evaluation of this dimension mainly draws on the results from the contribution analysis examining the module's impact from outcome to impact. Section 4.4 includes a more detailed description of this approach. Due to the limitations outlined to the right the assessment consists of a plausibility analysis. **Empirical methods and data sources:** See dimension 1, plus: • websites of other development partners and their modules (e.g. EU, UNICEF, Agence Française de Développement, WB), • semi-structured interviews with representatives of other donors, and/or national/local civil society, • expert interview with SP expert from academia, and • debriefing and validation workshop with module team.	See limitations in dimension 1, also applicable for this dimension. **Overall data quality for this assessment dimension:** good
Contribution to higher-level (unintended) development results/changes	This looks at the extent to which positive unintended results were seized upon and negative unintended results were mitigated by the module. This includes the extent to which the module applied conflict-sensitive module management and monitoring (especially of risks and unintended effects) and took into account a changing political and social context.	**Evaluation design:** Unintended results are assessed iteratively throughout the evaluation process. Specifically, the analysis relies on findings across other impact dimensions to assess whether additional unintended results occurred. **Empirical methods and data sources:** • Document analysis of contextual documents, esp. integrated peace and conflict assessment and gender analysis. • Semi-structured interviews with the module teams, other actors and experts. • Debriefing and validation workshop with module team.	See limitations in dimension 1, also applicable for this dimension. **Overall data quality for this assessment dimension:** good

4.6. Efficiency

This section analyses and assesses the efficiency of the project. It is structured according to the assessment dimensions specified by BMZ (see Annex).

Summarising assessment and rating of efficiency

Table 21: Rating of OECD DAC criterion efficiency

Criterion	Assessment dimension	Score and rating
Efficiency	Production efficiency (Resources/Outputs)	60 out of 70 points
	Allocation efficiency (Resources/Outcome)	26 out of 30 points
Efficiency score and rating		Score: **86 out of 100 points** Rating: **Level 2: successful**

Starting with **production efficiency**, the evaluation shows that the module resources were applied sensibly to the five outputs, and its instrument and staff costs are conducive to the observed results. The allocation of costs in terms of indicator achievement is also plausible, as most of the output indicators are (over-)achieved. Some challenges were identified (e.g. challenges in light of political turmoil, leading to some delays to conduct the communication campaigns or challenge to ensure the right national team members could be integrated into the team); however, the latest modification offer submitted in 2024 means the evaluation expects all indicators to be achieved by the end of the module. Overall, it can be concluded that the distribution of costs between the outputs is in line with the project's foci, namely increasing the registration capacities (Output 4), increasing coordination (Output 3) and increasing familiarity of beneficiaries with SHP programmes (Output 1). The efficiency of the module and its approach became evident with the changes in the context (e.g. 2022 floods), as the team managed to successfully adapt to changing conditions to adjust their resources towards the partner emergency response. **Allocation efficiency** is also rated as successful. The evaluation finds that the module has managed to effectively transform its activities into its outcomes. This is shown by the overall expected achievement at outcome level. While there might be some potential for cost minimisation to achieve the outcomes as measured by the MO indicators, since these are mostly achieved despite remaining time and budget, the module generally aimed to maximise results even beyond the module outcomes. Allocation efficiency comes from identifying opportunities for scalable impact and new cooperations such as the one with the BISP programme, which is the largest provider of SP in Pakistan although only to a limited extent within German DC. **In total, the efficiency of the project is rated successful (level 2), with 86 out of 100 points.**

Analysis and assessment of efficiency

The efficiency criterion measures the extent to which the objectives of an intervention have been achieved cost-effectively. An intervention, therefore, is efficient when maximum results are achieved with the available financial resources. This analysis was conducted on two dimensions: the module's production efficiency and its allocation efficiency. It considers the principles of yield maximisation and cost minimisation. The former is used to analyse the extent to which more results could be achieved with the same financial means. The objective is not to reduce the intervention's budget but to maximise results with the resources available. The latter principle analyses the extent to which costs were minimised while achieving the same level of results.

The financial data represented in this efficiency analysis is based on the module's cost-output monitoring and prognosis (KOMP) report from 2024 and represents the costs up until 30 September 2024. In addition, qualitative interviews with module staff and other stakeholders were used to assess to what extent the output/resource ratio and alternatives were considered during the design and implementation process.

Efficiency – Dimension 1: Production efficiency

The production efficiency was analysed using the follow-the-money approach. This approach allowed an assessment of the efficiency regarding the use of funds in the different output areas of the project.

With a **total budget** of **EUR 17,370,054** (GIZ, 2024a), the evaluated module had **used around 65% of its budget (EUR 11,292,207) by September 2024** (see Figure 4)**.** The residual funds of the budget are planned to be used in the final months of the module period until September 2025. With its last change offer, the module's budget includes co-financing of EUR 2,370,054 from GF for Output 5 (GIZ, 2024c).

Overall, the **shares of the module budget allocated to its five outputs are considered plausible and justified in terms of the module's approach.** Looking at the proportion of the total module expenditure that was spent on the outputs, 21% was allocated to Output 1 (strengthening of implementation capacities of SHP stakeholders), 17% to Output 2 (evidence-based decision-making of SP actors), 20% to Output 3 (strengthened capacities for better coordination of SP programmes), 38% to Output 4 (mechanisms of BISP for interaction with its beneficiaries) and 1% to Output 5 (pilot for the integration of social assistance and nutritional support) (see 3 and Figure 4). Since the module only spent 65% of its budget, one needs to consider the residual costs as well, to better understand in particular the low share of Output 5. Considering the residual costs, the share of Output 5, which was only in the initial phase of implementation at the time of the evaluation,

is expected to reach 19% by the end of the module period. Output 4, with 30% of the total expenditure by the end of the module, remains the largest share. Output 1 is expected to receive 23% of the budget, similar to its current share, while Outputs 2 and 3 are expected to have shares of 13% and 14% respectively by the end of the module, since there is little budget remaining for these outputs. Overarching costs make up only an insignificant share of the module costs (about 2%).

Figure 3: Efficiency Tool: Costs per output

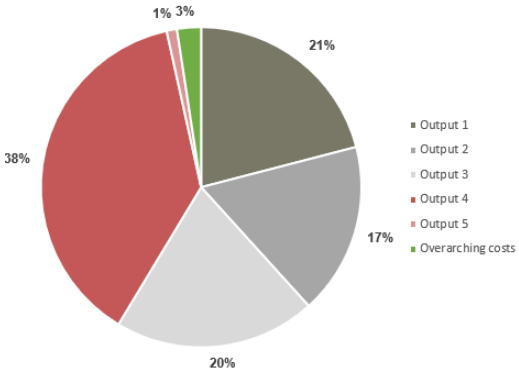


Figure 4: Efficiency Tool: costs and achievements per output

	Output 1	Output 2	Output 3	Output 4	Output 5	Overarching costs
Outputs	Approaches to strengthen the implementation capacities of SSP stakeholders are institutionalised.	The evidence-based decisionmaking of relevant actors in the field of social protection is improved.	The capacities for better coordination of social protection programmes in the partner provinces have been strengthened.	The mechanisms of the Benazir Income Support Programme for interacting with the target groups are more efficient.	The implementation of a framework for the crosssectoral integration of social assistance and nutritional support for adolescent girls has begun in selected parts of the country.	
Actual sum of individual cos	2,088,343.83 €	1,728,141.78 €	2,027,937.00 €	3,778,692.82 €	103,504.41 €	241,319.38 €
Partner inputs	0.00 €	0.00 €	0.00 €	0.00 €	0.00 €	0.00 €
Sum of individual costs and partner inputs	**2,088,343.83 €**	**1,728,141.78 €**	**2,027,937.00 €**	**3,778,692.82 €**	**103,504.41 €**	**241,319.38 €**
Sum in %	**20.95%**	**17.34%**	**20.34%**	**37.91%**	**1.04%**	**2.42%**

Additional residual costs	1,454,230.22 €	198,369.54 €	62,000.00 €	908,207.06 €	2,750,872.57 €	38,571.27 €

Output indicators	OP1.1: A communication strategy in line with the minimum requirements to raise awareness among all SSP stakeholders with regard to the key services and performance processes is operationalised by the SSP programmes.	OP2.1: A monitoring and evaluation system based on key performance indicators (KPIs) is used by the institutions involved in the SSP for annual reporting to the SSP Steering Committee.	OP3.1: In the province of Khyber Pakhtunkhwa, the use of a digitally supported system for coordinating social protection programmes (based on the Beneficiary Registration System, BRS) is documented by 2 responsible service providers in 7 programmes for marginalised groups	OP4.1: A digitally supported system for bidirectional communication between the BISP and the beneficiaries fulfils basic functionality criteria.	OP5.1: As part of the Taleemi Wazaif programme, the authorities responsible for health, education, social welfare and food security have begun piloting a cross-sectoral social welfare and nutrition programme for adolescent girls in 6 selected tehsils.	
Achievement	0%	100%	100%	100%	0%	

Output indicators	OP1.2: Three target group-specific and/or new training programmes for practitioners in the Social health insurance is integrated into the curricula of national educational institutions.	OP2.2: In 4 meetings of the relevant decision-making bodies, the results of the operational research of 4 studies, each on one topic area of social protection, were analysed by the decision-makers.	OP3.2: 60 social protection professionals in Khyber Pakhtunkhwa, Punjab and at federal level have completed a training course on the design, implementation and evaluation of social protection programmes integrated into a national training institution.	OP4.2: Dynamic registration of the NSER is permanently established in 625 of 650 tehsils.		
Achievement	100%	100%	127%	104%		

In the following, the analysis outlines how **the money was spent within each of the outputs in a sensible way, and the instruments and personnel concepts are mostly understandable**. This is then followed by a discussion of general challenges for the module's production efficiency applicable across outputs.

The share of the costs allocated to Output 1 compared to its expected achievements is considered justified. With a relatively large share of module costs allocated to Output 1 (23%), both of its output indicators (1.1 and 1.2) are expected to be achieved. While Output 1.2, new training modules for social health protection practitioners integrated into the HSA curriculum, is achieved, Output 1.1, the operationalised communication strategy, is only expected to be achieved during the remainder of the module duration (Int_10). The key input for the communication strategy, a study on communication campaigns, was delayed and first results were only

available by the start of 2025. Implementation of the pilot campaigns was severely delayed since some initially selected districts were not deemed feasible due to political turmoil and the flooding and it took several months until the selection of replacement districts could be finalised (Int_27). In addition to delays in the implementation of the activities, the endline survey and analysis were delayed as well, such that the development of an evidence-based communication strategy needed to be postponed (Int_11). Similar challenges were observed for the research studies implemented under Output 2. Apart from these delays, the implementation was designed to maximise yields by building on the experience and access to communities of the implementing partner NRSP (Int_27).

With respect to the allocation of funds to Output 2, the implementation of the research hub faced significant challenges in conflict with cost minimisation. Output 2 received the lowest allocation of the module's budget (13%), and indicator OP2.1, the use of a KPI-based M&E system for the SHP programmes, and indicator OP2.2, the discussion and analysis of four operational research studies by decision-makers, are both achieved (100%). One challenge for this output was that research institutions were not prepared to accept grants, including due to procurement laws, leading to delays in procurement and implementation of research studies (Int_16, 54). Another challenge was a weakness in the cooperation mode between the module, the implementing partner GFA and other actors. GFA could not directly engage with partners and instead had to wait until the module signed off decisions which needed to be discussed among the whole team (Int_54). This inefficiency, also stemming from GIZ's bureaucracy, is exemplified by one research institute waiting for nine months for a contract over EUR 4,000 since it was financed through GIZ (Int_54). The coordination between the module and GFA was further hindered by several changes in the AV and focal persons for the research hub (Int_54). Beyond this, the component suffered from weakness in the steering structure and management which hindered its effective and efficient implementation (Int_54). In addition, it should be noted that the innovative concept of the research hub was likely too optimistic and not well thought out (Int_16, 54). This is illustrated by the observation that the cooperation with policy-makers was challenging and the interest was limited (Int_54). This indicates that preconditions were not met, and the institutionalisation of a research hub such that efficient implementation and dissemination of research studies would require a considerably longer timeframe (Int_16). Therefore, the implementation of an innovative concept such as the research hub requires a high degree of flexibility, learning and adaptation; failure to do this led to inefficiencies which were then exacerbated by the challenged cooperation between the module and its implementer. Overall, the research hub, both in terms of its concept and its implementation, was not an efficient measure for indicator OP2.2 and Output 2 more generally. The research hub was highly ambitious, and the achieved results in terms of research studies likely could have been achieved with fewer resources if no resources were used for the institutionalisation of the research hub such that it cannot be considered to be cost minimising.

For Output 3, with both output indicators fully achieved and the allocated budget mostly spent, the allocated costs seem justified. Output 3 received the second lowest share of planned budget (14%), and indicator OP3.1, which measures the use of a digitally supported system for coordinating SP programmes, is achieved (100%) and documented for two responsible service providers. There was an overachievement of indicator OP3.2 (127%) as a higher number of SP professionals completed the training courses developed and implemented by the module. Apart from general challenges discussed below, there is no specific indication of inefficiency from this output.

The share of costs allocated to Output 4 overall seems justified based on principles of yield maximisation. Output 4 received the highest share of budget with 30% of allocated budget, and both output indicators OP4.1, a digitally supported bidirectional communication system between BISP and its beneficiaries, and OP4.2, dynamic registration with NSER is established in 625 Tehsils, have been achieved (100% and 104% respectively). The module adapted and shifted resources towards Output 4 in order to maximise yields (Int_11). The module underwent significant changes with respect to Output 4 which only became part of the module with the first change offer and significantly changed with the second change offer in response to developments on the ground. While the output first focused on the single-window services, this approach was

abandoned in lieu of the approach answering the emergency response to floods to support the dynamic registry aligned with shifting priorities within BISP (Int_11).

Currently, almost none of the spent budget is allocated to Output 5 (1%), in line with its implementation being in the early stages and the current achievement of output indicator OP5.1 being 0%. Eventually 19% of the costs are expected to be allocated to Output 5, most of it through instrument financing (expected EUR 2.0 million). It is expected that OP5.1 will be fully achieved by the end of the module. The implementation of Output 5, as that of other outputs (see below), is challenged by delays and GIZ's bureaucracy (e.g. through contractual procedures, discussed below) and was supported by a temporary module manager to facilitate the early stages of the component (Int_3).

Following the analysis of the budget allocation per output, the analysis now presents the challenges that have affected the efficiency of the module in reaching its outputs. Among interviewed stakeholders, there was a general perception of **inefficiency from significant delays** (Int_4, 12, 27, 36). The key factors contributing to these inefficiencies were **contracting**, **inexperienced staff** and the **challenge to retain staff** as elaborated below. Due to these factors, but also changes to the context, the module was never working according to plans (Int_4). Other factors include long communication channels through cluster coordinators and BMZ, which inhibited direct communication between the module and potential partners, as well as some internal training sessions which were perceived as unnecessary but time-consuming by parts of the module team (Int_4).

Setting up contracts took a long time, delaying timelines and generating inefficiencies. The module originally was set up to be implemented through a few large contracts, which took over a year to set up such that the module only started the implementation phase after 1.5 years (Int_4). Considering the module's initial three-year runtime, this appears disproportional but at the same time does not appear untypical for GIZ modules (Int_4). On the side of implementing partners, there were further challenges with procurement laws as some of the research institutes were not prepared to accept grants (Int_16). In addition to the time frame issues, there was a lack of experience and capacities in setting up contracts or grants in the module team; as the scope and budget of the module kept on growing, the team, however, did not expand, and the size of grants/contracts were too big for the staff to manage effectively. For the additional Output 5, as it was very time-sensitive, the module started to rely on the Regional Office West for contracting which was more costly but effective (Int_4). This example showcases that there are solutions to work more efficiently within the frameworks set by GIZ building on experienced and specialised staff rather than through standardised procedures.

The module lacked management capacities which led to inefficiencies. Delays in implementation were a major challenge caused by a lack capabilities potentially due to inexperienced staff (Int_4). Output 5, for which the Regional West Bureau manages the contract and supports implementers with GIZ's reimbursement rules, exemplifies the value of experienced staff to set up implementation efficiently (Int_4). The Regional West Bureau ensured dedicated resources focused on the specific administrative and financial set-up for this output without diverting resources away from the technical advisors and without generating delays in implementation. In addition, the Regional West Bureau also enables the collaboration with the partners to run smoothly since resources were used flexibly to onboard the output partners into GIZ processes and regulations, for example by organising regular calls and even field visits from the Regional West Bureau's focal point to the partners on-site in Pakistan (Int_3, 4, 65). A similar approach building on expertise in contract management could also have improved other outputs (Int_4). The module often worked through financial contributions which are particularly challenging to manage and require experienced staff as it is difficult to ensure the quality of results (Int_4).

While having qualified staff is key, the GIZ payment schedule is a major barrier to hire and retain qualified national staff leading to inefficiencies. The module builds on qualified and senior national staff; however, it is difficult for GIZ to retain staff in Pakistan due to the wage structure and inflation (Int_17, 23). Many national technical advisors were lost due to limited pay, which is exemplified by one member of staff

receiving an inflation-adjusted salary that barely increased over the course of seven years despite her being promoted to increasingly senior positions (Int_4, 12). Inflation and a more general brain drain from Pakistan exacerbate these problems (Int_17, 23). The wages allowed by GIZ are not perceived as competitive such that the module faces a significant barrier to hiring experienced staff for senior roles. Instead of turning to international service providers to fill this gap, the module team relied on contracts with trusted local partners that included consultancy services so they could locally contract the required profiles without the restriction of international standards to match the local fees (Int_4). This leads to inefficiencies as the module has challenges filling some open positions (Int_4, 12).

Overall, efficiency dimension 1 – production efficiency – scores **60 out of 70 points**. Deductions mainly relate to delays and challenges with contracting in line with challenges observed for the implementation of the research hub. While the concept of the research hub was not necessarily a cost-minimising measure, it was an innovative approach aimed at addressing a highly ambitious goal going beyond specific outputs or results and as such needs to be assessed by different standards.

Efficiency – Dimension 2: Allocation efficiency

Allocation efficiency concerns the use of resources in relation to the achievement of the module outcome. In this vein, the evaluation team analysed whether the distribution of costs between the outputs was appropriate to achieve the MO.

Firstly, the analysis shows an expected **achievement of the module outcome indicators (see Figure 5) despite underachieving some outputs**.[4] While this might indicate a potential for cost minimisation as not all outputs were required to achieve the outcomes measured by the MO indicators, this only holds to some extent. This discrepancy can also be traced back to the project's results logic which contains measurement gaps and should not strictly be interpreted as one-directional. For instance, output indicator 1.1, operationalisation of the communication strategy, was due to be informed by MO indicator 2 which measures the knowledge of beneficiaries linked to communication campaigns. In this case, the **MO indicator is only indicative of larger potential of the module's output**. Further, the underachievement of output indicator 2.2, measuring the consideration of operational research studies, did not hinder the achievement of MO indicator 1, which measures evidence-based adaptations of core processes of the SHP programmes. Especially in this case, **the output is not expected to always lead to tangible outcomes**, because ex ante research is inherently uncertain such that tangible outcomes cannot be guaranteed. Finally, **Output 5 is not clearly linked to the MO indicators** such that the achievement of MO indicators does not depend on the implementation of Output 5.

[4] For one MO indicator, a milestone has been set to account for the fact that the module is still due to run for almost a year after the evaluation mission. Therefore, the number of registered households within the dynamic NSER is only expected to be reached by the end of the module if progress continues.

Figure 5: Cockpit Efficiency Tool: Overall costs and module indicator achievements

Module objective	The supply- and demand-side conditions for the implementation of social protection programmes have improved.
Actual sum of individual costs	9,967,939.22 €
Partner inputs	0.00 €
Sum of individual costs and partner inputs	**9,967,939.22 €**
Additional residual costs	5,412,250.67 €

Module objective indicators	Three core health insurance processes were adapted on the basis of evidence-based recommendations.	The proportion of the target group in three selected districts who know the benefits and processes of the Sehat Sahulat Programme (SSP) increased from 20% to 65%.	A provincial government has developed a strategy for the development of an integrated social security system based on the evaluation of the Beneficiary Registration System (BRS) and taking into account the legal framework, financing issues, the necessary IT structures and gender equality	The number of households registered in the new dynamic National Socio-Economic Registry (NSER) for benefits under the Benazir Income Support Programme (BISP) and other social security programmes has risen from 6 million to 22 million.
Achievement	100%	138%	100%	68%

The module's allocation efficiency benefited from shifting resources towards opportunities as they arose to maximise results. Especially since the resources of GIZ are limited, the module strove for opportunities to increase effectiveness and efficiency through cooperation and leverage (Int_2). For instance, Output 4 was originally not part of the module design but presented an opportunity when political priorities shifted in response to the floods of 2022. The adjustment of the module to work on the dynamic registry illustrates its adaptability to minimise sunk costs and maximise outcomes (Int_4). In addition to tangible results with a plausible contribution from the module (see Section 4.4, effectiveness dimension 2), the SP-SHP's engagement opened the opportunity for the ASP module (Int_4). Similarly, Output 5, while not reflected in any of the outcome indicators, is expected to yield tangible results and continued collaboration with GF beyond the module's duration, thereby efficiently leveraging this co-financing.

However, it is plausible that the module **missed opportunities to maximise results from cooperation with KfW**. Working on similar goals with similar stakeholders, it appears that outputs of KfW could be leveraged to maximise the outcomes of the module and vice versa. At the same time, there is no indication that the programme effectively seized the cooperation with KfW; instead, efforts sometimes appear duplicate (see Section 4.3).

In addition, **the results of the impact analysis show that the module results rather plausibly contribute to impact-level results**, and strategies were deployed to facilitate the sustainability of the module results. By adopting a systems approach, the module can use a comprehensive approach to its activities and outputs that contributes to the MO of improving the supply and demand of the SP and SHP programmes to mutually reinforce each other for higher-level results. For example, the greater capacities for registration (including through digitalised data systems) together with the greater coordination between decentralised governmental institutions (i.e. provincial and federal) constitute a strong mechanism to increase access for the beneficiaries and to improve targeting (see also Section 4.5 and Section 4.7).

In conclusion, the evaluation finds that the module has managed to effectively transform its activities into its outcomes. This is shown by the overall expected achievement at outcome level. While there might be some potential for cost minimisation to achieve the outcomes as measured by the MO indicators, the module generally aimed to maximise results even beyond the module outcomes. Allocation efficiency comes from identifying opportunities for scalable impact and new cooperations; however, only to a limited extent within German DC. Overall, efficiency dimension 2 – allocation efficiency – **scores 26 out of 30 points**. Points were deducted for minor indications of missed opportunities to minimise costs and maximise results.

Methodology for assessing efficiency

Table 22: Evaluation matrix for OECD DAC criterion efficiency

Efficiency assessment dimensions	Basis for assessment	Evaluation design, empirical methods and data sources	Data quality and limitations
Production efficiency (Input/Outputs)	The module is understood to have operated efficiently if results were maximised with the given means (yield maximisation), and if results were achieved employing only the resources required (cost minimisation). Using the efficiency tool, the assessment considered: • the extent to which output indicators were achieved in the given timeframe, • the costs per output, type of costs, agreed and provided partner contributions, • the extent to which outputs were and could have been maximised with the given resources (by alternative approaches, different results hypotheses, activities, TC instruments within outputs, shifting resources between outputs), and to which costs per output were and could have been minimised while achieving the same outputs, and • the use of cost data and results-based-monitoring for steering and decision-making.	**Evaluation design:** The evaluation applied a 'follow-the-money' approach. Thereby, all expenses were identified and assigned to specific outputs of the intervention. With this mapping of costs concluded, the evaluation team assessed the appropriateness of costs per output, considering perspectives of the module team. **Empirical methods and data sources:** • Analysis of quantitative cost-output data using GIZ efficiency tool, incl. information on HR instruments per output and module cost-commitment report (foreseen 2024). • Document analysis of progress reports, operational plans and indicator progress sheets (2024). • Semi-structured interviews with module management and staff as well as with key implementation partners.	The data quality of the indicators seems strong. Some limitations come from the turnover of the AV and other project staff. **Overall data quality for this assessment dimension:** strong
Allocation efficiency (Input/Outcome)	The module is understood to have allocated its resources efficiently if cooperation potential within the GIZ and KfW structures and partner structures were identified and realised. Using the efficiency tool, the assessment considered: • the extent to which the MO was and could have been maximised with the available amount of resources (by alternative approaches, TC instruments, shifting resources, adapting module design), • the extent to which the module costs were and could have been minimised while achieving the same outcome, • the extent to which the outcome/resource ratio (costs vs achievement of MO indicators) as well as alternatives were monitored and considered for decision-making, and • the extent to which co-financing was efficient and had a leveraging effect.	**Evaluation design:** The evaluation design followed the questions from the evaluation matrix. **Empirical methods and data sources:** See dimension 1, plus: • semi-structured interviews with staff of other GIZ and KfW modules (namely ASP (GIZ); Health financing III; Strengthening SP for Health and climate resilience; Employment of women in the private health sector (KfW)) as well as BMZ's representative in Pakistan, and • semi-structured interviews with module management and staff of development partners, in particular GF.	See above. In addition, Output 5, which covers a significant share of costs and co-financing, is not directly linked to any of the four MO indicators; therefore, the output's contribution to outcomes and impacts are considered. **Overall data quality for this assessment dimension:** good

4.7. Sustainability

This section analyses and assesses the sustainability of the project. It is structured according to the assessment dimensions specified by BMZ (see Annex).

Summarising assessment and rating of sustainability

Table 23: Rating of OECD DAC criterion sustainability

Criterion	Assessment dimension	Score and rating
Sustainability	Capacities of the beneficiaries and stakeholders	17 out of 20 points
	Contribution to supporting sustainable capacities	29 out of 30 points
	Durability of results over time	45 out of 50 points
Sustainability score and rating		Score: **91 out of 100 points** Rating: Level 2: successful

The evaluation concludes that the sustainability of the target groups' capacities is successful. The direct target groups have strong capacities that they can use, especially technical, institutional and technological capacities and tools. Institutional capacities, although currently strong, could be tied to a single person which would affect their use over time at the level of entire institutions. The indirect target groups show promising examples of capacities that could be used over time but cannot be entirely confirmed yet. The module has established a strong foundation for sustainable SP in Pakistan through strategies supporting institutional (e.g. integrated sustainability approach, stakeholder engagement, needs-based approach), technological (e.g. development of tools and systems to reinforce actors at multiple levels relevant for the sectors), and financial sustainability (e.g. tailoring its activities to the partners' capacities and adopting a pilot approach), though it lacks systematic monitoring of sustainability outcomes. Lastly, while risks such as the politicisation of SP-SHP sectors, staff turnover, funding constraints, and data security concerns present significant challenges, the module has adopted successful strategies to alleviate some of these (such as policy work, integration of sustainability across activities since their initiation, and stakeholder engagement) and the ongoing institutional reforms offer promising pathways to ensure the durability of the module's results. **In total, the sustainability of the project is rated successful (level 2), with 91 out of 100 points.**

Analysis and assessment of sustainability

The sustainability criterion concerns the extent to which positive results of the intervention can be expected to continue once the intervention has ended.

Sustainability – Dimension 1: Capacities of the beneficiaries and stakeholders

To assess this dimension the evaluation team examined the extent to which capacities relevant to the MO were used by direct and indirect target groups and whether multidimensional sustainability (institutional, technological, financial) was developed.

Starting with the direct target groups and cooperation partners (i.e. specialists and managers responsible for the effective implementation of social protection and social health protection programmes at federal and provincial level, the insurance companies, and other governmental and non-governmental actors in the SP landscape), the data shows the groups have durable **institutional, financial and technological capacities**.

First, **the specialists and managers** responsible for the effective implementation of social protection and social health protection programmes at federal and provincial level **have the technical capacities on SP and SHP to support the development of the sectors in Pakistan.** On the one hand, the data shows that SP and SHP practitioners now have individual technical capacities (e.g. on SP, M&E, adaptation to climate change, planning, and financial sustainability) to integrate them into their daily practice to implement SP-SHP programmes effectively (Int_4, 41, 7, 16, 69, FGD_1, FGD_2). The risk that the sustainability relied on a single individual remains, since capacities are mostly developed at individual level (Int_4). On the other hand, national training institutions have also been reinforced to integrate the SP and SHP courses in their curricula, which

allows them to independently roll out and disseminate the core concepts, creating a unified framework for the implementation and evaluation of SP and SHP programmes. Also, the close collaboration with the National Institute of Management and the CSA to deliver the courses and for them to pilot using the Moodle technology platform for PSPA further ensured the courses' institutional sustainability by enabling continuous training for newly inducted staff (i.e. all mid-level managers who move to more senior levels) (Int_4, 26). Furthermore, HSA has also gained prominence as a leading agency for social health protection training, as the host of the online version of the health insurance management course, through HSA's Moodle platform and website. This digitalised course allows a broader audience to access training and resources on health insurance management. For example, the SP course created a critical mass of skilled practitioners within government systems, with over 200 managers in social health protection being trained across Pakistan (Int_4). HSA also gained visibility in the SHP landscape through its enhanced capacity to participate in and lead policy analysis. For example, HSA played a pivotal role in the development of the National Health Policy. Furthermore, through initiatives like the endline study to assess the impact of the communication strategy, HSA is contributing to the generation of an evidence base for policy-making, driving informed decision-making and contributing to overall improvement in governance of the SP in Pakistan (Int_46).

Second, **the specialists and managers** responsible for the effective implementation of social protection and social health protection programmes at federal and provincial level **have the technical capacities on SP-SHP, including on adjustment of processes and reforms of the sectors, to ensure their institutional sustainability.** On the one hand, the data shows that the newly acquired technical capacities fostered a deeper understanding of SP in the policy discourse and governance frameworks, and led to the development of organisational capacities of stakeholder institutions for sustainable and effective implementation. For example, qualitative data shows that the SHP programmes have now gained technical, technological and organisational capacities that enabled it to become independent from the support of the module and a model for others (including to support Balochistan to learn from its experience) (Int_4, 36, 68). In addition, the development of SLIC is also indicative of the development of actors in the field. Over eight years, SLIC developed greater operational agility, allowing for rapid programme deployment. For instance, Punjab's co-payment method was implemented within a day, a significant improvement compared to the six-to-twelve-month planning phases in 2015. This efficiency, supported by systems development and streamlined processes, enabled SLIC to respond rapidly to new requirements and expand efficiently across regions, to serve a growing client base, including over 70 corporate clients (Int_16).

On the other hand, the data highlights evidence of process-related changes adopted by partners to conduct their own analysis. For example, the idea of a research hub addressing gaps in evidence-based policy-making was institutionalised by the Punjab Government with the launch of the SDGs Plus Project. This initiative includes a dedicated component for generating SP evidence, detailed in the public PC-1 document, which secures funding and establishes an in-house research team. This model not only supports ongoing research but also fosters collaboration and investment in evidence-driven policy-making (Int_28). In addition, the greater digitalisation of various stakeholder organisations has led to system-wide improvements, especially in the utilisation of data for evidence-based decision-making. The SHPI reported that it would continue the practice of data utilisation initiated by the module to carry out regular analysis of data generated by SLIC to assess various key indicators like hospital footfall as well as mapping disease incidence (Int_41). Other digitalised systems such as NSER, BRS and other MIS systems are also expected to contribute to the institutional sustainability of the results and further create opportunities for collaboration (Int_3, 4, 20).

Third, **the specialists and managers** responsible for the effective implementation of social protection and social health protection programmes at federal and provincial level **have the technical capacities on SP-SHP, including on the appropriate technological capacities and human resources to support the institutional sustainability of the good governance practices of the SP-SHP sectors.** The data indicates that significant advances have been made with regard to data integration and coordination. For example, the Memorandum of Understanding with Sindh and the completed audit in Punjab underscore enhanced data accessibility, critical

for informed decision-making and accountability. The involvement of international organisations such as ADB and WB further strengthens governance frameworks (Int_4, 40). The first SP Conference in Bhurban in 2022 also facilitated the commitment to data integration between BISP and other actors, ensuring provinces have access to complete datasets (Int_4, 40). Other indications of greater coordination were highlighted such as the establishment of coordination structures, like the Technical Working Groups which have institutionalised regular quarterly meetings among all partners, ensuring sustained collaboration and policy alignment (Int_4, 7, 36). Moreover, there is evidence that SP-SHP actors have the capacities to ensure some financial and institutional sustainability. For example, the SP framework has advanced with the adoption of a social protection policy in KP in 2022, as well as a social protection reforms strategy in KP in 2023, but also the development of a draft national framework for social protection and poverty alleviation that is still pending adoption by the Cabinet (Int_4, 9, 10, 28). Further, financial measures have been adopted such as co-payment models by the PHIMC, inspired by international best practices and exchange with other regions, which enhance financial sustainability for health services (Int_4, 28).

Lastly, there are also some indications that indirect beneficiaries have reinforced their capacities for more sustainable changes. For instance, beneficiaries have greater awareness of the different SHP programmes available to them (GIZ, 2025). They also have greater access to registration processes for SP and SHP programmes, including through static, mobile and digital processes (Int_4, 40). In particular, mobile registration vans have improved accessibility, bringing services directly to underprivileged communities (Int_40). Some beneficiaries also have the opportunity to enter fixed employment positions, as social case workers (ashas) in BISP registration centres (Int_4, 9, 40). Also, through the introduction of iron supplements in the BISP programme Taleemi Wazaif, it is also expected that young women will be developed to become agents of change in their communities. By integrating nutrition education combined with voucher incentives while empowering girls as agents of change, the initiative is expected to improve dietary practices at home in the long term, expand access to health information beyond direct beneficiaries, and increase school enrolment among out-of-school girls through peer-driven incentives. This approach has a strong sustainability component to it but no results yet (Int_3, 4, 12).

In conclusion, the sustainability of the target groups' capacities is assessed as being high. The direct target groups have strong technical, institutional and organisational capacities that they can use, especially the technological capacities and tools to implement SP-SHP programmes. Institutional capacities, although currently strong, could be tied to a single person which would affect their use over time at the level of entire institutions. In addition, while digitalisation and enhanced data utilisation have further contributed to evidence-based decision-making, initiatives like the research hub are currently not considered sustainable. The indirect target groups show promising examples of capacities that could be used over time but cannot be entirely confirmed yet. Thus, sustainability dimension 1 – capacities of the beneficiaries and stakeholders – scores **17 out of 20 points.**

Sustainability – Dimension 2: Contribution to supporting sustainable capacities

To assess this dimension, the evaluation team examined the extent to which the module facilitated the anchoring of results in (partner) structures. To this end, it assessed whether the module chose appropriate approaches, methods and policies/strategies in cooperating with direct target groups. The evaluation team also analysed the extent to which the ownership of the direct target groups/intermediaries was strengthened by a participatory approach and a shared vision.

Regarding the integration of sustainability aspects into the module, the data indicates that the module adopted a **sustainability-oriented strategy for its activities since their initiation that contributes to the institutional sustainability of the results** (Int_4, 8, 9). Firstly, the module paid attention to sustainability in its **stakeholder engagement with German DC partners and with module partners**. With German DC partners, the module was involved in several GIZ clusters (e.g. economic development, governance and climate change)

which contributed to the dissemination of information on results and to other GIZ projects (see also Section 4.3). While only a limited number of synergies and concrete collaborations can be observed, the collaboration with the sister module ASP that runs alongside the SP-SHP module is a clearly successful example. Together, both modules developed the offer for a potential follow-on module, but were also integrated under a common DC programme on social protection, to ensure the continuity of the SP-SHP module by integrating three out of five components into the ASP design (Int_4, 17). This is ensured, for example, by the fact that the ASP module continues to facilitate the momentum (through a dedicated indicator and measures) and supports the sustainability of the results of the SP-SHP module (Int_9, 10, 17).

With the partners in relation to the development of the SP course, SDPI was considered a potential local partner to integrate the SP course in its portfolio on a permanent basis (in addition to HSA) to ensure the sustainability of the course (Int_4, 46, 55). However, the capacities of SHPI in SP later needed to be strengthened which led to, ultimately, the course being hosted at HSA and CSA. This is also in line with the growing SP-SHP portfolio in Pakistan in the last 10 years, which calls for the constant development of capacities in these sectors, in order to enable the relevant actors to develop their own strategies. In addition, through policy work, the module also contributed to tightening the linkages between actors and establishing strong frameworks for SP and SHP, which is also considered a facilitating factor for institutional sustainability. For example, the organisation of an SP conference in May 2023 with a focus on data sharing led to a joint declaration that BISP would share data with provinces (Int_4, 40, 55). Policy work is also considered a solid basis for the durability of the results. In this context, the integration of module results into policies, such as the SP policy in KP and mentions of BRS, strengthens the institutionalisation of the programme's outcomes, embedding them within broader governance frameworks (see also Section 4.4).

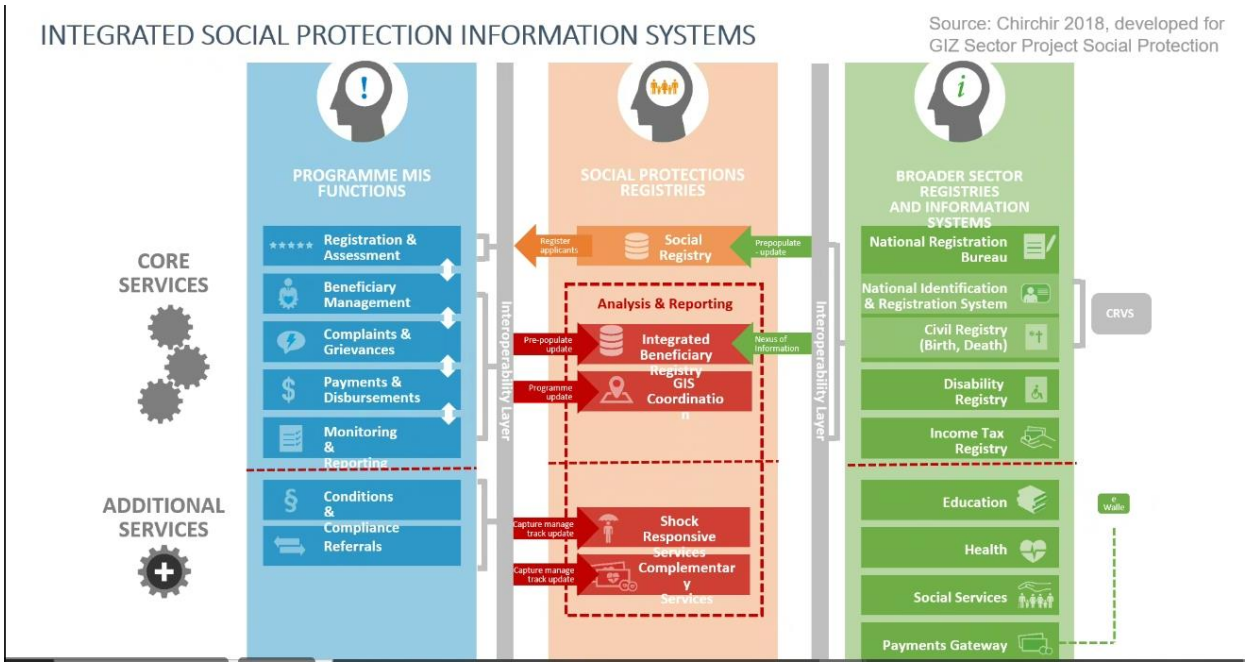
Secondly, the module also supported the development of **coordination groups** that contributed to the institutional sustainability of the results. For example, in KP, the module supported provincial actors in establishing technical and steering committees, with the objective of coordinating SP-related activities. The module support also included the development of terms of reference to specify the way in which these committees were expected to function, further anchoring the practices among the participants (Int_7, 9, 10). In addition, together with the sister module ASP and UNICEF, the SP-SHP module helped set up the Cash Transfer Coordination Group, hosted at BISP, which represents a sustainable mechanism for coordination among the relevant national and international actors (governmental and non-governmental). For example, a social security conference was organised through this working group (Int_4, 11, 16, 40). Lastly, the module also established steering groups at activity level. For example, the data indicates that the steering committee established for the coordination of the interventions under Output 5 (with GF) are perceived as good practice to foster transparency and buy-in from national and international partners involved (Int_3, 4, 10, 11).

Another key aspect of the module's sustainability strategy that supports institutional sustainability is its **needs-based approach** to ensure the activities are tailored to their partners' needs in the sectors and therefore continue to be used. In this case, the use of a **participatory approach** throughout the module time frame was a core element to sustain the results. Specifically, consultation with partners before the implementation of the module contributed to developing an adequate design to meet the partners' needs (Section 4.2) and make continued use of the results (see dimension 1 above) (Int_4, 8, 9). In this context, the support of the module to develop technical and organisational skills was a key driver for institutional sustainability. For example, by targeting several levels (e.g. middle- and high-level management) of public institutions like SHP programmes in capacity development activities, the data shows that the entire institution has now adopted a harmonised and coherent approach towards SHP that makes it a model in the sector (see also Section 4.5, dimension 1). Notably, the module interventions including strengthening organisational systems and processes especially for evaluating and onboarding health service providers as well as developing mechanisms for data supported evidence-based planning and helped SHPI to sustainably anchor these results (Int_8, 9, 13, 36). Other technical capacity development efforts from the module are also expected to support the institutional sustainability of the results. This is the case, for example, for the concept of the agents of change embedded in

Output 5, whereby adolescent girls are expected to be trained on nutritional supplements to then disseminate the knowledge and approach to their respective communities. The concept of the approach supports the idea of institutional sustainability; however, the first pilot will provide the concrete evidence to confirm whether this is the case or not (Int_3, 11, 12).

Regarding the technological sustainability of the results, the data confirms that the module has contributed to this aspect through its support to investing in fixed resources and digitalisation. The data shows that the module's contribution to technological sustainability is multifaceted, addressing both immediate and long-term needs. By strengthening institutional capacity, promoting inclusive access, and integrating scalable, modular systems, the module contributed to establishing integrated SP information systems that can be sustainably used (see Figure 6 below). As presented in the figure, the module has worked on an integrated three-pillar approach which led its capacity-building, infrastructure and licencing investments to develop an MIS system for two SSP departments, to advance data-sharing agreements within provinces and with the federal government, and to launch an app for the registration of people with disabilities (Int_4, 5, 9, 10). Notably, SSP has evolved into a fully digitalised programme, achieving remarkable operational efficiency. For instance, the e-claims system enables the processing of up to 125,000 daily admissions, significantly streamlining administrative processes (Int_16). Additionally, the development of a decision support system, powered by real-time data, allows for precise disease incidence mapping, enhancing the ability to address health challenges effectively. Owing to the solid integrated digitalisation concept, the module also contributed to obtaining key results that can be sustainably used by the beneficiaries for robust decision-making, adaptive social protection, and disaster risk management, and can continue to be supported where needed, notably by its sister project, ASP (Int_10, 17). For example, the SSP plans to expand its technological infrastructure, including the implementation of an MIS in KP, further signalling the progression towards sustainable digitalisation and also reflecting a mindset shift towards embracing technology.

Figure 6: GIZ-integrated SP information systems



Lastly, **regarding the contribution of the module to financial sustainability, the data shows that the module contributed to this aspect in various ways**. First, the module provided the partners with equipment for their sustainable use. For example, at BISP tehsil level, the module supported the offices in their emergency response to the floods by providing them with equipment such as vans, megaphones, trays, benches, fans and water dispensers. The data shows that this not only supported the institution in its emergency response but also prepared it for future disasters, which are increasingly happening. Second, the module also contributed to

ensuring the financial transition for new staff positions such as ushers and mobile registration staff in response to the emergencies. This has led BISP to continue financing these positions but also contributes to the financial sustainability of the ushers, who are beneficiaries of the BISP cash transfers programme and are therefore expected to graduate out of poverty (Int_4, 5, 40). Third, through dedicated capacity development measures (e.g. actuary study on financial sustainability and SP course module on sustainable financing), the data confirms that participants continue to apply these principles in their daily work, which is expected to contribute to the financial sustainability of the results. For instance, following the actuary study conducted with SSP, BHCP is now using this analysis to estimate its utilisation rate (FGD_1; Int_4, 5, 16, 29). Lastly, through close coordination with its partners, the module supported a pilot intervention in line with their financial capacities. For example, the module supported SSP with the UHC priority of the SHPI, although financial sustainability was identified as a weakness (Int_8).

In conclusion, the module has established a strong foundation for sustainable SP in Pakistan through strategies supporting institutional (e.g. integrated sustainability approach, stakeholder engagement, needs-based approach), technological (e.g. development of tools and systems to reinforce actors at multiple levels relevant for the sectors), and financial sustainability (e.g. tailoring its activities to the partners' capacities and adopting a pilot approach), though it lacks systematic monitoring of sustainability outcomes. Therefore, sustainability dimension 2 – contribution to supporting sustainable capacities – scores **29 out of 30 points**.

Sustainability – Dimension 3: Durability of results over time

Several factors pose a risk to the sustainability of the results. First, **political and financial constraints** can represent a risk to the durability of the results. As such, political priorities at the provincial level, coupled with financial resource limitations, pose significant challenges for SP-SHP programmes. Tensions between federal and provincial governments further exacerbate the issue, creating an environment of instability that could jeopardise the results obtained. Additionally, funding availability, combined with inflation, drive the sustainability of the results for some institutions (e.g. financing of public hospitals) (Int_4). Second, **staff turnover and political affiliations** can also represent a key challenge for sustainability. As such, frequent staff turnover, inadequate forecasting of funds, and the politicisation of SP and SHP data undermine the sustainability of the module results. These challenges are further compounded by political affiliations influencing staffing decisions, as well as capacity constraints due to insufficient training and expertise (Int_12). Third, **data security concerns** remain a critical issue (e.g. for data-sharing agreements in particular) especially in light of previous instances of political misuse of data and mistrust between federal and provincial entities (Int_11, 12). Lastly, the SP-SHP systems are relatively **new systems**, which are still under development, and therefore require continued support (see also Section 4.2).

In contrast, there are influencing factors that should positively affect the durability of the module results. This first includes all the **strategies implemented by the module to ensure the sustainability** of the results (e.g. integration of sustainability-oriented strategy into module activities, fostering strong stakeholder engagement, needs-based approach to answer the various (emerging) needs of the SP-SHP actors, and policy work) (see dimension 1 above). Second, **institutional** changes can be a risk to the durability of the results but also an opportunity to anchor them. For example, positive institutional developments further solidify the foundation for sustainable implementation, providing an opportunity to address structural inefficiencies and align initiatives with long-term strategic objectives, such as the development of formalised SP and SHP frameworks.

In conclusion, while risks such as the politicisation of SP-SHP sectors, staff turnover, funding constraints and data security concerns present significant challenges, the module has adopted successful strategies to alleviate some of these (such as policy work, integration of sustainability across activities since their initiation, and stakeholder engagement) and the ongoing institutional reforms offer promising pathways to ensure the durability of the module's results. However, some of the risks remain high despite the adopted strategies and this could jeopardise the results of the module, such as data protection. Therefore, sustainability dimension 3 – durability of results over time – scores **45 out of 50 points**.

Methodology for assessing sustainability

Table 24: Evaluation matrix for OECD DAC criterion sustainability

Sustainability assessment dimensions	Basis for assessment	Evaluation design, empirical methods and data sources	Data quality and limitations
Capacities of the beneficiaries and stakeholders	Sustainability in this dimension is understood to have been achieved if results have been anchored in (partner) structures and instruments were employed appropriately to this end.	**Evaluation design:** To assess this dimension, the evaluation analysed capacities of direct and indirect target groups in the areas of the module's capacity development approach. To this end, the evaluation team, as part of the needs assessment and comparison of partner structures (chapter 4.2), closely differentiated between target groups. **Empirical methods and data sources:** • Document analysis of progress reports, SOPs and contractual agreements of SHP programmes (if available), baseline and endline survey reports, study reports, relevant strategic documents of the KP and Punjab provincial governments, and data and documentation of NSER registrations. • Secondary data analysis of baseline and endline data if data but no reports are available. • Interviews with module staff. • Interviews with relevant stakeholders, in particular the direct target groups (SHP programmes, ministries of health, NSER, PIDE).	Considering the strong focus of the module on sustainability (e.g. institutionalisation of training courses in local training centres), the availability and quality of data was good. However, several modification offers have slightly adjusted the design of the module as initially designed, in part due to the changing framework conditions (i.e. 2022 floods) and following the interest to formalise a cooperation partnership with GF (Output 5). This output is still ongoing and results were therefore not yet available for this intervention. **Overall data quality for this assessment dimension:** good
Contribution to supporting sustainable capacities	This dimension analyses the extent to which the module has contributed to beneficiaries and stakeholders having the capacity, resources, ownership and resilience to sustain the results achieved over time. To this end, the dimension assessed whether the module chose appropriate approaches, methods and policies/strategies in cooperating with direct target groups. In this regard, the evaluation team also analysed how far the ownership of partner institutions has been strengthened by means of a participatory approach and a shared vision, and to what extent the module's exit strategy complied with the three dimensions of sustainability, i.e. financial, technical and organisational sustainability.	**Evaluation design:** Based on the contribution analysis, and in particular findings from the effectiveness and impact assessments, the evaluation team analysed the module's contribution to supporting sustainable capacities along the dimensions of the Capacity WORKS framework. To differentiate contributions at the level of direct target groups, the assessment relied on results from the comparison employed in Section 4.4. This further enabled the identification of factors impacting sustainability that were internal and external to the module. **Empirical methods and data sources:** • Document analysis of progress reports, SOPs and contractual agreements of SHP programmes (if available), baseline and endline survey reports, study reports, relevant strategic documents of the KP and Punjab provincial governments, and data and documentation of NSER registrations. • Secondary data analysis of baseline and endline data if data but no reports are available. • Interviews with module staff. • Interviews with relevant stakeholders (SHP programme, ministries of health, NSER, PIDE).	See assessment in dimension 1, also applicable for this dimension. **Overall data quality for this assessment dimension:** good

Sustainability assessment dimensions	Basis for assessment	Evaluation design, empirical methods and data sources	Data quality and limitations
Durability of results over time	Sustainability in this dimension is understood to have been achieved if results can be forecast to be permanent, stable and long-term resilient, also considering trade-offs between sustainability dimensions. For the skills development component of the module's approach, the assessment of sustainability focused on the plausibility of skills being used in the future. For the digitalisation component, the evaluation team examined whether the necessary infrastructure will be available on a long-term basis.	**Evaluation design:** This evaluation dimension relates to a prognosis of durability. The assessment rested on a plausibility analysis of the durability of results at the level of target groups. The analysis took into account potential risks and other influencing contextual factors. **Empirical methods and data sources:** • See dimension 1.	See assessment in dimension 1, also applicable for this dimension. **Overall data quality for this assessment dimension:** good

4.8. Key results and overall rating

Considering all the evidence described above, the module – Support to SP including social health protection in Pakistan (PN 2019.2140.2) – is rated successful, with 88 points out of 100. The module was particularly well designed for the needs of the SP and SHP sectors in Pakistan, adopting a systemic approach, which effectively combined policy, technical and technological capacity development to enhance social protection coverage. Its multilevel engagement strategy strengthened governance, improved data-driven decision-making, and increased outreach through collaboration with key stakeholders. The module's adaptability to emerging challenges, such as political shifts and environmental crises, reinforced its effectiveness, while its strategic focus on sustainability ensured the longevity of its interventions. Strong coordination with international partners, proactive partner selection, and the capitalisation on existing relationships further contributed to its success. However, the module faced challenges, including weaknesses in the design and management of the research hub, which limited the potential impact of this intervention on policy development. Additionally, the M&E system, while functional, lacked a systemic approach to adequately capture sustainability outcomes and long-term impacts. External political risks, such as governance fragmentation and policy instability, also posed hurdles to seamless implementation.

Table 25: Overall rating of OECD DAC criteria and assessment dimensions

Evaluation criteria	Dimension	Max	Score	Total (max. 100)	Rating
Relevance	Alignment with policies and priorities	30	30	**92**	Level 1: highly successful
	Alignment with the needs and capacities of the beneficiaries and stakeholders	30	30		
	Appropriateness of the design	20	12		
	Adaptability – response to change	20	20		
Coherence	Internal coherence	50	30	**73**	Level 3: moderately successful
	External coherence	50	43		
Effectiveness	Achievement of the (intended) objectives	30	30	**91**	Level 2: successful
	Contribution to achievement of objectives	30	26		
	Quality of implementation	20	18		
	Unintended results	20	17		
Impact	Higher-level (intended) development changes/results	30	28	**93**	Level 1: highly successful
	Contribution to higher-level (intended) development results/changes	40	35		
	Contribution to higher-level (unintended) development results/changes	30	30		
Efficiency	Production efficiency	70	60	**86**	Level 2: successful
	Allocation efficiency	30	26		
Sustainability	Capacities of the beneficiaries and stakeholders	20	17	**91**	Level 2: successful
	Contribution to supporting sustainable capacities	30	29		
	Durability of results over time	50	45		
Mean score and overall rating		100	**88**		**Level 2: successful**

Table 26: Rating and score scales

100-point scale (score)	6-level scale (rating)
92–100	Level 1: highly successful
81–91	Level 2: successful
67–80	Level 3: moderately successful
50–66	Level 4: moderately unsuccessful
30–49	Level 5: unsuccessful
0–29	Level 6: highly unsuccessful

5 Conclusions and recommendations

5.1 Key findings and factors of success and failure

By summarising the most important success and failure factors, this section will provide lessons particularly for other projects working in a similar thematic area and for the SP projects.

Factors of success

Internal

One of the key strengths of the module resides in its **systemic approach** to SP-SHP, by combining its support to policy, technical and technological capacities development. By developing capacities to strengthen local systems, enhancing governance (e.g. addressing policy gaps through political engagement and fostering interprovincial coordination) and facilitating data availability (including through data-sharing mechanisms) for evidence-based decision-making, the module used strong mechanisms to improve the coverage and the protection of the poorest-of-the-poor households. In line with the systemic approach, the module also **worked with system actors** to increase outreach and impact, through its multilevel approach. This allowed the module to obtain key results at macro level (adoption of the draft National Poverty Alleviation Policy and SP conference), meso level (data-sharing agreements between provinces, development of systems and processes, including the NSER) and macro level (improved overall conditions of the SP and SHP sectors for improved coverage and protection of vulnerable households).

Another strength of the module is that it relied on an **adaptable design** for the development of the system of SP-SHP sectors to address the key barriers to improve the supply and demand conditions for the implementation of social protection programmes. Through its collaborative working approach with its partners, the module could tailor its interventions to the (emerging) needs of the key stakeholders in the sectors. For example, the module could seize the opportunity to support with the dynamic registry and ASP necessitated after the 2022 floods.

In addition, the module design included a **clear orientation towards sustainability**. As such, most of the interventions planned by the module included considerations for sustainability. The module also deployed several successful strategies to ensure the sustainability of its results.

On an operational level, the module made **strategic use of its technical and administrative staff** to ensure the smooth implementation of the activities and limit delays. Of particular importance is the use of the Regional West Bureau administrative unit for financial and contract management.

The module also focused on **obtaining concrete results and changes**, not only fulfilling indicators, which drove innovative responses in its interventions, such as dynamic registration, intersecting nutritional and cash transfer SP programmes, and piloting the poverty graduation approach (by involving women beneficiaries of cash transfers as staff members for the dynamic registration), which were also key highlights of the module.

Lastly, the module could also rely on **GIZ's long-standing work on SHP in Pakistan** to capitalise on previous results and on trust-based relationships with key stakeholders and to continue developing further cooperation.

External

The quality of the systemic approach of the module was further reinforced by the selection of proactive partners that are well integrated in the selected sectors, that are responsive to the module support, and know how to strategically use the collaboration with GIZ.

The presence of multiple donors interested in the module's thematic and geographic focus is an opportunity to build on existing results and tools and to further disseminate products to other target groups.

Factors of failure

Internal

One of the first factors of failure that should be mentioned is a **design weakness**: there were some flaws in the design and the management of the research hub; the hub had been expected to encourage the use of research for policy development (under Output 2). In particular, the evaluation found a tension between the ambitious nature of the initiative and the lack of readiness among Pakistani actors. The decision to contract an external consultancy, rather than a politically embedded local entity, reduced stakeholder buy-in and hindered the policy impact of the research. Delays in research execution further limited its ability to inform decision-making.

The **M&E system** could be improved to better adhere to the systemic approach of the project. The evaluation finds that the M&E system does not entirely capture the effects of the systemic approach of the project. In this sense, the evaluation finds that the reported results only partially assess the effective results in terms of sustainability. In addition, the decentralised M&E system is not an effective component of a strong steering structure as it lacks visibility over (intermediary) advances, delays and a centralised identification of evidence.

External

The main external risk in Pakistan revolved around the **political environment**. First, the compartmentalisation of SP-SHP remains a barrier for synergies, coordination, building of trust between governmental institutions and the federal government. Second, the governance structure in Pakistan continues to represent a challenge for the module to align federal and provincial governments in SP and social health protection. For example, the lack of a clear federal strategy and the ad hoc nature of governance in territories like Kashmir and GB complicate nationwide implementation. Lastly, political turmoil also constitutes a risk for module implementation. For example, in the case of rising insecurity, module activities need to be adjusted, which will lead to delays in implementation and results.

Findings regarding 2030 Agenda

Universality, shared responsibility and accountability

The module contributed to SDG 1 (No Poverty) and SDG 3 (Good Health and Well-Being) by improving the supply- and demand-side conditions for SP programmes through strengthened capacities, enhanced evidence-based decision-making, and improved coordination among key actors across provinces. It also enhanced BISP's efficiency and digital communication, further strengthening the link between supply and demand. Furthermore, the integration of social assistance with nutritional support for adolescent girls, in collaboration with the GF, expanded the project's impact to SDG 5 (Gender Equality). The module also collaborated with international development organisations such as WB, ILO and GF, while leveraging its expertise in SP through partnerships with key national stakeholders, including MoPASS and BISP.

Interplay of economic, environmental and social development

The module fostered a holistic understanding of sustainable development. Bearing in mind the overall goal of the module is to improve the supply- and demand-side conditions for the implementation of social (health) protection programmes, the module has a clear objective to contribute to the social dimension of sustainability. This is particularly the case through the systemic and multilevel approach adopted by the module to ensure all relevant actors and systems of the decentralised governments are supported to improve social (health) protection programmes. In addition, the module also dedicates special attention to the integration of women, youth and people with disabilities in its activities and results. Economic sustainability is addressed by developing the capacities of the key actors to ensure financial sustainability in the implementation of the social (health) protection programmes (e.g. through courses, including modules on financial sustainability, studies, like the willingness-to-pay study, and technical support to the implementation of the UHC approach in KP, for instance) but also supporting the agreement on the development of a PC-1, which should institutionalise communication within the SHP programmes by including a 10% addition to the premium for the purpose of communication activities, ensuring that these elements can continue beyond the module. Although the module does not contribute to the objectives of environmental protection and resource conservation, growing attention has been dedicated to climate adaptation and disaster risk management. This was especially the case in the context of the last modification offers and integration with the ASP sister module into the DC programme.

Inclusiveness/leave no one behind

The module successfully contributed to advancing cross-cutting themes, particularly human rights and gender safeguards, in line with its proposal. It promoted the right to SP by improving the availability and accessibility of services for vulnerable groups, aligning with human rights standards. The module specifically enhanced access to health services, directly supporting the realisation of the human right to health. Its poverty-focused design, embodying the principle of ‘leave no one behind’, also contributed to the realisation of the right to health, consistent with Agenda 2030. In terms of gender, the module facilitated equal access to SP services, contributing to equal opportunities as a core human rights principle. By creating a unified database for beneficiaries of SP programmes, particularly for vulnerable groups like people with disabilities, it streamlined access to services and increased transparency. Additionally, the module raised awareness among stakeholders about the importance of gender-sensitive, inclusive and barrier-free implementation of social programmes, ensuring that citizens’ centres served as accessible points of contact for advisory services.

Findings regarding follow-on project

The module has submitted an offer to BMZ for a follow-on initiative called SPSHP-i. This follow-on module aims to improve the accessibility, effectiveness, and quality of SP programmes, including social health protection and nutrition. It aims to i) support the evidence-based development at policy level in the field of SP (building upon the research hub experience and lessons learned), ii) strengthen the capacities of social workers, particularly in communities in the province of KP, and iii) promote the nutritional status of adolescent girls through coordinated interventions at the interface between SP programmes (notably BISP) and provincial governments.

5.2 Recommendations

Based on the findings and conclusions presented in this report, the evaluation suggests the following recommendations:

Steering and coordination

1. **The module and future projects should consolidate their M&E system needs to improve the quality and availability of the monitoring data**. The evaluation finds that the decentralised M&E system does not yet play the expected steering role in monitoring advances of the module interventions,

tracking detailed information on all the intended (and unintended) results, and also complicates the exercise of evaluations for accountability and learning purposes. Although the module has mitigated this weakness, the M&E systems should become standard practice for future projects. In addition, the consolidation of the M&E system could also represent an opportunity to:

- **Ensure that the strong sustainability orientation of the module is reflected in the M&E system**. The report highlights all the strengths of the module approach towards sustainability, including the integration of sustainability-oriented goals in module outputs. The formalisation of sustainability-oriented measures in the M&E system would allow future projects to track, highlight and report on the results on sustainability. For example, the module should ensure that it closely monitors the implementations and the results of the final concept that will be chosen for the training of the agents of change embedded in Output 5 and whether the expected dissemination takes place.

2. **The module and future projects should continue to improve coordination with KfW**. Although the coordination and collaboration with KfW was limited to regular meetings under the SP-SHP module, the evaluation finds that not enough information is shared, and concrete opportunities for collaboration have not materialised, despite the overlap in locations and thematic scope of the respective interventions. The evaluation recommends continuing to use the current communication channels and expanding the opportunities for joint actions. For instance, further joint studies and joint presentations should be conducted. In addition, the respective networks and resources (including human resources) of the two organisations should be further leveraged.
3. **The module and future projects should compile information and lessons learned from their innovative interventions.** The evaluation highlights the innovative approaches tested throughout the module, such as linking SP with nutrition (Output 5), supporting the deployment of mobile vans (Output 4), the introduction of Moodle for the HSA to offer courses and build a platform which is used by the CSA and its partners (Output 3), and the work along the humanitarian development nexus (e.g. with World Food Programme and UNICEF in the context of Output 5). Considering further projects will continue to work in the same fields, it would be beneficial for them to access this information in a formalised way. In addition, in line with the strong commitments made in the context of the Global Compact for Refugees and efforts to bring humanitarian and development actors closer together, the SP-SHP module should also explore the extent to which its experience (with World Food Programme, UNICEF, amongst others, under Output 5) could make a contribution to the ongoing dialogues. This is also particularly relevant in the Pakistani context, considering the situation of Afghan refugees.

Strategic and conceptual

4. **The module and future projects should continue to create linkages between their key stakeholders to complement service offering and coordination efforts, in line with the integrated approach of GIZ and the module.** The report highlights the importance of the efforts to bring closer key SP-SHP actors (through data integration but also in the context of capacity development measures) from different regions and from different federal administrative levels to create more understanding and opportunities for exchange and collaboration. The SP-SHP module should keep an updated list of partners and their interconnections and also conduct closing events at the end of the module to keep engaging partners. Future projects and other GIZ projects within the governance cluster (among others) should access this list, participate in closing events and continue maintaining interlinkages whenever possible and relevant for their respective interventions.
5. **The module and future projects should consider opening an advanced course on SP.** The report highlights the effectiveness and the sustainability of the capacity development measures. As the risk of staff turnover remains high for the sustainability of the capacities on an institutional level, the module and future projects should consider involving a larger range of positions (e.g. high-level management) in the coherent framework of capacities already developed. In particular, the module and future projects should consider creating a pool of experts in SP, by also involving academia in the training

sessions (either to receive or to provide) in order to reach a larger audience in the general population (e.g. through their own networks at universities).

6. **Future projects should consider adopting the adaptable design of the module**. The evaluation shows that the high adaptability and flexibility of the module concept has been able to answer emerging needs, remain relevant for partners including supporting them in emergency responses, and pilot various components that can drive innovation in activities concepts.

Operational and implementation

7. **Future projects and other GIZ projects should systematise their consideration to use the Regional West Bureau administrative unit**. The evaluation report highlights the effective use of the administrative unit for high-quality contract management in the context of the co-financing with GF, without diverting resources away from the technical advisors and without generating delays in implementation. In addition, the Regional West Bureau also allows the collaboration with the partners to run smoothly.
8. **Future projects should reinforce the coherence between SP and SHP for greater information and to search for complementarity, synergies, effectiveness and impact**. The evaluation highlights on several occasions the fragmentation of the SP and SHP sectors in Pakistan. However, there are a growing number of opportunities to bring the two sectors closer for greater effects on a larger scale (for example in the context of contributory health protection and financing of SP).
9. **Future projects should continue to support federal and provincial governments to consolidate their SP and SHP frameworks.** The report underlines the importance of establishing clear parameters for the development of the sectors, including through support to policy work. In a context where the development of policy frameworks for the SP-SHP sectors is still new, the evaluation team recommends continuing this process to further solidify the foundation for sustainable implementation, providing an opportunity to address structural inefficiencies and align initiatives with long-term strategic objectives, such as the development of formalised SP and SHP frameworks. In particular:
    - **Continued support should be provided to develop a policy framework for the SP and SHP sectors, at federal and provincial levels**. Any further support to policy work should rely on the coordination structures already established during the SP-SHP module in order to further anchor them institutionally. For example, **future projects should consider supporting federal and provincial governments to develop solid data protection policies.** The report highlights the strength of the module support to establish data-sharing agreements to reinforce coordination between governmental actors. However, the high risk for data security is also identified in the report. The evaluation team therefore recommends reinforcing the data protection policies to mitigate this risk.
10. **Future projects should reinforce the support to evidence-based development, in particular at policy level in the field of SP**. The report shows that the concept of the research hub supported Output 2 and was relevant to answer a clear need for evidence-based decision-making in Pakistan. However, the main components of the concept should be revised based on the experience and lessons learned from the SP-SHP module. This includes finding better-suited partners and structures that have a stronger footing in the research and political environment in Pakistan, and potentially collaborating with KfW who already supports similar interventions.

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Note: Due to data source and data protection, interviews are not listed here in detail as sources. An overview of the number and type of interviews and surveys conducted can be found in chapter 3.

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Annex: Evaluation criteria, dimensions, and questions

Relevance

OECD DAC Criterion Relevance - Is the intervention doing the right things?

The **'relevance'** criterion focuses on the intervention's design. It refers to the extent to which the objectives and design of a development intervention are consistent with the (global, country and institution-specific) requirements, needs, priorities and policies of beneficiaries and stakeholders (individuals, groups, organisations and development partners). It also identifies the ability of the intervention's design to adapt to a change in circumstances. "Relevance" is assessed in relation to 1) the **time of the intervention design**[1] and 2) from **today's perspective**[2].

Assessment dimensions	Question Type	Application in this evaluation	Evaluation questions	Clarifications
Alignment with policies and priorities	BMZ Standard	Mandatory	To what extent are the intervention's objectives aligned with the (global, regional and country specific) policies and priorities of the BMZ and of the beneficiaries and stakeholders and other (development) partners? To what extent do they take account of the relevant political and institutional environment?	• Orientation at BMZ country strategies and BMZ sector concepts • Strategic reference framework for the project (e.g. national strategies including the national implementation strategy for Agenda 2030, regional and international strategies, sectoral and cross-sectoral change strategies, in bilateral projects especially partner strategies, internal analytical framework e.g. safeguards and gender[4] • Orientation of the project design at the (national) objectives of Agenda 2030 • Project contribution to certain Sustainable Development Goals (SDGs) • Explanation of a hierarchy of the different policies, priorities (especially in case of contradictions) • For global and sector projects please use CPE factsheet on SV / GV
	Context / Conflict Sensitivity (fragility)	Applicable	To what extent was the (conflict) context of the project adequately analysed and considered for the project concept?	• Key documents: (Integrated) Peace and Conflict Assessment (I)PCA, Safeguard Conflict and Context Sensitivity documents
Alignment with the needs and capacities of the beneficiaries and stakeholders	BMZ Standard	Mandatory	To what extent are the intervention's objectives aligned with the development needs and capacities of the beneficiaries and stakeholders involved (individuals, groups and organisations)?	• Also: consideration of stakeholders such as civil society and private sector in the design of the measure
	Context / Conflict Sensitivity (fragility)	Applicable	How were deescalating factors/ connectors[5] as well as escalating factors/ dividers[6] in the project context identified and considered for the project concept (please list the factors)?[7]	• e.g., see column I and II of the (Integrated) Peace and Conflict Assessment
	Context / Conflict Sensitivity (fragility)	Applicable	To what extent were potential (security) risks for (GIZ) staff, partners, target groups/final beneficiaries identified and considered?	
	BMZ Standard	Mandatory	To what extent are the intervention's objectives geared to the needs and capacities of particularly disadvantaged and vulnerable beneficiaries and stakeholders (individuals, groups and organisations)? With respect to groups, a differentiation can be made by age, income, gender, ethnicity, etc.?	• Reaching particularly disadvantaged groups (in terms of Leave No One Behind, LNOB) • Consideration of potential for human rights and gender aspects • Consideration of identified risks
Appropriateness of the design[3]	BMZ Standard	Mandatory	To what extent is the intervention's design appropriate and realistic (in terms of technical, organisational and financial aspects)?	• Realistic project goal from today's perspective and in view of the available resources (time, finances, partner capacities) • Consideration of potential changes in the framework conditions • Dealing with the complexity of framework conditions and strategic reference frameworks and with possible overloading • Strategic focusing

Note: The evaluation matrix, including methodology, is included in the individual chapters on the OECD DAC criteria in this report.

Note: Questions in *italic* are descriptive 'learning and help questions' specified by BMZ that are relevant for the analysis but not for the final assessment. Standard questions are defined in 'Evaluation criteria for German bilateral cooperation' (BMZ, 2021)

	BMZ Standard	Mandatory	To what extent is the intervention's design sufficiently precise and plausible (in terms of the verifiability und traceability of the system of objectives and the underlying assumptions)?	Assessment of the (current) results model and results hypotheses (Theory of Change, ToC) of the actual project logic: • Adequacy of activities, instruments and outputs in relation to the project objective to be achieved • Plausibility of the underlying results hypotheses • Clear definition and plausibility of the selected system boundary (sphere of responsibility) • Appropriate consideration of potential influences of other donors/ organisations outside the project's sphere of responsibility • completeness and plausibility of assumptions and risks for the project results • How well is co-financing (if any) integrated into the overall concept of the project and what added value could be generated for the ToC/project design?
	BMZ Standard	Mandatory	To what extent is the intervention's design based on a holistic approach to sustainable development (interaction of the social, environmental and economic dimensions of sustainability)?	• Presentation of the interactions (synergies/trade-offs) of the intervention with other sectors in the project design - also regarding the sustainability dimensions in terms of Agenda 2030 (economic, ecological and social development)
Adaptability – response to change	BMZ Standard	Mandatory	To what extent has the intervention responded to changes in the environment over time (risks and potentials)?	• Reaction to changes during project including modification offers (e.g., local, national, international, sectoral changes, including state-of-the-art sectoral know-how)

(1) The 'time of the intervention design' is the point in time when the offer/most recent modification offer was approved.
(2) In relation to the current standards, knowledge and framework conditions.
(3) The design of an intervention is usually assessed by evaluating its intervention logic. The intervention logic depicts the system of objectives used by an intervention. It maps out the systematic relationships between the individual results levels. At the time an intervention is designed, the intervention logic, in the form of a logical model, is described in the offer for the intervention both as a narrative and generally also based on a results framework. The model is reviewed at the start of an evaluation and adjusted to reflect current knowledge. Comprehensive (re)constructed intervention logics are also known as "theories of change". In GIZ the 'project design' encompasses project objective (outcome) and the respective theory of change (ToC) with outputs, activities, TC-instruments and especially the results hypotheses as well as the implementation strategy (e.g., methodological approach, Capacity Development (CD) strategy). In GIZ the Theory of Change is described by the GIZ results model as graphic illustration and the narrative results hypotheses.
(4) In the GIZ Safeguards and Gender system risks are assessed before project start regarding following aspects: gender, conflict, human rights, environment and climate. For the topics gender and human rights not only risks but also potentials are assessed. Before introducing the new safeguard system in 2016 GIZ used to examine these aspects in separate checks.
(5) Deescalating factors/connectors/peace needs: e.g., peace-promoting actors and institutions, structural changes, peace-promoting norms and behaviour. For more details on 'connectors' see: GIZ (2007): 'Peace and Conflict Assessment (PCA). Ein methodischer Rahmen zur konflikt- und friedensbezogenen Ausrichtung von EZ-Maßnahmen', p. 55/135 and the iPCA Writing Template.
(6) Escalating factors/ dividers: e.g., destructive institutions, structures, norms and behaviour. For more details on 'dividers' see: GIZ (2007): 'Peace and Conflict Assessment (PCA). Ein methodischer Rahmen zur konflikt- und friedensbezogenen Ausrichtung von EZ-Maßnahmen', p. 135 and the iPCA Writing Template.
(7) All projects in fragile contexts, projects with FS1 or FS2 markers and all transitional development assistance projects should weaken escalating factors/dividers and must mitigate risks in the context of conflict, fragility and violence (and human rights). Projects with FS1 or FS2 markers should also consider how to strengthen deescalating factors/ connectors and how to address peace needs in its project objective/sub-objective.

Coherence

OECD DAC Criterion Coherence - How well does the intervention fit?

This criterion refers to the intervention's compatibility with other interventions in a country, sector or institution as well as with international norms and standards. Internal coherence addresses the synergies and division of tasks between the intervention and other interventions of German development cooperation and also the intervention's consistency with the relevant international norms and standards to which German development cooperation adheres. External coherence considers the intervention's complementarity, harmonisation and coordination with the interventions of other partners, donors and international organisations. The "coherence" criterion relates both to the intervention's design as well as to the results it achieves.

Assessment dimensions	Question Type	Application in this evaluation	Evaluation questions	Clarifications
Internal coherence	BMZ Standard	Mandatory	Within German development cooperation, to what extent is the intervention designed and implemented (in a sector, country, region or globally) in a complementary manner, based on the division of tasks?	• Also, analysis of whether the project takes the necessary steps to fully realize synergies within German development cooperation • For global and sector projects please use CPE factsheet on SV / GV
	BMZ Standard	Mandatory	To what extent are the instruments of German development cooperation (Technical and Financial Cooperation) meaningfully interlinked within the intervention (in terms of both design and implementation)? Are synergies leveraged?	• if applicable, also consider projects of different German resorts/ministries
	BMZ Standard	Mandatory	To what extent is the intervention consistent with international and national norms and standards to which German development cooperation is committed (e.g., human rights)?	
External coherence	BMZ Standard	Mandatory	To what extent does the intervention complement and support the partner's own efforts (principle of subsidiarity)?	
	BMZ Standard	Mandatory	To what extent has the intervention's design and implementation been coordinated with other donors' activities?	• Also: To what extent could synergies be achieved through co-financing (where available) with other bilateral and multilateral donors and organizations and how did co-financing contribute to improved donor coordination?
	BMZ Standard	Mandatory	To what extent has the intervention's design been designed to use existing systems and structures (of partners/other donors/international organisations) for implementing its activities? To what extent are these systems and structures used?	• Also, analysis of whether the project is taking the necessary steps to fully realize synergies with interventions of other donors at the impact level
	BMZ Standard	Mandatory	To what extent are common systems (together with partners/other donors/international organisations) used for M&E, learning and accountability?	

Effectiveness

OECD DAC Criterion Effectiveness - Is the intervention achieving its objectives?

'Effectiveness' refers to the extent to which the intervention has achieved, or is expected to achieve, its objectives (at outcome level), including any differential results across beneficiary and stakeholder groups. It examines the achievement of objectives in terms of the direct, short-term and medium-term results.

Assessment dimensions	Question Type	Application in this evaluation	Evaluation questions	Clarifications
Achievement of the (intended) objectives[1]	BMZ Standard	Mandatory	To what extent has the intervention achieved, or is the intervention expected to achieve, the (intended) objectives as originally planned (or as modified to cater for changes in the environment)?	• Assessment based on the project objective indicators (agreed with BMZ) • Check whether more specific or additional indicators are needed to adequately reflect the project objective
	Context / Conflict Sensitivity (fragility)	Applicable	For projects with FS1 or FS2 markers: To what extent was the project able to strengthen deescalating factors/ connectors/ capacities of peace?[2, 4]	
Contribution to achievement of objectives	BMZ Standard	Mandatory descriptive learning question	*To what extent have the intervention's outputs been delivered as originally planned (or as modified to cater for changes in the environment)?*	
	BMZ Standard	Mandatory	To what extent have the delivered outputs and increased capacities been used and equal access (e.g., in terms of physical, non-discriminatory and affordable access) guaranteed?	
	BMZ Standard	Mandatory	To what extent has the intervention contributed to the achievement of objectives?	• Assessment based on the activities, TC-instruments and outputs of the project (contribution-analysis as focus of this assessment dimension and minimum standard, see annotated reports) • What would have happened without the project? (Usually qualitative reflection)
	BMZ Standard	Mandatory	To what extent has the intervention contributed to the achievement of objectives at the level of the intended beneficiaries?	
	BMZ Standard	Mandatory	To what extent has the intervention contributed to the achievement of objectives at the level of particularly disadvantaged or vulnerable groups of beneficiaries and stakeholders? (These may be broken down by age, income, gender, ethnicity, etc.)?	
	BMZ Standard	Mandatory descriptive learning question	*Which internal factors (technical, organisational or financial) were decisive for achievement/non-achievement of the intervention's intended objectives?*	• Internal factors = within the project's sphere of responsibility / system boundary. The project is implemented jointly by GIZ and the official partner(s).
	BMZ Standard	Mandatory descriptive learning question	*Which external factors were decisive for achievement/non-achievement of the intervention's intended objectives (considering the anticipated risks)?*	• External factors = outside the project's sphere of responsibility / system boundary. The project is implemented jointly by GIZ and the official partner(s).
Quality of implementation	BMZ Standard	Mandatory	What assessment can be made of the quality of steering and implementation of the intervention in terms of the achievement of objectives? What assessment can be made of the quality of steering and implementation of, and participation in, the intervention by the partner/executing agency?	Capacity Works considerations: - **Results-oriented monitoring (RoM / WoM)** is established and used, e.g., for evidence-based decisions, risk management. Data are disaggregated by gender and marginalized groups. unintended positive and negative results are monitored. Conflict-sensitive monitoring and explicit risk-safety monitoring are particularly important for projects in fragile contexts. - A bindingly communicated **strategy** agreed with the partners is pursued - Involvement and **cooperation** of all relevant actors (including partners, civil society, private sector) - **Steering**: decisions influencing the project's results are made in time and evidence-informed. Decision processes are transparent. - **Processes**: Relevant change processes are anchored in the cooperation system; project-internal processes are established and regularly reflected and optimised. - **Learning and innovation**: There is a learning and innovation-friendly work

				culture that promotes the exchange of experience; learning processes are established; context-specific adjustments are possible
Unintended results	BMZ Standard	Mandatory	To what extent can unintended positive/negative direct results (social, economic, environmental and among vulnerable beneficiary groups) be observed/anticipated?	• The focus is on the outcome level, but for the analysis the unintended effects can also be included on the output level
	Context / Conflict Sensitivity (fragility)	Applicable	To what extent was the project able to ensure that escalating factors/ dividers[3] have not been strengthened (indirectly) by the project[4]? Has the project unintentionally (indirectly) supported violent or 'destructive' actors (also regarding human rights violations)?	
	BMZ Standard	Mandatory	What potential benefits/risks arise from the positive/negative unintended results? What assessment can be made of them?	• also check whether the risks were already mentioned and monitored in the design phase
	Context / Conflict Sensitivity (fragility)	Applicable	To what extent have risks and unintended-negative results in the context of conflict, fragility and violence[5] been monitored (context and/or conflict-sensitive monitoring) in a systematic way?	

(1) The first and second assessment dimensions are interrelated: If the project's contribution to achieving the objective is small (2nd assessment dimension), this must also be considered when evaluating the first assessment dimension.
(2) Deescalating factors/ connectors/ peace needs: e.g., peace-promoting actors and institutions, structural changes, peace-promoting norms and behaviour. For more details on 'connectors' see: GIZ (2007): 'Peace and Conflict Assessment (PCA). Ein methodischer Rahmen zur konflikt- und friedensbezogenen Ausrichtung von EZ-Maßnahmen', p. 55/135 and the iPCA Writing Template.
(3) Escalating factors/ dividers: e.g., destructive institutions, structures, norms and behaviour. For more details on 'dividers' see: GIZ (2007): 'Peace and Conflict Assessment (PCA). Ein methodischer Rahmen zur konflikt- und friedensbezogenen Ausrichtung von EZ-Maßnahmen', p. 135.
(4) All projects in fragile contexts, projects with FS1 or FS2 markers and all transitional development assistance projects should weaken escalating factors/dividers and must mitigate risks in the context of conflict, fragility and violence (and human rights). Projects with FS1 or FS2 markers should also consider how to strengthen deescalating factors/ connectors and how to address peace needs in its project objective/sub-objective.
(5) Risks in the context of conflict, fragility and violence: e.g., contextual (e.g., political instability, violence, economic crises, migration/refugee flows, drought, etc.), institutional (e.g., weak partner capacity, fiduciary risks, corruption, staff turnover, investment risks) and personnel (murder, robbery, kidnapping, medical care, etc.). For more details see: GIZ (2014): 'Context- and conflict-sensitive results-based monitoring system (RBM). Supplement to: The 'Guidelines on designing and using a results-based monitoring system (RBM) system.', p.27 and 28.

Impact

OECD DAC Criterion Impact (higher-level development results) - What difference does the intervention make?

Based on recognisable higher-level development changes (at impact level), the criterion of "higher level development results (at impact level)" relates to the extent to which the intervention has already produced significant positive or negative, intended or unintended results at the overarching level (contributions to the observed changes) or is expected to do so in the future. This includes any differential results across different stakeholders and beneficiaries. This criterion refers to the results of the development intervention.

Assessment dimensions	Question Type	Application in this evaluation	Evaluation questions	Clarifications
Higher-level (intended) development changes[1]	BMZ Standard	Mandatory	To what extent can the higher-level development changes (social, economic and environmental dimensions and the interactions between them) to which the intervention will/is designed to contribute be identified/foreseen)? (Specify time frame where possible.)	• Consider module proposal for suggested impact and program objective indicators (program proposal), if it is not an individual measure • Potential basis for assessment: program objective indicators, identifiers, connection to the national strategy for implementing 2030 Agenda, connection to SDGs • For global and sector projects please use CPE factsheet on SV / GV
	BMZ Standard	Mandatory	To what extent can the higher-level development changes (social, economic, environmental dimensions and the interactions between them) be identified/foreseen at the level of the intended beneficiaries? (Specify time frame where possible.)	
	BMZ Standard	Mandatory	To what extent can higher-level development changes to which the intervention will/is designed to contribute be identified/foreseen at the level of particularly disadvantaged/vulnerable groups of beneficiaries and stakeholders? (These may be broken down by age, income, gender, ethnicity, etc.) (Specify time frame where possible.)	
Contribution to higher-level (intended) development changes	BMZ Standard	Mandatory	To what extent has the intervention actually contributed to the identified and/or foreseeable higher level development changes (social, economic, environmental dimensions and their interactions, considering political stability) that it was designed to bring about?	• Contribution analysis (evaluation design) as minimum standard and focus of this assessment dimension, further approaches are possible and welcome, see also annotated reports • Evaluation of the project's contribution to impacts based on an analysis of the results hypotheses from outcome to impact level
	BMZ Standard	Mandatory	To what extent has the intervention achieved its intended (original and, where applicable, revised) development objectives?	• This question can already be assessed in Dimension 1 Question 1, the contribution to impact is assessed in Dimension 2, Question 1
	BMZ Standard	Mandatory	To what extent has the intervention achieved its (original and, where applicable, revised) development objectives at the level of the intended beneficiaries?	
	BMZ Standard	Mandatory	To what extent has the intervention contributed to higher-level development changes/changes in the lives of particularly disadvantaged or vulnerable groups of beneficiaries and stakeholders that it was designed to bring about? (These may be broken down by age, income, gender, ethnicity, etc.).	
	BMZ Standard	Mandatory descriptive learning question	*Which internal factors (technical, organisational or financial) were decisive for achievement/non-achievement of the intervention's intended development objectives?*	• Internal factors = within the project's sphere of responsibility / system boundary. The project is implemented jointly by GIZ and the official partner(s)
	BMZ Standard	Mandatory descriptive learning question	*Which external factors were decisive for the achievement/non-achievement of the intervention's intended development objectives?*	• External factors = outside the project's sphere of responsibility / system boundary. The project is implemented jointly by GIZ and the official partner(s). • Consider the activities of other actors or other policies, framework conditions, other policy areas, strategies or interests (German ministries, bilateral and multilateral development partners)
	BMZ Standard	Mandatory	To what extent has the intervention achieved structural or institutional changes (e.g., for organisations, systems and regulations)?	

	BMZ Standard	Mandatory	To what extent did the intervention serve as a model and/or achieve broad-based impact?	• Scaling-up is a consciously designed process to anchor changes in organisations and cooperation systems (e.g., concepts, approaches, methods) to generate broad impact • There is vertical scaling-up, horizontal scaling-up, functional scaling-up or a combination of these[2] • also analyse possible potential and reasons for not exploiting it
	BMZ Standard	Mandatory descriptive learning question	*How would the situation have developed without the intervention?*	• usually qualitative reflection, quantitative approaches welcome
Contribution to higher-level (unintended) development changes	BMZ Standard	Mandatory	To what extent can higher-level, unintended development changes (social, economic and environmental dimensions and their interactions, considering political stability) be identified/foreseen? (Specify time frame where possible.)	
	Context / Conflict Sensitivity (fragility)	Applicable	To what extent did the project have (unintended) negative or escalating effects on the conflict, context of fragility or human rights (e.g., conflict dynamics, violence, legitimacy of state and non-state actors/institutions)? To what extent did the project have positive or deescalating effects on the conflict, context of fragility or human rights (e.g., conflict dynamics, violence, legitimacy of state and non-state actors/institutions)?	
	BMZ Standard	Mandatory	To what extent has the intervention brought about foreseeable/identifiable unintended (positive and/or negative) higher-level development results?	• Analyse whether the risks were already known in the design phase • Check how the assessment of risks in connection with (unintended) negative or (not formally agreed) positive results at the impact level in the monitoring system has been carried out (e.g. use of 'compass') • measures taken to avoid or counteract the risks/ negative effects/ trade-offs[3] • Determine relevant framework conditions for negative results and the project's reaction to them • Examine to what extent potential (not formally agreed) positive results and synergies between the ecological, economic and social development dimensions have been monitored and exploited
	BMZ Standard	Mandatory	To what extent has the intervention contributed to foreseeable/identifiable unintended (positive and/or negative) higher-level development results at the level of particularly disadvantaged or vulnerable groups of beneficiaries and stakeholders? (These may be broken down by age, income, gender, ethnicity, etc.)	

(1) The first and second assessment dimensions are interrelated: If the project's contribution to achieving the objective is small (2nd assessment dimension), this must also be considered when evaluating the first assessment dimension.

(2) See GIZ 2016 'Guidelines on scaling-up for programme managers (AV) and planning officers'

(3) Risks, negative effects and trade-offs are separate aspects that should be discussed individually at this point.

Efficiency

OECD DAC Criterion Efficiency - How well are resources being used?

This criterion describes the extent to which the intervention delivers results in an economic and timely way (relationship between input and output, outcome and impact level). The evaluation dimension “production efficiency” refers to the appropriateness of the relationship between inputs and outputs. The evaluation dimension “allocation efficiency” refers to the appropriateness of the relationship between the inputs and the results achieved (project/development objective; outcome/impact level) by the intervention. The "efficiency" criterion relates both to the intervention’s design and implementation and to the results it achieves.

Assessment dimensions	Question Type	Application in this evaluation	Evaluation questions	Clarifications
Production efficiency	BMZ Standard	Mandatory descriptive learning question	*How are the intervention’s inputs (financial, human and material resources) distributed (e.g.by instruments, sectors, sub-interventions, considering the cost contributions of partners/executing agencies/other beneficiaries and stakeholders etc.)?*	• Description of the data: Costs per output, type of costs, agreed and provided partner contributions • Description of the deviations between original planned costs and actual costs (with comprehensible justification, changes are certainly desirable for increased efficiency) • For global and sector projects please use CPE factsheet on SV / GV
	BMZ Standard	Mandatory	To what extent have the intervention’s inputs (financial, human and material resources) been used economically in relation to the outputs delivered (products, investment goods and services)? If possible, refer to data from other evaluations in a region or sector, for instance.	• Use of 'Efficiency tool' including instructions and use of the follow-the-money approach as evaluation design (may be combined with other high-quality approaches) • Output level: Analysis of approaches and activities as well as TC instruments (personnel instruments, financing, materials and equipment)[1] compared to possible alternatives with a focus on the minimum principle (use of comparative data if available) • The project is oriented on internal or external benchmarks in order to achieve its effects economically • Regular reflection of the resources used by the project with focus on economically use of resources and cost risks • The overarching costs of the project are in an appropriate proportion to the costs of the outputs
	BMZ Standard	Mandatory	To what extent could the intervention’s outputs (products, investment goods and services) have been increased through the alternative use of inputs (financial, human and material resources)? If possible, refer to data from other evaluations of a region or sector, for instance. (If applicable, this question adds a complementary perspective*) * This case is always applicable in the technical cooperation (TC), please answer the question bindingly	• Use of 'Efficiency tool' including instructions and use of the follow-the-money approach as evaluation design (may be combined with other high-quality approaches) • Output level: Analysis of approaches and activities as well as TC instruments (personnel instruments, financing, materials and equipment)[1] compared to possible alternatives with focus on output maximization (use of comparative data if available) • Analysis of alternative options for allocating resources and shifts between outputs for output maximisation • saved resources can and should be used to maximise outputs • Reflection of the resources during the design phase and regularly during the implementation of the project with focus on output maximisation (with comprehensible justification, changes are certainly desirable for increased efficiency) • 'maximising outputs' means with the same resources, under the same conditions and with the same or better quality
	BMZ Standard	Mandatory	Were the outputs (products, investment goods and services) produced on time and within the planned time frame?	
Allocation efficiency	BMZ Standard	Mandatory descriptive learning question	*By what other means and at what cost could the results achieved (higher-level project objective) have been attained?*	
	BMZ Standard	Mandatory	To what extent – compared with alternative designs for the intervention – could the results have been attained more cost-effectively?	• Outcome level: Analysis of approaches and activities as well as TC-instruments in comparison to possible alternatives with focus on minimum principle (use of comparative data if available) • Regular reflection in the project of the input-outcome relation and alternatives as well as cost risks • The partner contributions are proportionate to the costs for the outcome of the project

	BMZ Standard	Mandatory	To what extent – compared with alternative designs for the intervention – could the positive results have been increased using the existing resources? (If applicable, this question adds a complementary perspective*) * This case is always applicable in the technical cooperation (TC), please answer the question bindingly	• Outcome level: Analysis of applied approaches and activities as well as TC-instruments compared to possible alternatives with focus on maximizing the outcome (real comparison if available) • The project manages its resources between the outputs in such a way that the maximum effects in terms of the module objective are achieved • Regular reflection in the project of the input-outcome relation and alternatives • Reflection and realization of possibilities for scaling-up • If additional funds (e.g. co-financing) have been raised: Effects on input-outcome ratio (e.g. via economies of scale) and the ratio of administrative costs to total costs • Losses in efficiency due to insufficient coordination and complementarity within German DC are sufficiently avoided

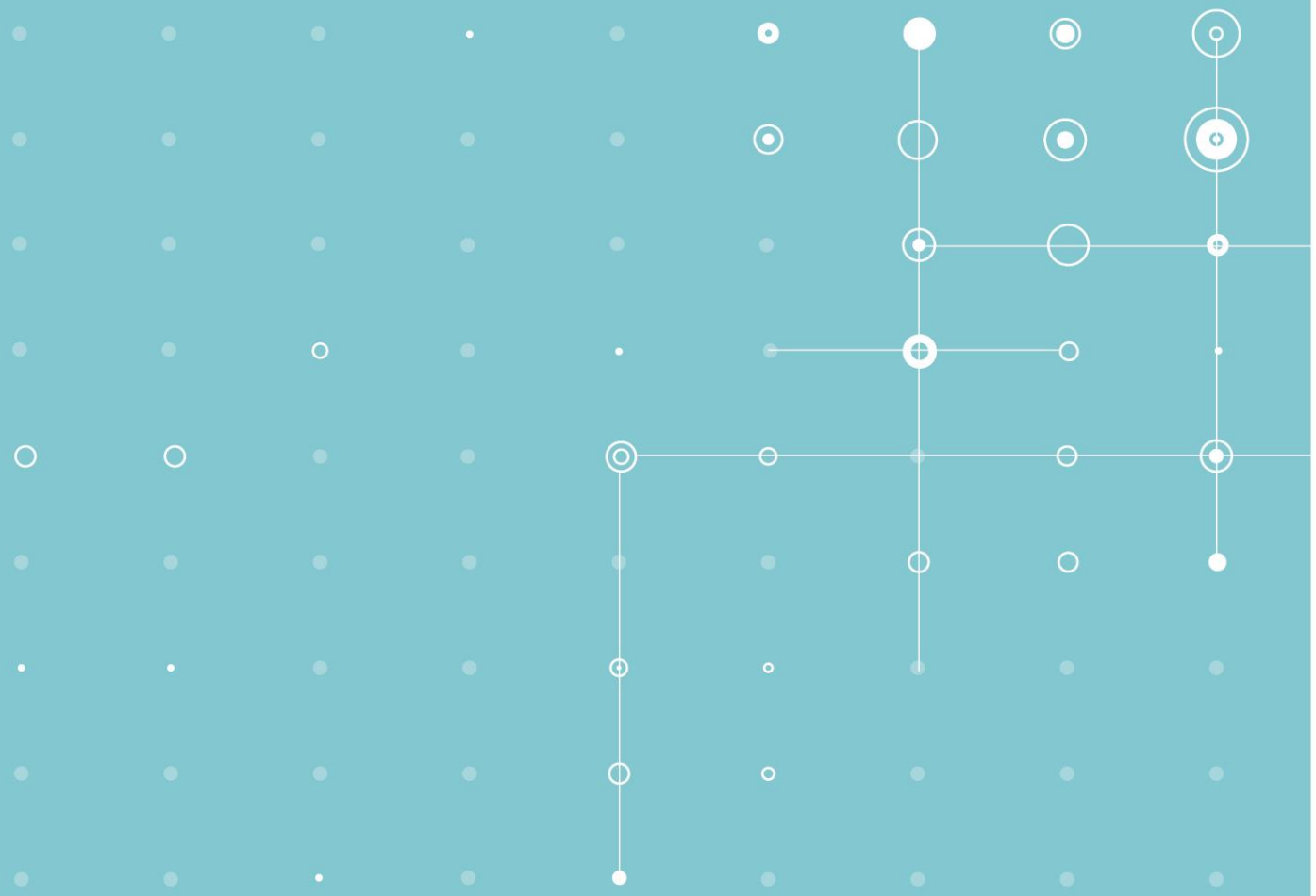
(1) see GIZ 2015: 'Integration of TC Instruments – Key Elements', based on BMZ 2014: 'Handbuch der bilateralen TZ Verfahrensinformation Nr. VI0362014 'Eckpunkte zur Instrumentenintegration'

Sustainability

OECD DAC Criterion Sustainability - Will the benefits last?

The 'sustainability' criterion relates to continued long-term benefits (at the outcome and impact level) or the probability of continued long-term benefits – taking into account observed or foreseeable risks – over time, particularly after assistance has ended.

Assessment dimensions	Question Type	Application in this evaluation	Evaluation questions	Clarifications
Capacities of the beneficiaries and stakeholders	BMZ Standard	Mandatory	To what extent do the beneficiaries and stakeholders (individuals, groups and organisations, partners and executing agencies) have the institutional, human and financial resources as well as the willingness (ownership) required to sustain the positive results of the intervention over time (once assistance has drawn to a close)?	• Transitional Development Assistance (TDA) projects primarily address final beneficiaries, whose resilience to crises and recurring shocks is to be strengthened. The focus for TDA projects is thus often on the resilience of final beneficiaries and/or at least the continuity of the measure (see explanation in dimension 3) (clarification in the inception phase of the evaluation). • For global and sector projects please use CPE factsheet on SV / GV
	BMZ Standard	Mandatory	To what extent do the beneficiaries and stakeholders (individuals, groups and organisations, partners and executing agencies) have the resilience to overcome future risks that could jeopardise the intervention's results?	
Contribution to supporting sustainable capacities	BMZ Standard	Mandatory	To what extent has the intervention contributed to the beneficiaries and stakeholders (individuals, groups and organisations, partners and executing agencies) having the institutional, human and financial resources as well as the willingness (ownership) required to sustain the intervention's positive results over time and to limit the impact of any negative results?	• Analysis of the preparation and documentation of learning experiences • Description of the anchoring of contents, approaches, methods and concepts in the partner system • Reference to exit strategy of the project • If there is a follow-on project, check to what extent the results of the evaluated project are taken up; the anchoring of the effects in the partner's organisation should be pursued independently of a follow-on project, since sustainability should be achieved even without donor funds • Transitional Development Assistance (TDA) projects primarily address final beneficiaries, whose resilience to crises and recurring shocks is to be strengthened. The focus for TDA projects is thus often on the resilience of final beneficiaries and/or at least the continuity of the measure (see explanation in dimension 3) (clarification in the inception phase of the evaluation).
	BMZ Standard	Mandatory	To what extent has the intervention contributed to strengthening the resilience of the beneficiaries and stakeholders (individuals, groups and organisations, partners and executing agencies)?	
	BMZ Standard	Mandatory	To what extent has the intervention contributed to strengthening the resilience of particularly disadvantaged groups? (These may be broken down by age, income, gender, ethnicity, etc.)	
Durability of results over time	BMZ Standard	Mandatory descriptive learning question	*How stable is the context in which the intervention operates?*	
	BMZ Standard	Mandatory descriptive learning question	*To what extent is the durability of the intervention's positive results influenced by the context?*	• Consideration of risks and potentials for the long-term stability of the results and description of the reaction of the project to these
	BMZ Standard	Mandatory	To what extent can the positive (and any negative) results of the intervention be deemed durable?	• Consideration of the extent to which continued use of the results by partners and beneficiaries can be foreseen • Reference to conditions and their influence on the durability, longevity and resilience of the effects (outcome and impact) • In the case of projects in the field of Transitional Development Assistance (TDA), at least the continuity of the measure must be examined: To what extent will services or results be continued in future projects (of GIZ or other donors/organizations) or their sustainability ensured? (Clarification in the inception phase)



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